AED Operation -- Child and Infant

Israel — CHILD

CALL 101 (MDA) — USE AED IMMEDIATELY

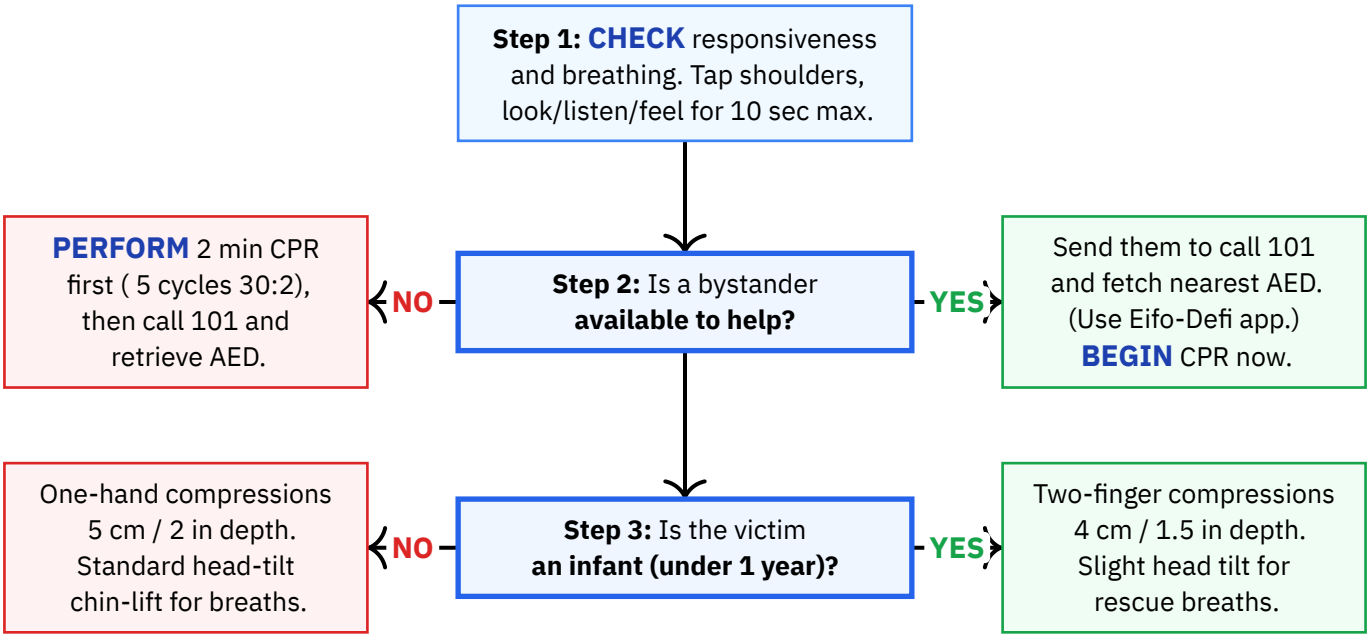
MDA: 101 **Police:** 100 **Fire:** 102 **Hatzalah:** 1221 **Poison:** 04-7771900

When to apply:

Child (age 1–8) or infant (under 1 year) who is unresponsive, not breathing normally, and in suspected cardiac arrest. If alone with a child: perform 2 minutes of CPR first, then call 101 and retrieve AED (cardiac arrest in children is more often respiratory in origin).

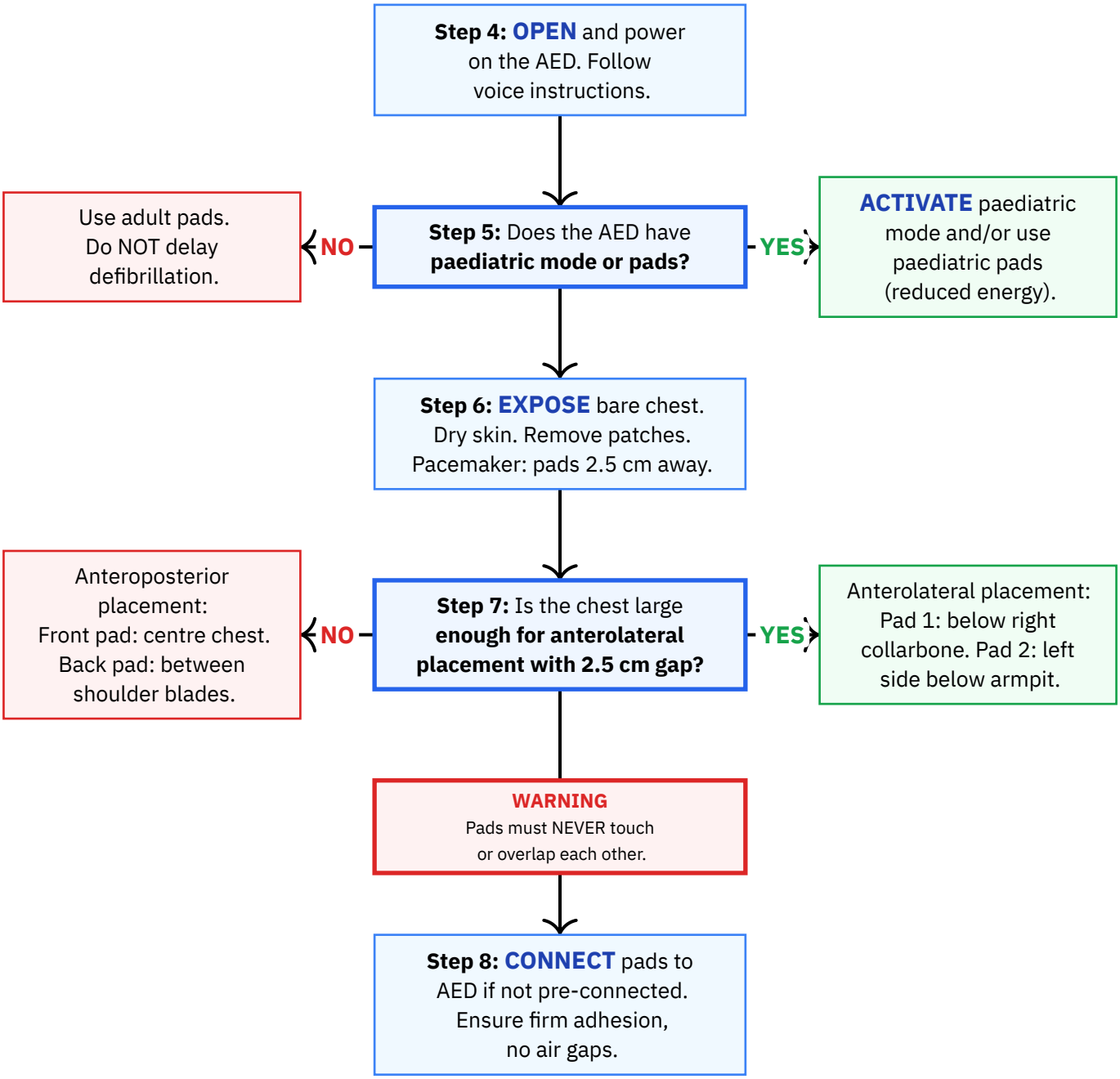
Summary: Activate paediatric mode or use paediatric pads. For small chests: anteroposterior (front-and-back) pad placement. Follow AED voice prompts. Integrate with age-appropriate CPR. Do NOT delay defibrillation for paediatric pads – use adult pads if needed.

Recognition and CPR – Steps 1-3

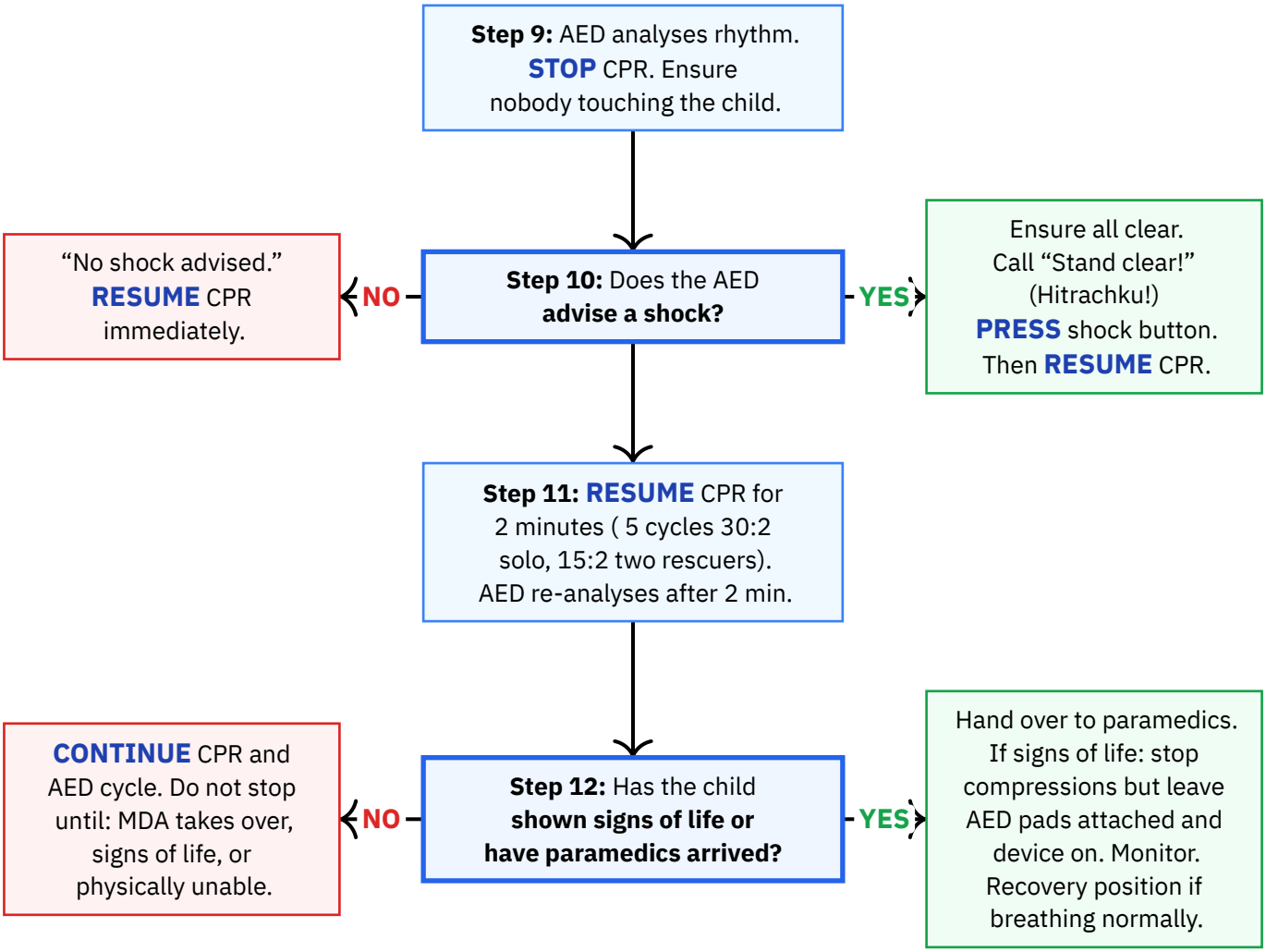


Personal reference only — not medical advice

AED Setup and Pad Placement – Steps 4-8



AED Analysis and CPR Cycles – Steps 9-12



Key Paediatric CPR/AED Parameters

Child (1–8 yr) depth: 5 cm / 2 in
Compression rate: 100–120/min
Ratio (2 rescuers): 15:2
Infant technique: 2-finger or 2-thumb
Infant (under 1 yr) depth: 4 cm / 1.5 in
Ratio (solo): 30:2
Child technique: 1-hand compression
AED re-analysis: every 2 minutes

DO NOT:

X DO NOT use AED on a conscious, breathing child -- only for cardiac arrest.
X DO NOT use AED in water or near flammable gases.
X DO NOT delay defibrillation because paediatric pads are unavailable -- use adult pads.
X DO NOT allow pads to touch or overlap -- use anteroposterior if chest too small.
X DO NOT touch the child during rhythm analysis or shock delivery.
X DO NOT turn off AED or remove pads once applied, even if child regains consciousness.
X DO NOT perform adult-depth compressions on a child/infant -- one-third chest depth only.
X DO NOT use anterolateral placement on infant/small child if pads would overlap.

Equipment needed:

— AED (Automated External Defibrillator)
— Paediatric AED pads (preferred for children under 8 / under 25 kg)
— Adult AED pads (if paediatric pads unavailable)
— Scissors or shears (to cut away clothing)
— Towel or cloth (to dry the chest)

Source: Magen David Adom (MDA) – AED Operation for Children and Infants, based on AHA 2025 Guidelines for CPR and ECC
URL: https://www.mdais.org/kursim/defibrillator
Imported: 2026-03-16 · **Last verified:** 2026-03-16
Notes: Unified from MDA and United Hatzalah sources. Age categories per MDA/AHA: infant (under 1 year), child (1–8 years). Children 8+ or showing puberty signs use adult AED protocol.

Anaphylaxis (Severe Allergic Reaction) — Child

Israel — CHILD

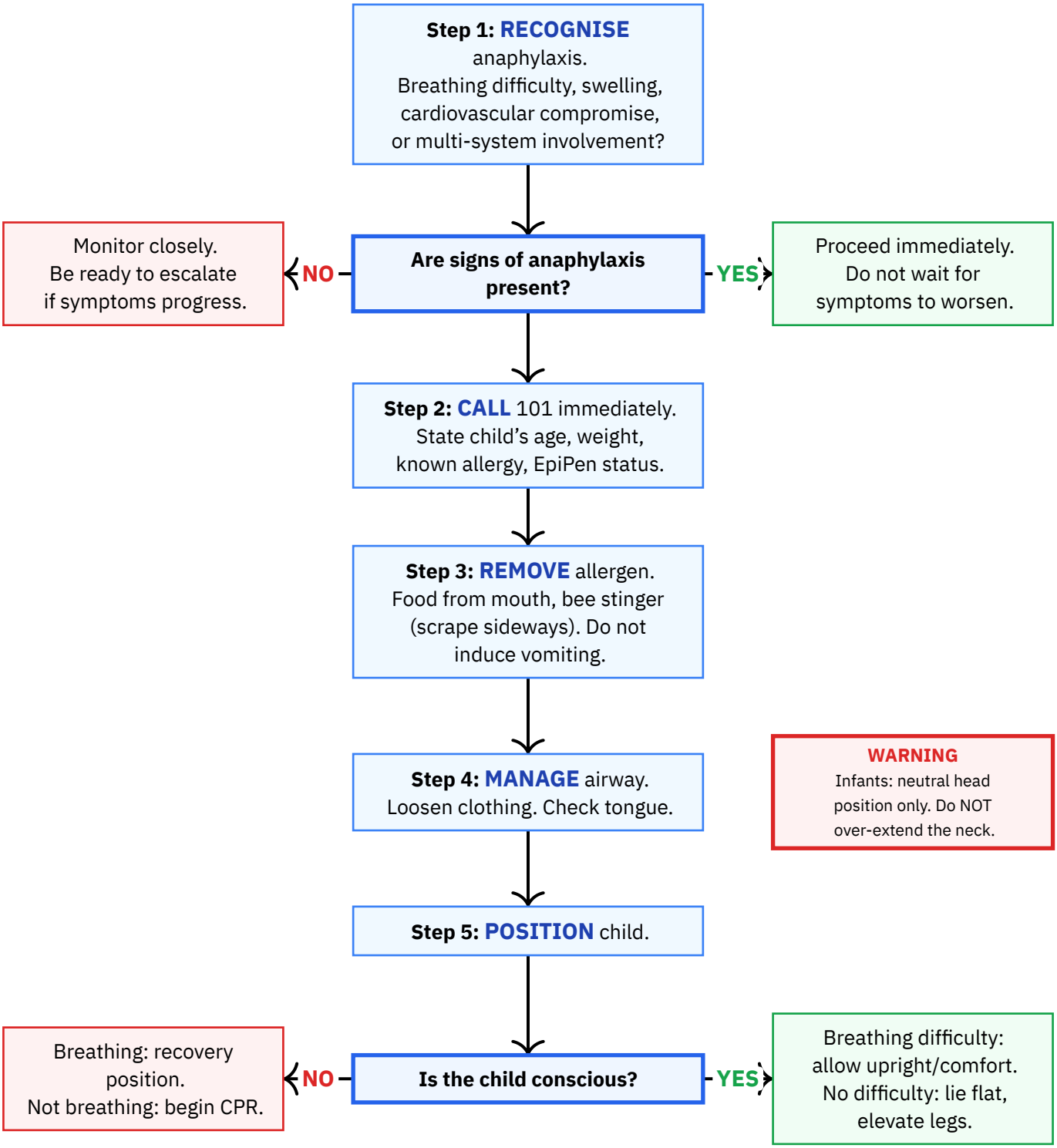
CALL 101 (MDA) — EPIPEN FIRST, THEN CALL

MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

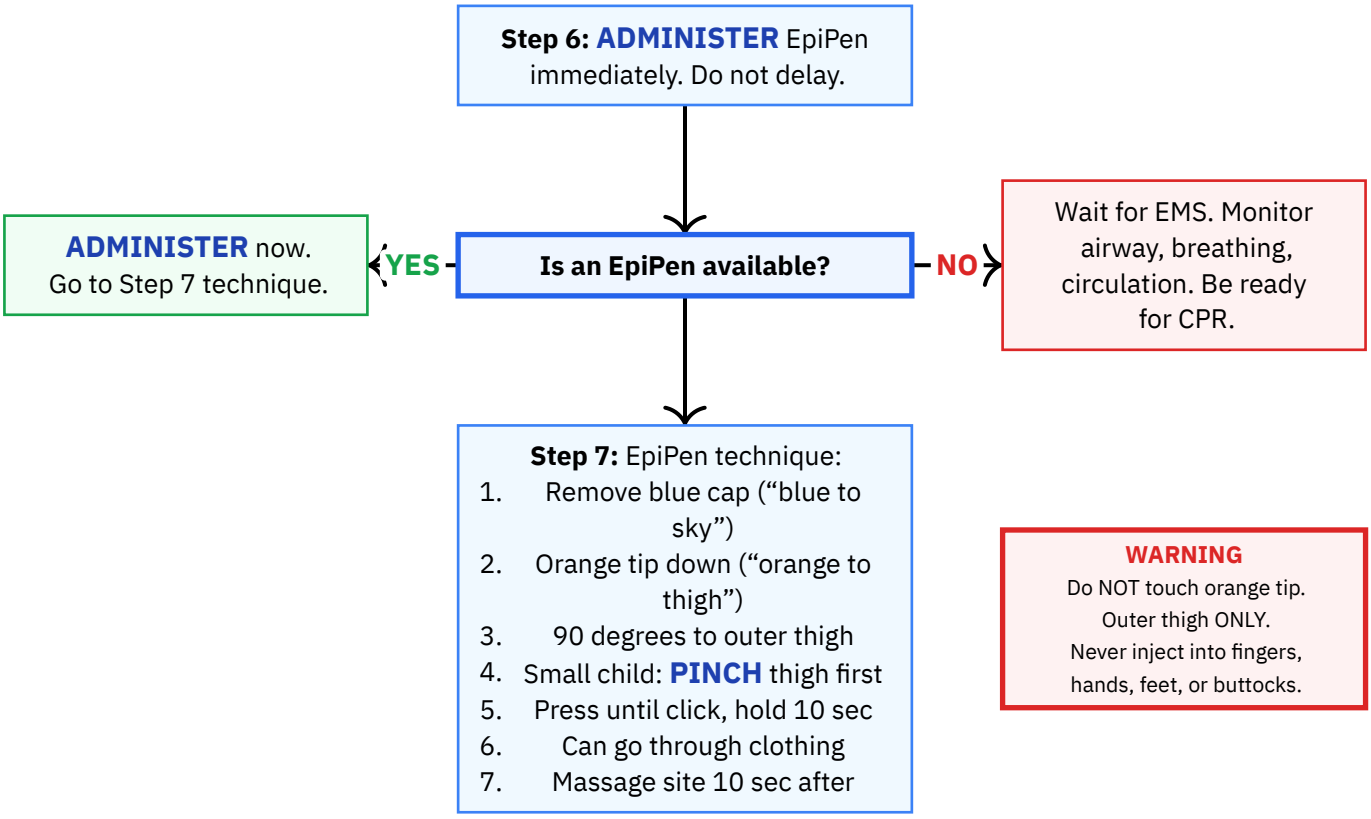
When to apply:

Child showing signs of severe allergic reaction: hives, facial/throat swelling, wheezing, stridor, difficulty breathing, weak/rapid pulse, confusion, loss of consciousness. Any involvement of breathing, circulation, or consciousness indicates anaphylaxis – do not wait.

Recognition and Initial Response (Steps 1–5)



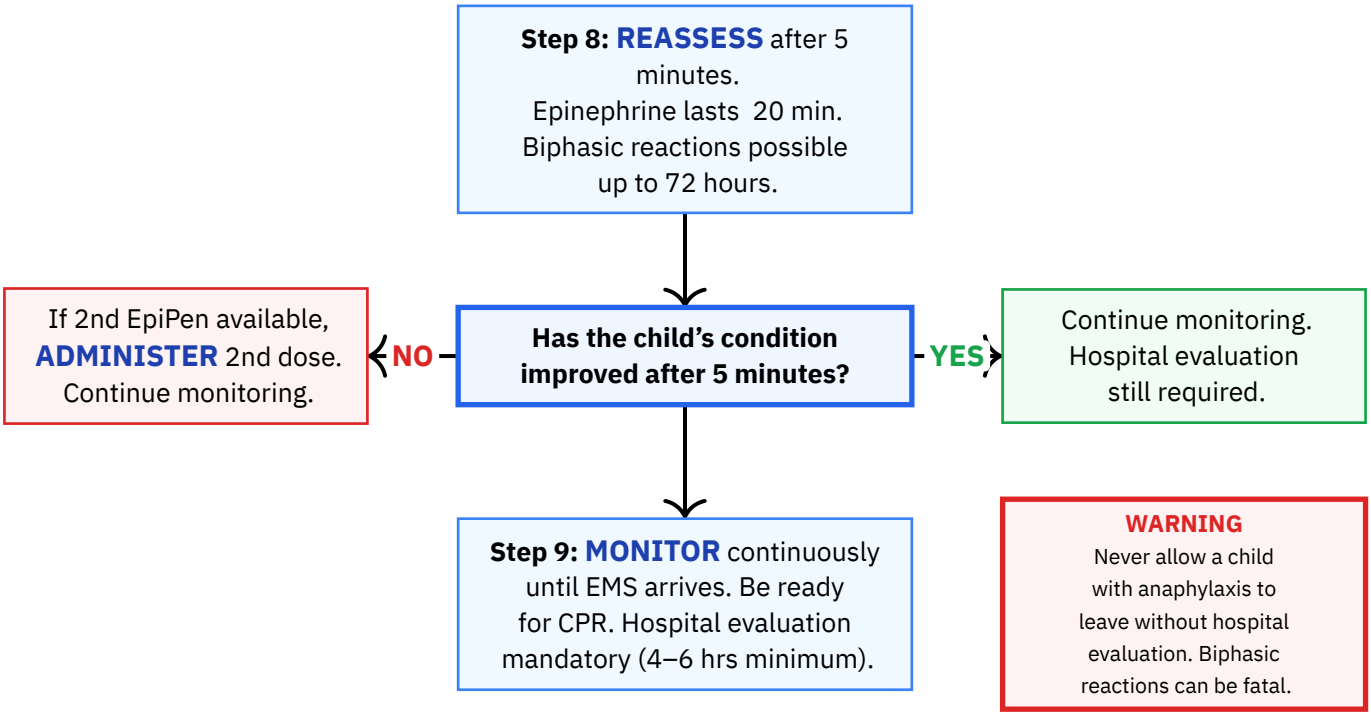
EpiPen Administration (Steps 6–7)



EpiPen Dosing by Weight

15–30 kg: EpiPen Junior 0.15 mg (green label)
Over 30 kg: EpiPen 0.3 mg (yellow label)
Under 15 kg: No auto-injector for this range, but 0.15 mg EpiPen Junior is still recommended over withholding treatment.
Note: Some Israeli allergists prescribe 0.3 mg from 20 kg – follow individual prescription.

Reassessment and Monitoring (Steps 8–9)



DO NOT:

X Do not delay epinephrine -- it is the only effective treatment for anaphylaxis.
X Do not substitute antihistamines (Benadryl) for epinephrine in anaphylaxis.
X Do not force the child to lie down if having breathing difficulty -- allow upright position.
X Do not over-extend an infant's neck -- use neutral position only.
X Do not inject epinephrine into fingers, hands, feet, or buttocks -- outer thigh only.
X Do not touch the orange tip of the EpiPen.
X Do not induce vomiting if allergen was ingested.
X Do not allow a child with anaphylaxis to leave without hospital evaluation.
X Do not store EpiPen in refrigerator, car, or direct sunlight -- Israeli heat risk.
X Do not give oral medications to a child with throat swelling or difficulty swallowing.

Equipment needed:

— EpiPen Junior 0.15 mg (green label) -- for children 15-30 kg
— EpiPen 0.3 mg (yellow label) -- for children over 30 kg
— Phone to call MDA at 101

Source: Magen David Adom (MDA) — ALS Manual 2024 / BLS Guide 2016
URL: https://www.mdais.org/media/5033/als-2024.pdf
Publication date: 2024-04-01 · **Imported:** 2026-03-15 · **Last verified:** 2026-03-15
Notes: Unified from MDA and United Hatzalah. Israeli Ministry of Education directive requires epinephrine availability in schools/ganim.

Asthma Attack First Aid -- Child

Israel — CHILD

CALL 101 (MDA) IF NO IMPROVEMENT

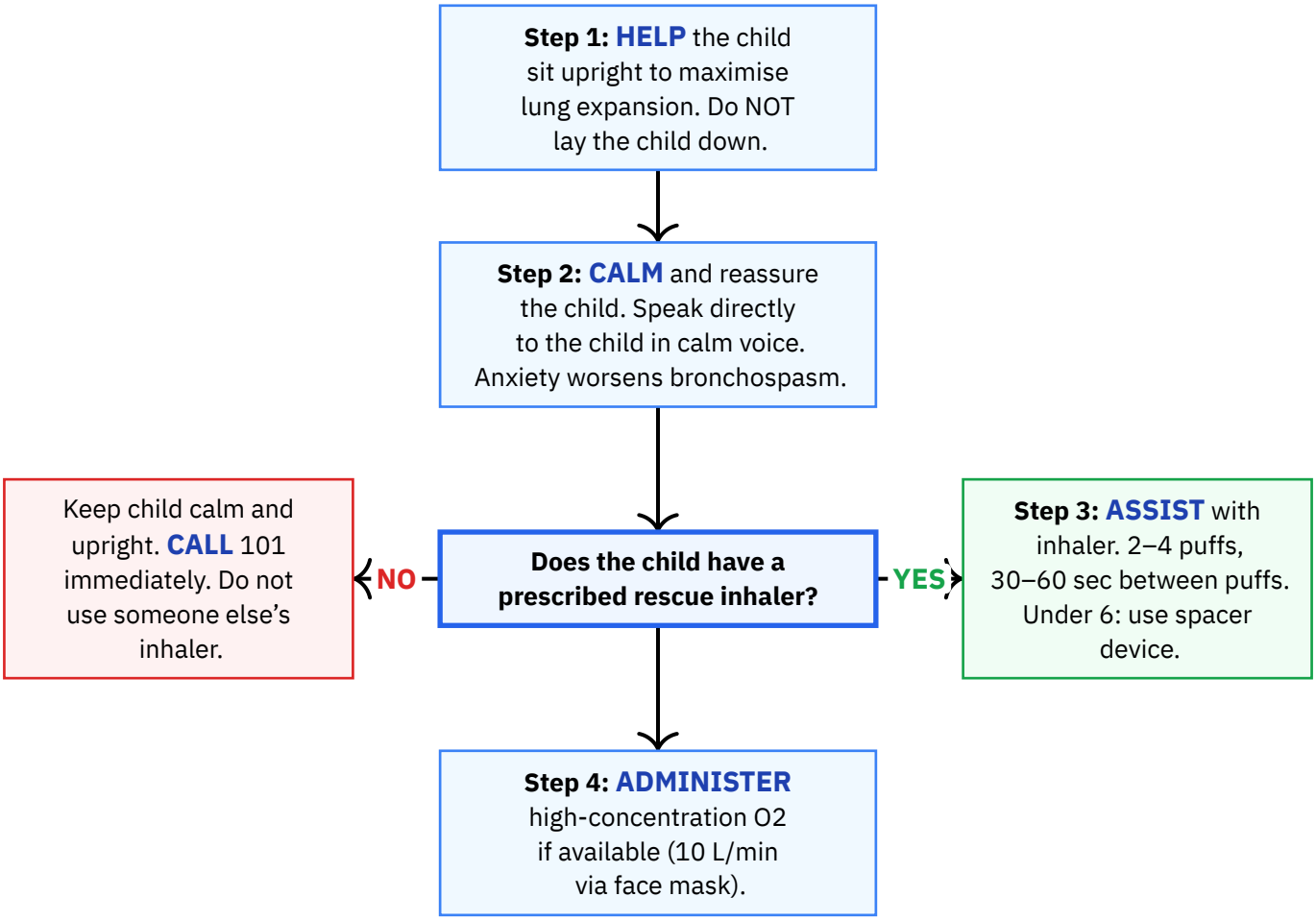
MDA: 101 **Police:** 100 **Fire:** 102 **Hatzalah:** 1221 **Poison:** 04-7771900

When to apply:

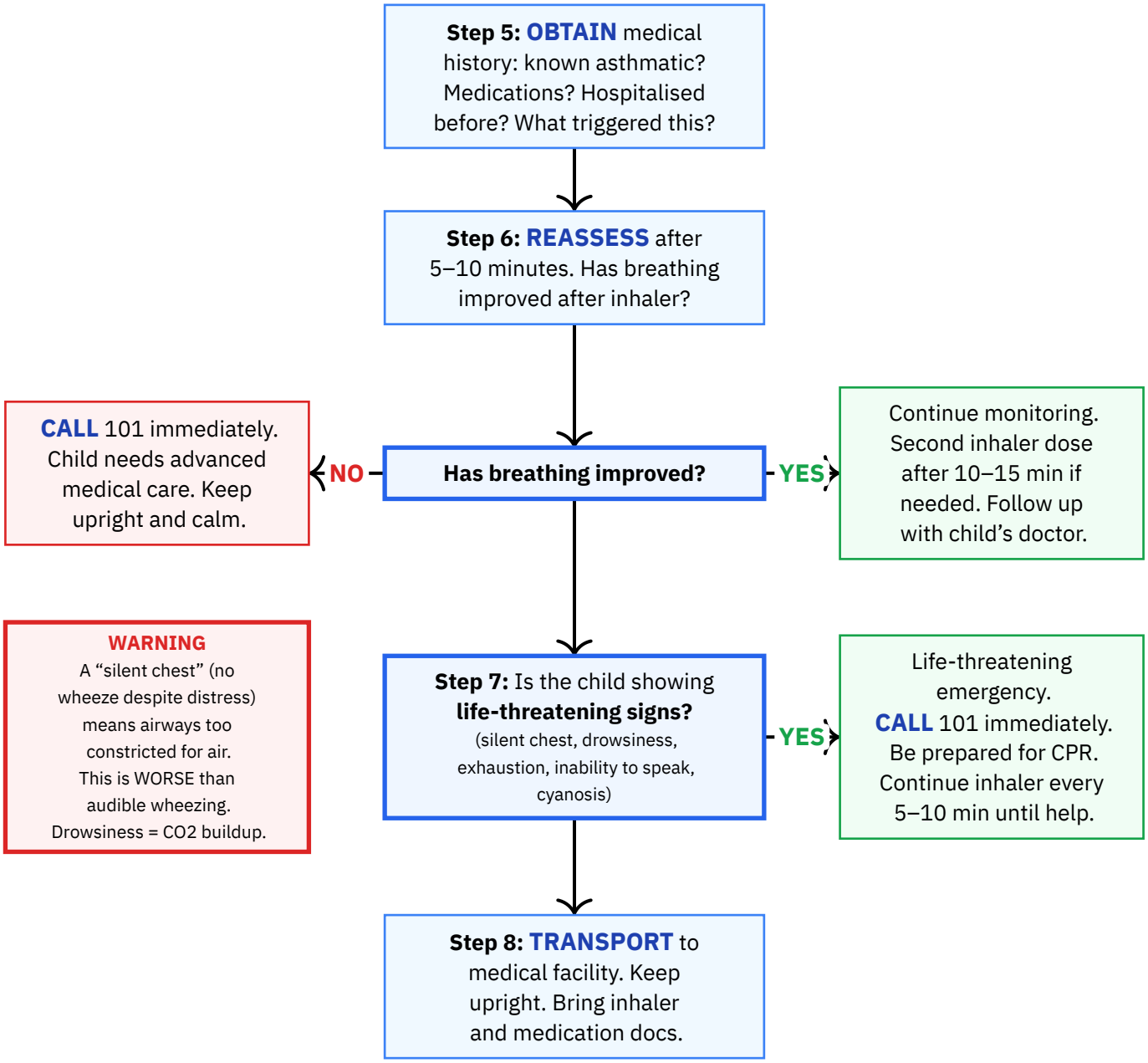
When a child is experiencing an asthma attack: persistent cough, wheezing, difficulty breathing (especially exhaling), chest tightness, lip cyanosis, accessory muscle use, intercostal retractions, nasal flaring. Asthma is the most common chronic disease in Israeli children (5–10% prevalence).

Summary: Sit upright. Calm the child. Assist with rescue inhaler (2–4 puffs, spacer for under 6). Monitor for improvement. Call 101 if no improvement or worsening. A “silent chest” is a danger sign.

Initial Treatment – Steps 1-4



Monitoring and Escalation – Steps 5-8



DO NOT:

- X Do NOT lay the child down -- upright position is essential for breathing.
- X Do NOT give cold drinks during an attack -- cold can worsen bronchospasm.
- X Do NOT leave the child alone during an attack.
- X Do NOT administer someone else's prescription inhaler without medical direction.
- X Do NOT delay calling 101 if the child is not improving.
- X Do NOT ignore drowsiness during an asthma attack -- dangerous sign of CO2 buildup.
- X Do NOT assume a quiet chest means improvement -- silent chest during distress is life-threatening.

Equipment needed:

— Rescue inhaler (Ventolin/salbutamol) -- child's own prescribed inhaler
— Spacer device (for children under 6)
— Supplemental oxygen and face mask (if available)
— Pulse oximeter (if available)
— Phone (to call 101)

Source: Magen David Adom (MDA) – ALS Professional Manual (April 2024), supplemented by Ichud Hatzalah asthma and respiratory distress guidelines
URL: https://www.mdais.org/101/first-aid
Publication date: 2024-04-01
Imported: 2026-03-16 · **Last verified:** 2026-03-16
Notes: Unified from MDA and United Hatzalah sources. Asthma is the most common chronic disease in Israeli children (5–10% prevalence).

Blast / Shrapnel Injuries First Aid -- Child

Israel — CHILD

CALL 101 (MDA) — SCENE SAFETY FIRST

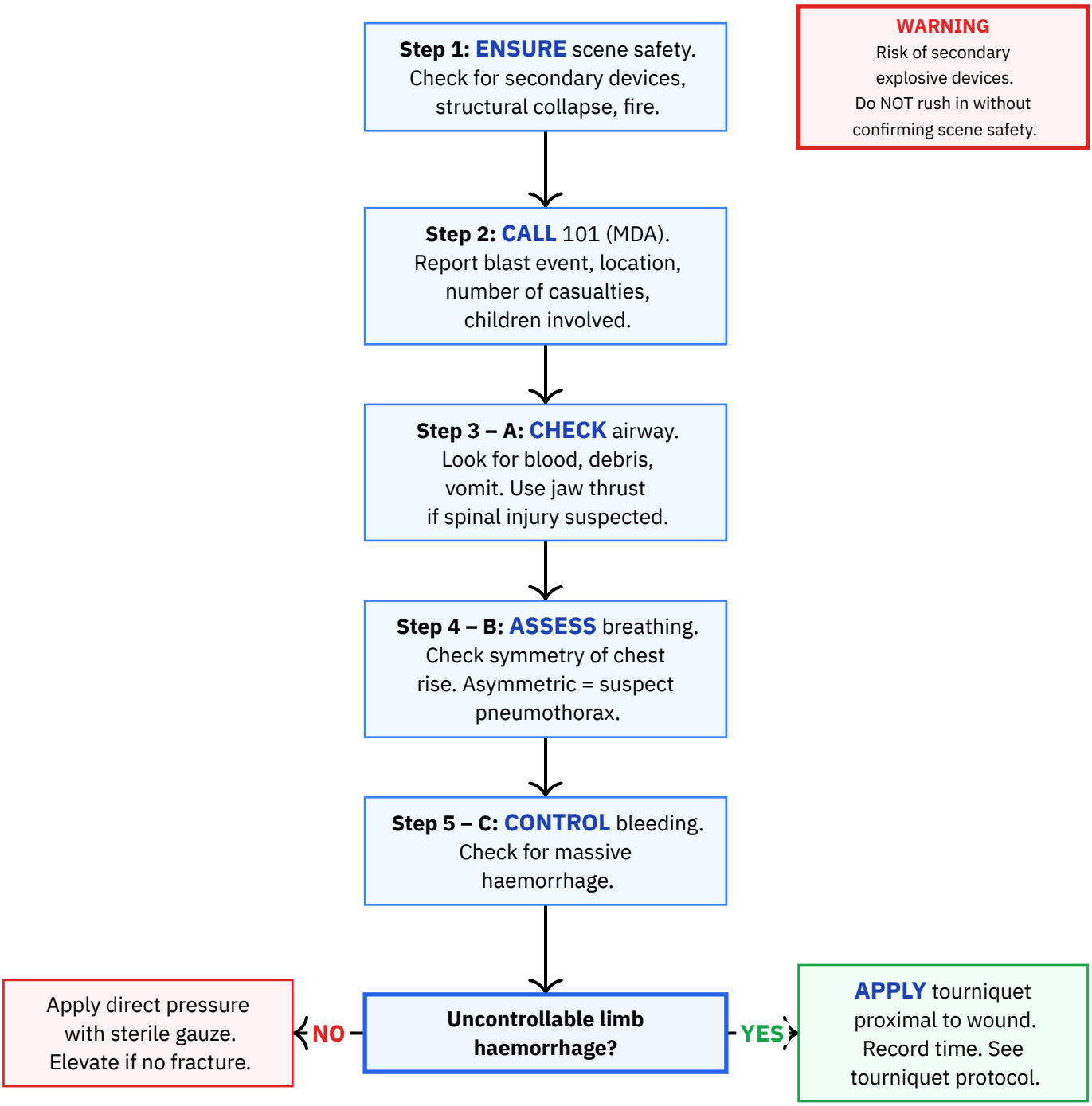
MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

When a child has been injured by a blast event: rocket/missile attack, explosive device, or any explosion. Children are at higher risk due to smaller body mass, thinner chest wall, less blood volume, and greater energy transfer per kilogram. Follows PHTLS methodology (SABCDE).

Summary: Scene safety first (secondary device risk). SABCDE assessment: Scene, Airway, Breathing, Circulation, Disability, Exposure. Control bleeding. Do NOT remove embedded objects. Prevent hypothermia. Keep child with parent/caregiver.

Scene Safety and ABC – Steps 1-5



Disability, Exposure, and Care – Steps 6-10

Step 6 – D: ASSESS neurological status (AVPU). Check pupil size/reactivity.

WARNING
A silent, non-communicative child after blast may have serious injuries. Do not assume uninjured because they are quiet.

↓

Step 7 – E: EXPOSE and examine for additional wounds, burns, embedded objects, fractures. Then **COVER** immediately to prevent hypothermia.

WARNING
Do NOT remove embedded objects (shrapnel, fragments). Stabilise them in place.

↓

Step 8: COVER open wounds with sterile dressings. Build padding around embedded objects.

↓

Step 9: KEEP child with parent or caregiver. Separation compounds psychological trauma. Speak calmly and honestly.

↓

Step 10: MONITOR continuously. Keep warm and covered. Monitor breathing, pulse, consciousness. Be prepared for CPR.

DO NOT:

- X Do NOT enter an unsafe blast scene -- risk of secondary devices and structural collapse.
- X Do NOT remove embedded objects (shrapnel, fragments) -- stabilise them in place.
- X Do NOT assume a quiet child is uninjured -- children may be in psychological shock.
- X Do NOT remove a tourniquet once applied -- hospital personnel will manage removal.
- X Do NOT forget to prevent hypothermia after examination -- children lose heat rapidly.
- X Do NOT separate the child from their parent or caregiver unnecessarily.
- X Do NOT ignore asymmetric chest rise -- indicates thoracic trauma requiring urgent attention.
- X Do NOT give food or drink if significant injuries present -- may need anaesthesia at hospital.

Equipment needed:

— Tourniquet (CAT or equivalent -- see tourniquet protocol for child-specific sizing)
— Sterile gauze and bandages
— Blankets (for hypothermia prevention)
— Rigid splints (for fracture immobilisation)
— Phone (to call 101)
— Supplemental oxygen and mask (if available)

Source: Magen David Adom (MDA) – PHTLS-aligned trauma protocols, supplemented by Ichud Hatzalah missile strike response and SABCDE assessment guidelines
URL: https://www.mdais.org/101/first-aid
Publication date: 2024-04-01
Imported: 2026-03-16 · **Last verified:** 2026-03-16
Notes: Unified from MDA and United Hatzalah sources. Both follow PHTLS methodology. Both operate psychotrauma units for paediatric psychological first aid.

Burns First Aid — Child

Israel — CHILD

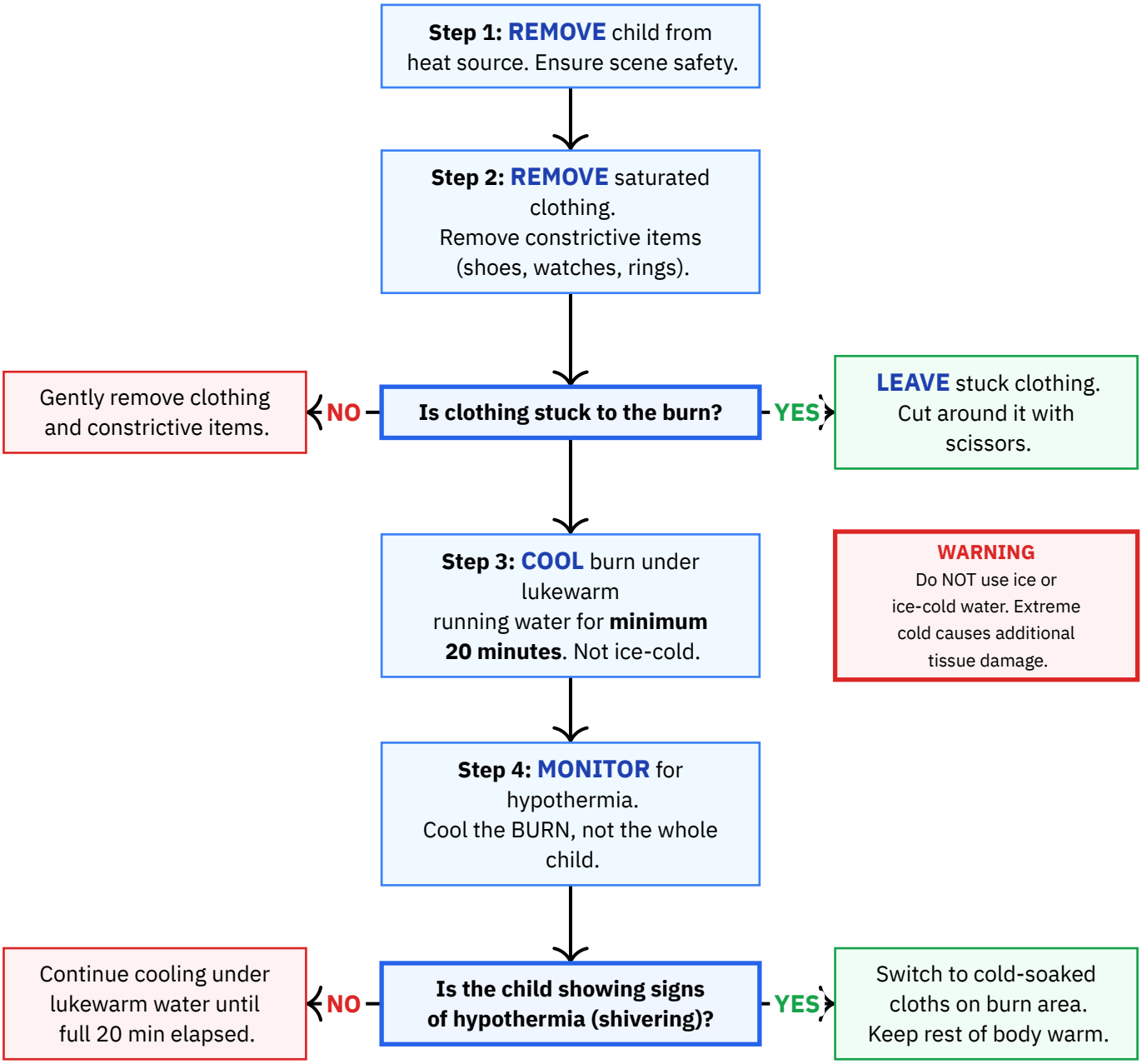
CALL 101 (MDA) FOR SERIOUS BURNS

MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

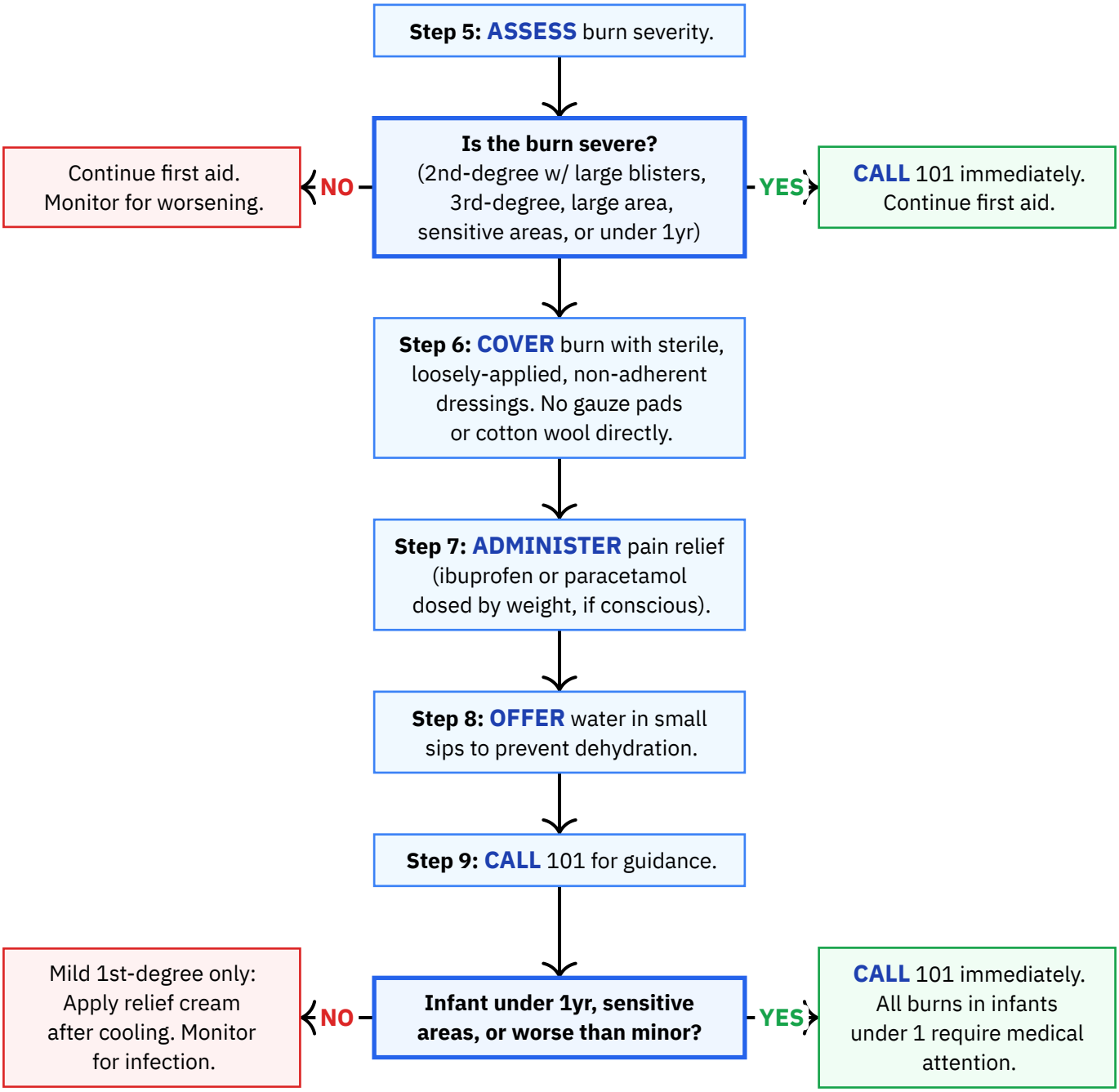
When to apply:

Child has sustained a burn from heat, hot liquids (scalds), chemicals, electricity, or radiation. 84% of paediatric burns in Israel are scalds. All burns in infants under 1 year require immediate medical attention.

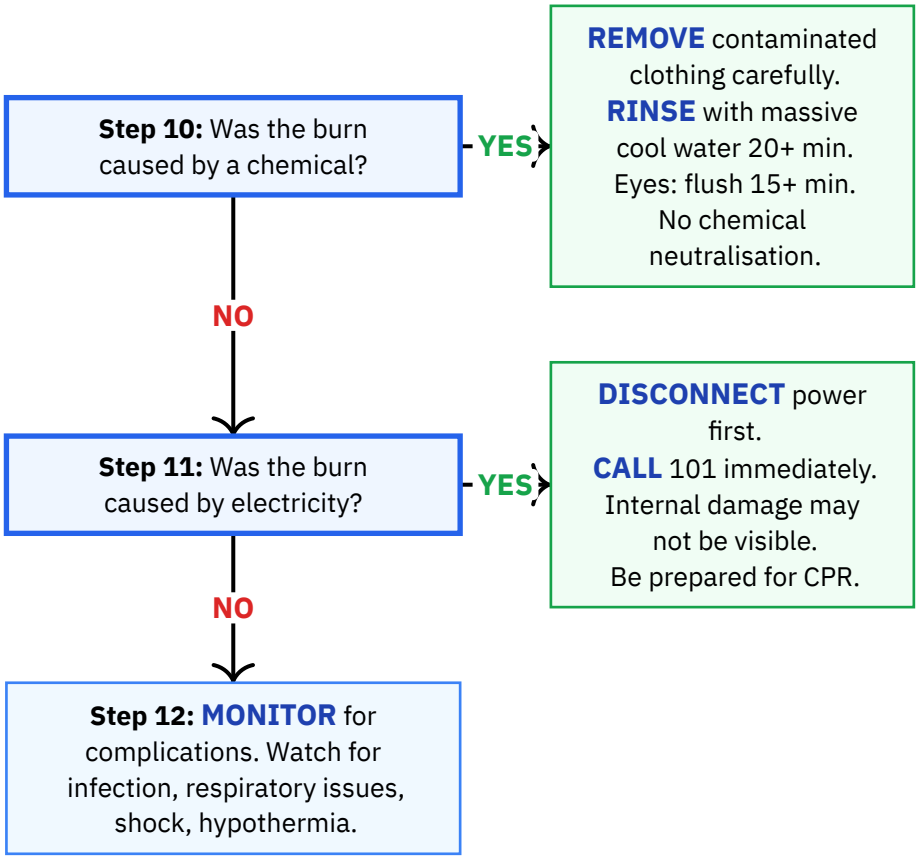
Initial Response (Steps 1–4)



Assessment and Treatment (Steps 5–9)



Chemical and Electrical Burns (Steps 10–12)



DO NOT:

X Do NOT apply ice or ice-cold water -- extreme cold causes additional tissue damage.
X Do NOT apply toothpaste, coffee grounds, raw eggs, yogurt, or butter.
X Do NOT puncture or pop blisters -- fluid protects tissue.
X Do NOT apply ointments, oils, or powders to serious burns before medical evaluation.
X Do NOT use gauze pads or cotton wool directly on burns -- fibres adhere.
X Do NOT leave the child unattended if unconscious.
X Do NOT remove clothing stuck to the burn -- cut around it.
X Do NOT immerse the whole child in cold water -- cool the burn, not the child.
X Do NOT attempt chemical neutralisation on chemical burns.

Equipment needed:

— Cool (lukewarm) running water source
— Sterile non-adherent dressings or clean dry sheet
— Scissors (to cut clothing around stuck areas)
— Blanket or towel (warmth during cooling)
— Age-appropriate pain relief (ibuprofen or paracetamol, dosed by weight)
— Drinking water (for hydration)

Source: Magen David Adom (MDA) — MDA Burns First Aid Guidelines
URL: https://www.mdais.org/101/burns
Publication date: 2026-01-01 · **Imported:** 2026-03-15 · **Last verified:** 2026-03-15
Notes: Key child difference: minimum 20 minutes cooling (vs 10 for adults). Cool the burn, not the whole child. All burns in infants under 1 year require immediate medical attention.

Choking (Foreign Body Airway Obstruction) — Child/ Infant

Israel — CHILD

CALL 101 (MDA) — COMPLETE OBSTRUCTION

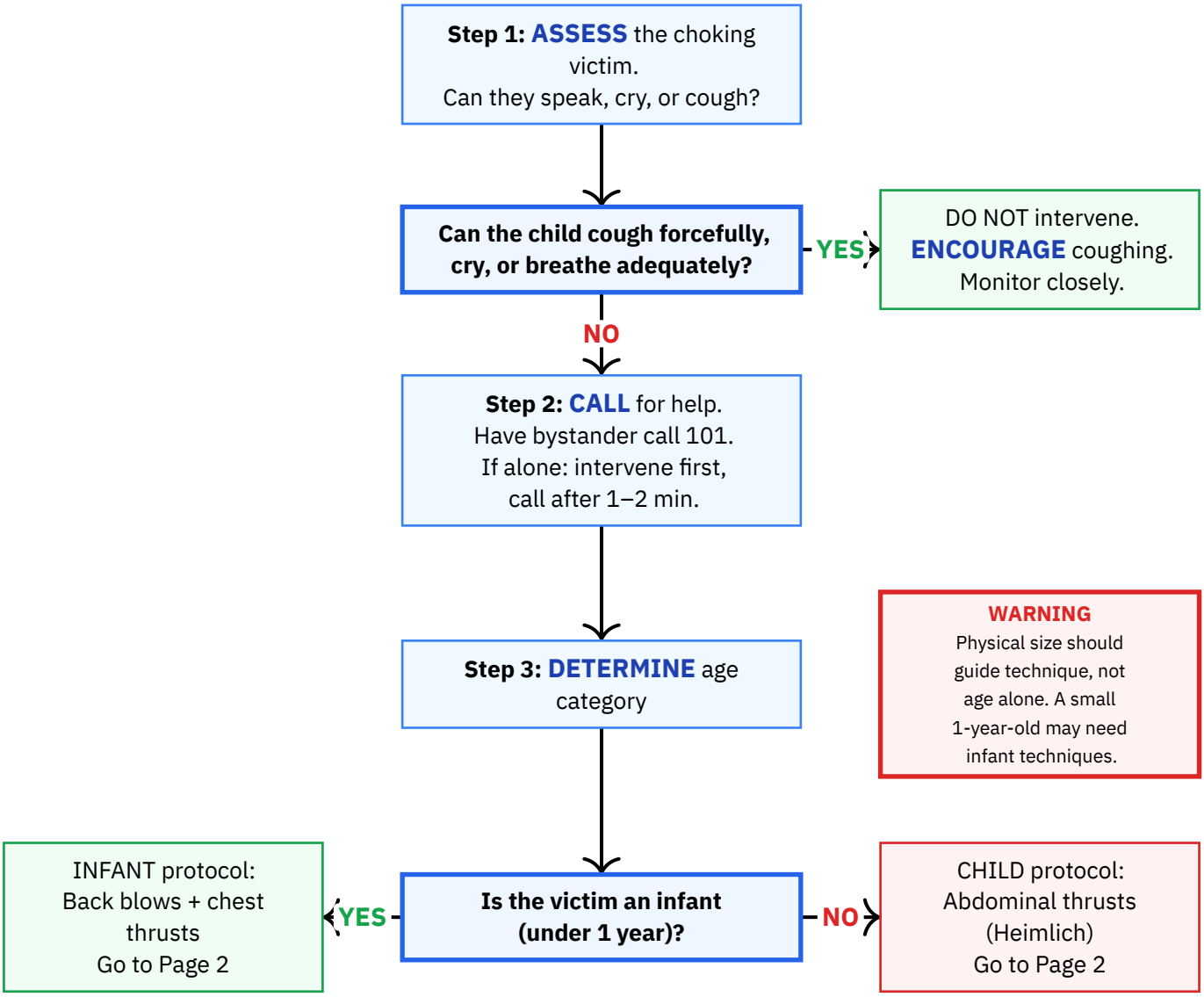
MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

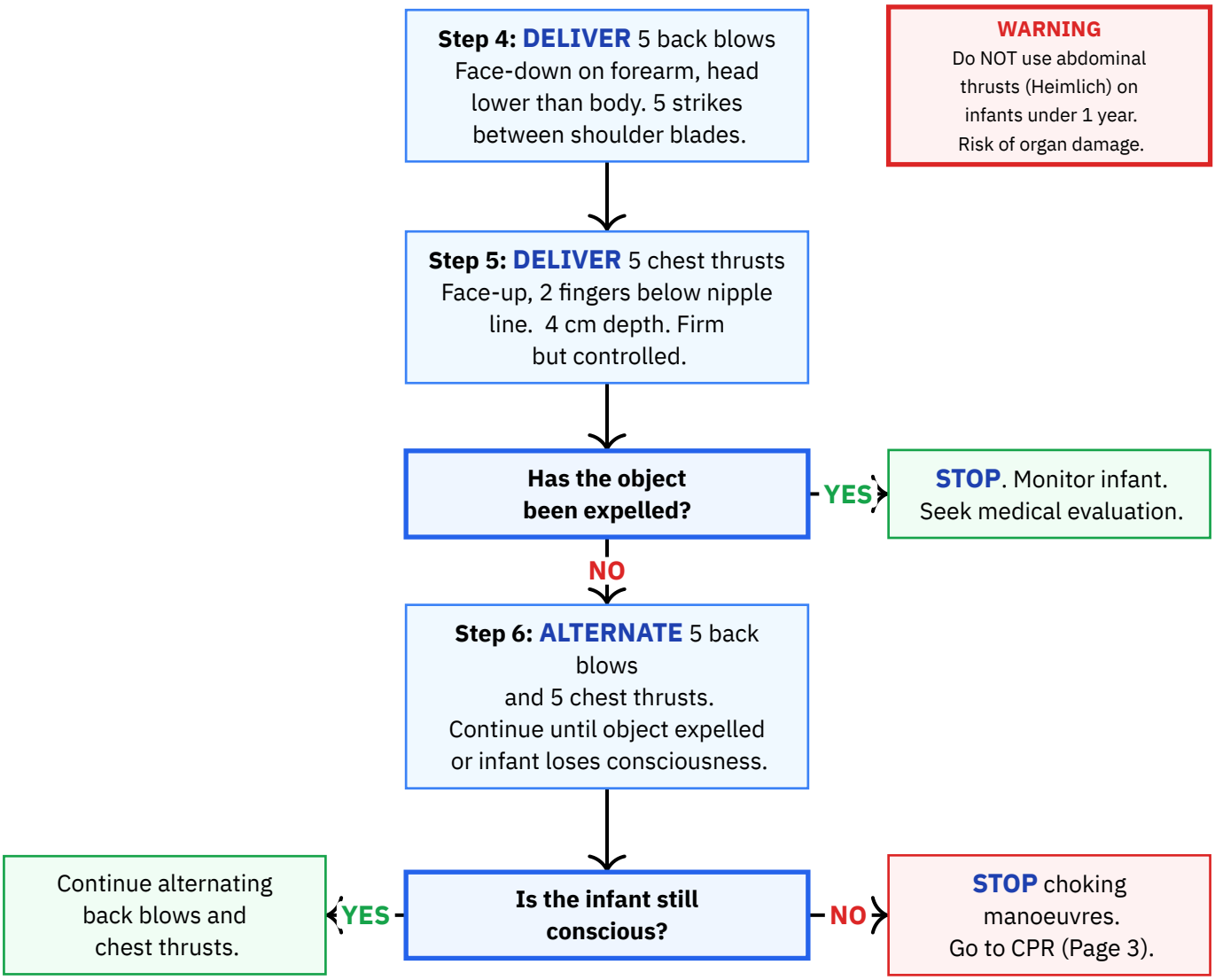
Child or infant is choking: inability to speak, cry, or make sounds; weak or absent cough; difficulty breathing; cyanosis (bluish skin); high-pitched wheezing. Note: young children may NOT grasp their throats.

Summary: Back blows and chest thrusts for infants (under 1 year); abdominal thrusts (Heimlich) for children (1–8 years); transition to CPR if victim becomes unresponsive.

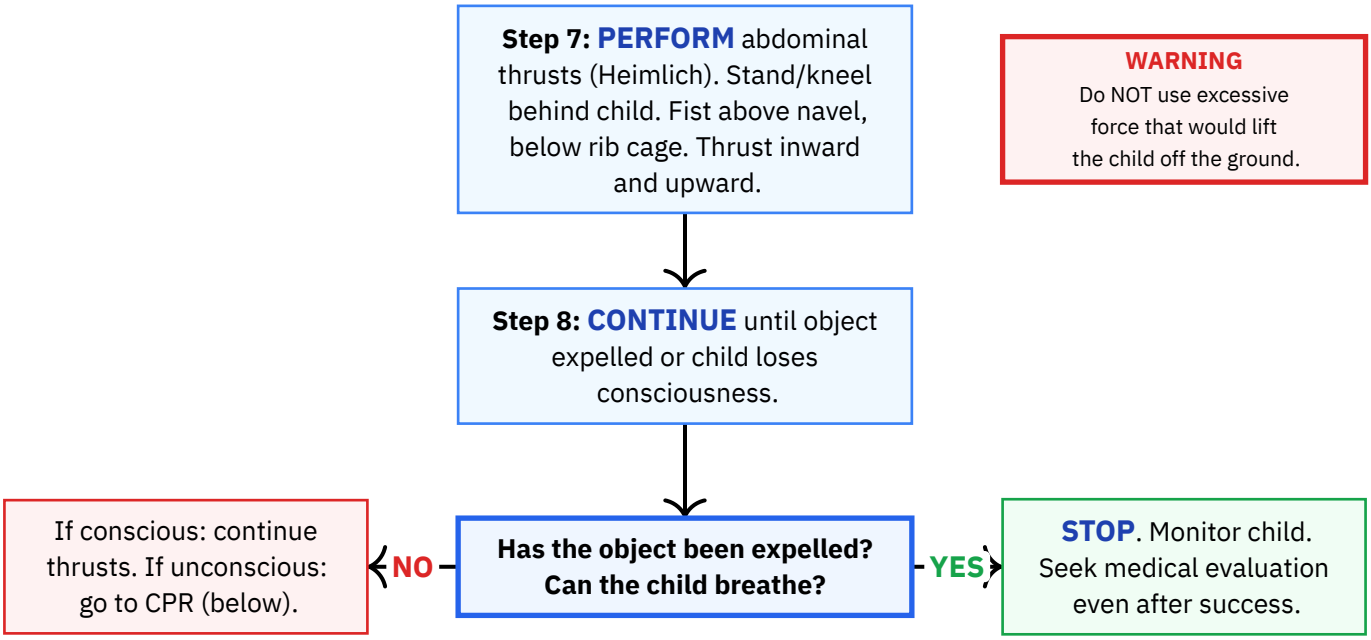
Assessment and Age Determination



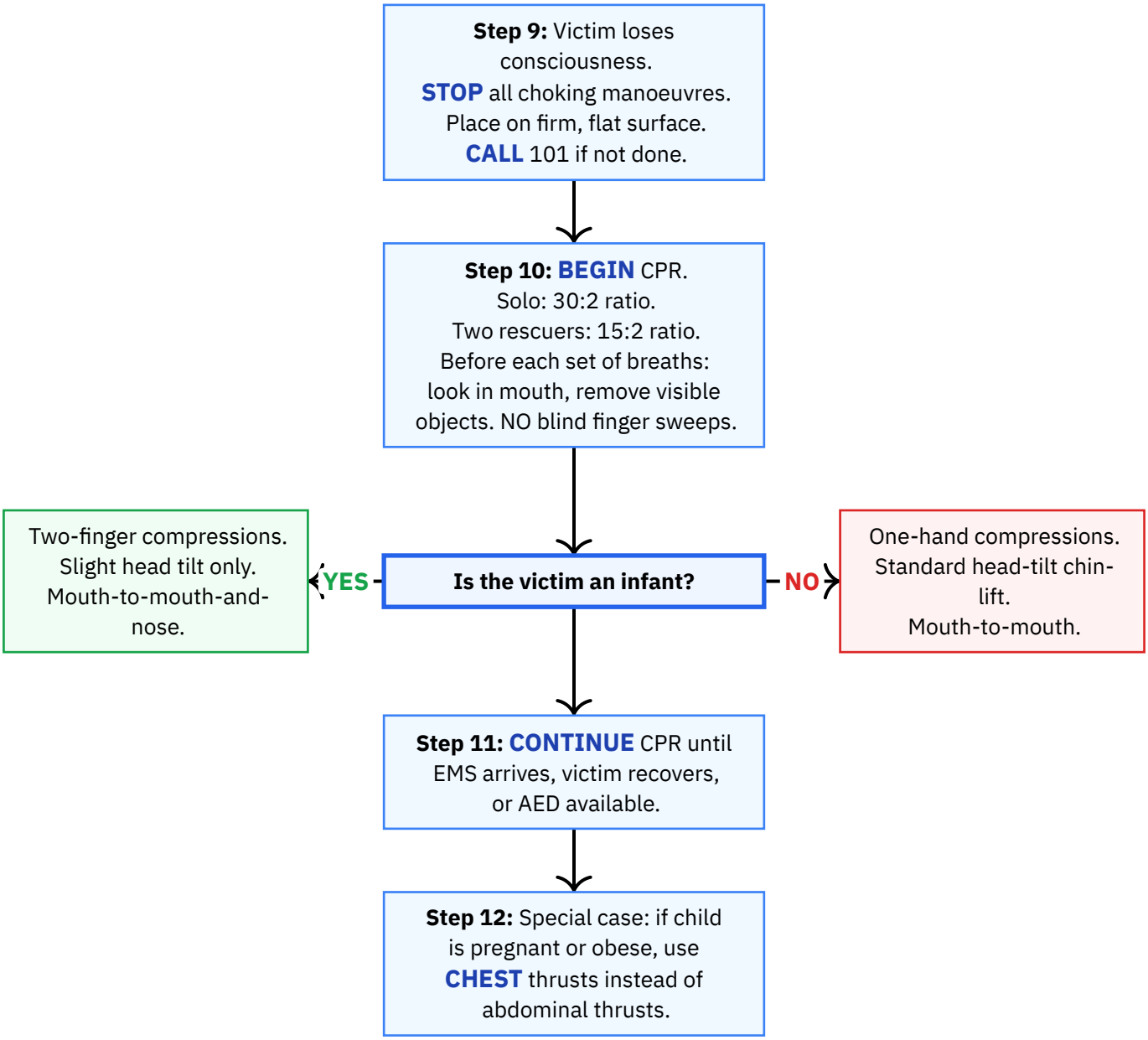
Infant Protocol (Under 1 Year)



Child Protocol (1–8 Years)



CPR Transition (Unconscious Victim)



DO NOT:

X DO NOT perform abdominal thrusts on infants under 1 year -- use only back blows and chest thrusts.
X DO NOT insert fingers blindly into the mouth (blind finger sweeps) -- only remove objects you can clearly see.
X DO NOT force anything into the mouth -- risk of pushing object deeper.
X DO NOT attempt mouth manipulation if teeth are clenched.
X DO NOT give food or fluids to a choking victim.
X DO NOT intervene if the child is coughing effectively -- allow the body to clear the obstruction.
X DO NOT use this protocol for choking caused by asthma, infection, or head injury.
X DO NOT use excessive force during abdominal thrusts that would lift the child off the ground.
X DO NOT fully extend an infant's head when opening the airway.

Equipment needed:

— AED with paediatric pads (if available, for CPR transition)

Source: Magen David Adom (MDA) — MDA 101 Public First Aid Guidance Series
URL: https://www.mdais.org/101/asphyxia
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: Unified from MDA and United Hatzalah sources. Infant (0–1 year), child (1–8 years). Physical size should guide technique selection, not age alone.

CPR (Cardiopulmonary Resuscitation) — Child/Infant

Israel — CHILD

CALL 101 (MDA) — IF ALONE, DO 2 MIN CPR FIRST

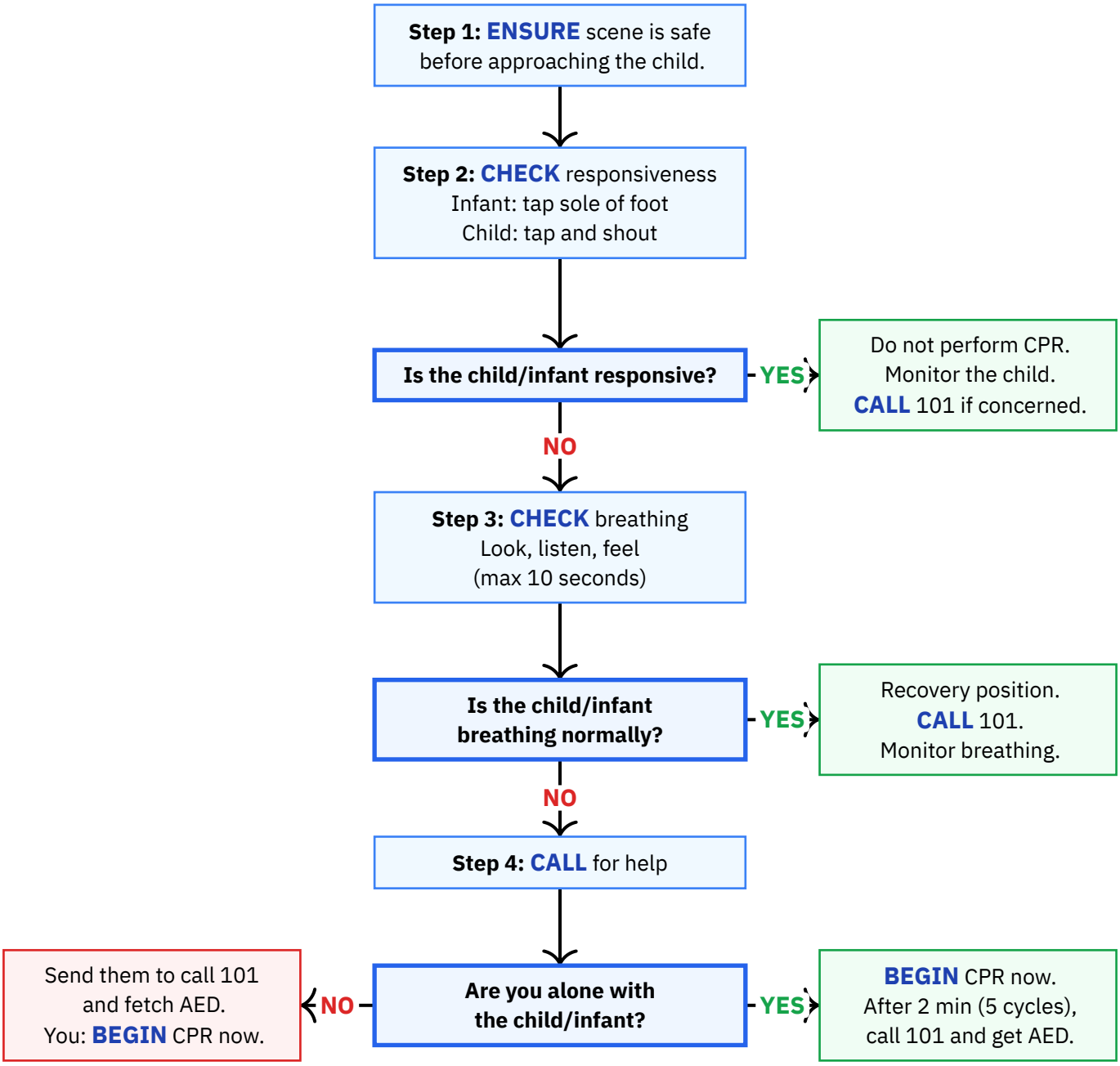
MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

Child (1 year to puberty) or infant (1 month to 1 year) is unresponsive and not breathing normally. In children, cardiac arrest is almost always secondary to respiratory failure – rescue breaths are critically important.

Summary: CPR protocol for unresponsive children/infants not breathing normally, following MDA guidelines aligned with ILCOR consensus. If alone, perform 2 minutes of CPR before calling 101.

Assessment Phase



CPR Technique (C-A-B)

Step 5: BEGIN chest compressions
Place child on firm, flat surface.
Rate: 100–120/min.
Allow full chest recoil.

Is the patient an infant (under 1 year)?

YES → Solo: 2-finger technique
Two rescuers: two-thumb encircling hands.
Depth: 4 cm (1/3 chest).

NO → Heel of one or two hands (size-dependent).
Depth: 5 cm (1/3 chest).

Step 6: OPEN airway

WARNING
Do NOT hyperextend an infant's neck.
Use neutral/sniffing position only.

Is the patient an infant (under 1 year)?

YES → Neutral/sniffing position.
Gentle chin lift only.

NO → Head-tilt chin-lift with mild extension.

Step 7: DELIVER 2 rescue breaths
1 second each, watch for chest rise.
Rescue breaths are ESSENTIAL in paediatric CPR.

WARNING
Compression-only CPR is NOT adequate for children/infants.
Arrest is almost always respiratory in origin.

Is the patient an infant?

YES → Seal mouth over infant's mouth AND nose.
Gentle puffs from cheeks.

NO → Pinch nose, seal mouth.
Breathe until chest rise.

CPR Cycles and Continuation

Step 8: CHECK chest rise
If no rise, reposition airway and reattempt. Consider foreign body obstruction.

↓

Does the chest rise with rescue breaths?

- **NO** → Reposition head/airway. Reattempt. If still no rise: suspect foreign body.
- **YES** → Continue CPR cycles.

↓

Step 9: CONTINUE CPR cycles

↓

Are there two or more rescuers?

- **NO** → Use 30:2 ratio.
- **YES** → Use 15:2 ratio. Infants: switch to two-thumb technique.

↓

Step 10: APPLY AED when available. Use paediatric pads if available. If only adult pads, ensure no overlap.

WARNING
If adult pads overlap on small child/infant: use anteroposterior placement (front/back).

↓

Step 11: After AED shock, **RESUME** CPR immediately. 5 cycles before reassessing.

↓

Step 12: CONTINUE CPR until: MDA arrives, child starts breathing/responding, or you are physically unable.

↓

Has the child started breathing and responding?

- **NO** → **CONTINUE** CPR without interruption.
- **YES** → **STOP** CPR. Recovery position. Monitor until MDA arrives.

Reference

Key Paediatric CPR Parameters

Compression rate: 100–120 / min
Infant depth: 4 cm (1/3 chest)
Two-rescuer ratio: 15 : 2
Lone rescuer: 2 min CPR, then call 101
Child depth: 5 cm (1/3 chest)
Solo ratio: 30 : 2
Breath duration: 1 second each
Emergency number: 101 (MDA)

DO NOT:

X Do NOT hyperextend an infant's neck when opening the airway -- use neutral/sniffing position only.
X Do NOT insert fingers into the infant's or child's mouth (risk of pushing objects deeper).
X Do NOT deliver deep lung breaths to infants -- use gentle puffs from your cheeks.
X Do NOT use excessive volume or pressure when delivering rescue breaths (risk of gastric distension).
X Do NOT skip rescue breaths in paediatric CPR -- compression-only CPR is NOT adequate for children and infants.
X Do NOT delay CPR to call for help when alone -- perform 2 minutes of CPR first, then call 101.
X Do NOT allow AED pads to touch or overlap each other on a small child or infant.
X Do NOT stop CPR to check for pulse unless there are clear signs of recovery (breathing, movement).
X Do NOT compress over the xiphoid process (the very bottom tip of the sternum).

Equipment needed:

— AED with paediatric pads or paediatric dose attenuator (if available)
— Barrier device or pocket mask (optional, for rescue breaths)
— Bag-valve-mask -- infant size (250-350 cc) or paediatric size (if available)

Source: Magen David Adom (MDA) — BLS Guide 2016 / ALS Professional Handbook 2024
URL: https://www.mdais.org/kursim/cpr-children
Publication date: 2024-04-01 · **Edition:** BLS Guide 2016 / ALS Professional Handbook 2024
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: Unified from MDA and United Hatzalah sources. Both aligned on C-A-B sequence, compression depths, ratios, and AED guidance. If alone with the child, perform 2 minutes of CPR before calling 101.

Dehydration First Aid -- Child

Israel — CHILD

CALL 101 (MDA) FOR SEVERE DEHYDRATION

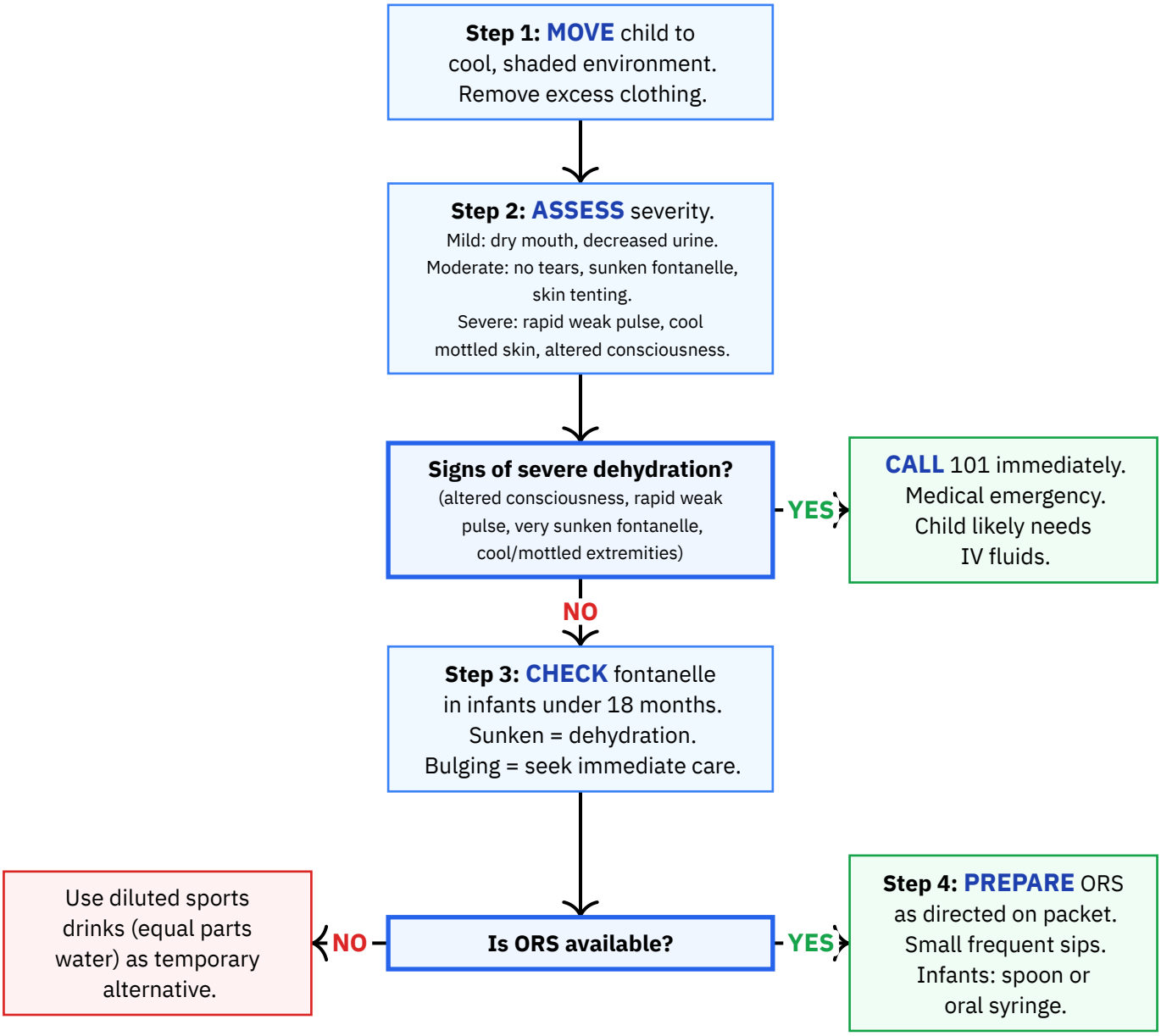
MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

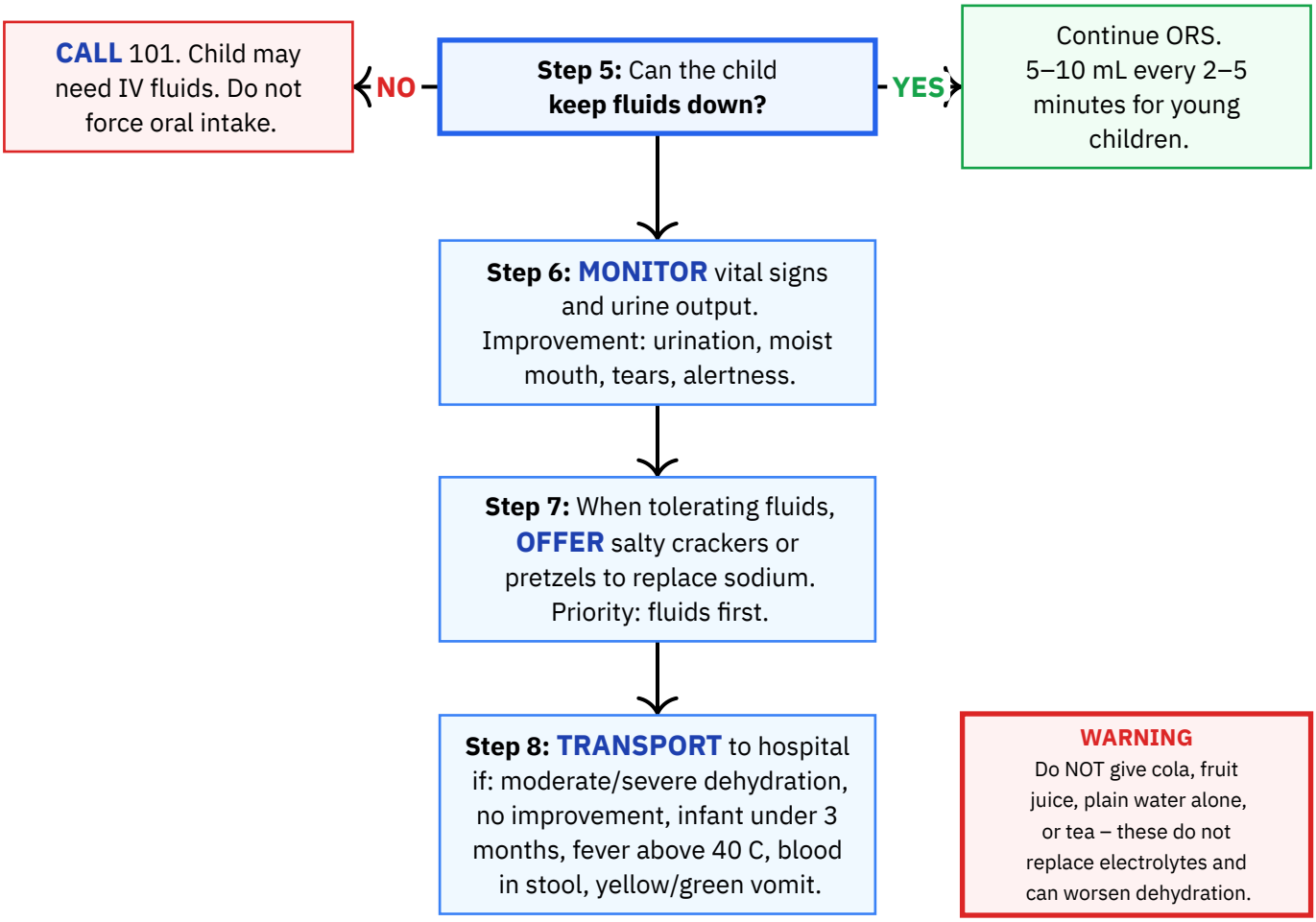
When a child shows signs of dehydration from any cause: insufficient fluid intake, excessive sweating, vomiting, diarrhoea, or fever. Particularly relevant during Israeli summer (May–October), sharav heat waves, school field trips in desert areas (Negev, Arava, Dead Sea), and during childhood gastroenteritis.

Summary: Move to cool environment. Assess severity. Check fontanelle in infants. Oral rehydration with ORS (not cola, juice, or plain water). Small frequent sips. Hospital for severe cases, all infants under 3 months, or persistent vomiting.

Assessment and Rehydration – Steps 1-5



Fluid Tolerance and Transport – Steps 5-8



DO NOT:

X Do NOT give cola, fruit juice, plain water alone, or tea -- do not replace electrolytes adequately.
X Do NOT use homemade salt/sugar solutions unless ORS is completely unavailable.
X Do NOT force large volumes of fluid at once -- small frequent sips are more effective.
X Do NOT delay calling 101 for an infant under 3 months with any signs of dehydration.
X Do NOT ignore a sunken fontanelle in an infant -- key dehydration indicator.
X Do NOT give anti-diarrhoea medications to children without medical direction.
X Do NOT stop breastfeeding -- continue breastfeeding alongside ORS.

Equipment needed:

— ORS (Oral Rehydration Solution) packets
— Spoon or oral syringe (for infant fluid administration)
— Phone (to call 101)
— Thermometer (to check for fever)

Source: Magen David Adom (MDA) – supplemented by Ichud Hatzalah hot climate injuries and Hatzalah Without Borders dehydration guide
URL: https://www.mdais.org/101/first-aid
Publication date: 2026-01-01
Imported: 2026-03-16 · **Last verified:** 2026-03-16
Notes: Unified from MDA and United Hatzalah sources. Israeli climate context: sharav heat waves, Negev field trips, summer months May–October.

Diabetic Emergency First Aid -- Child

Israel — CHILD

CALL 101 (MDA) — WHEN IN DOUBT, GIVE SUGAR

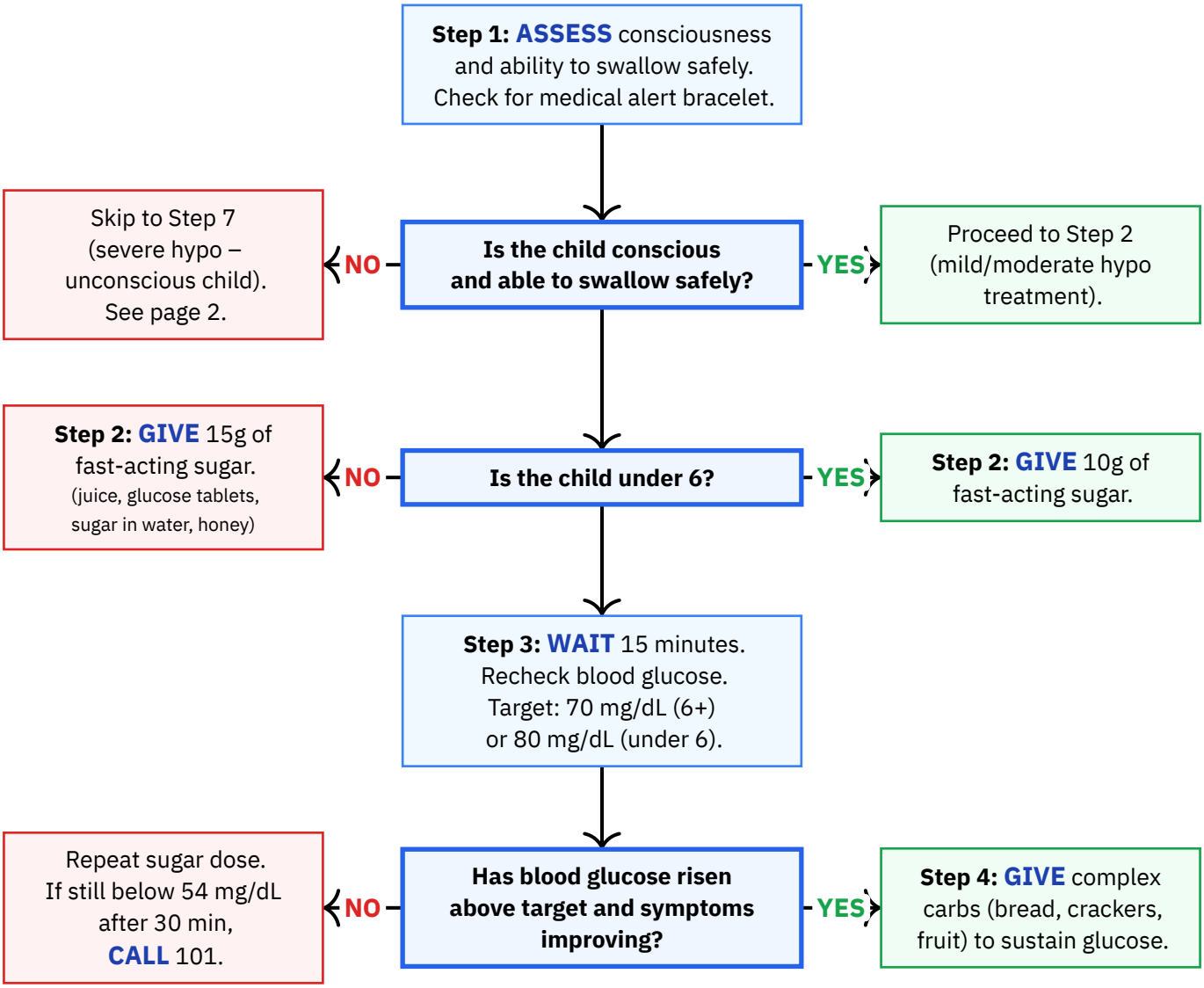
MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

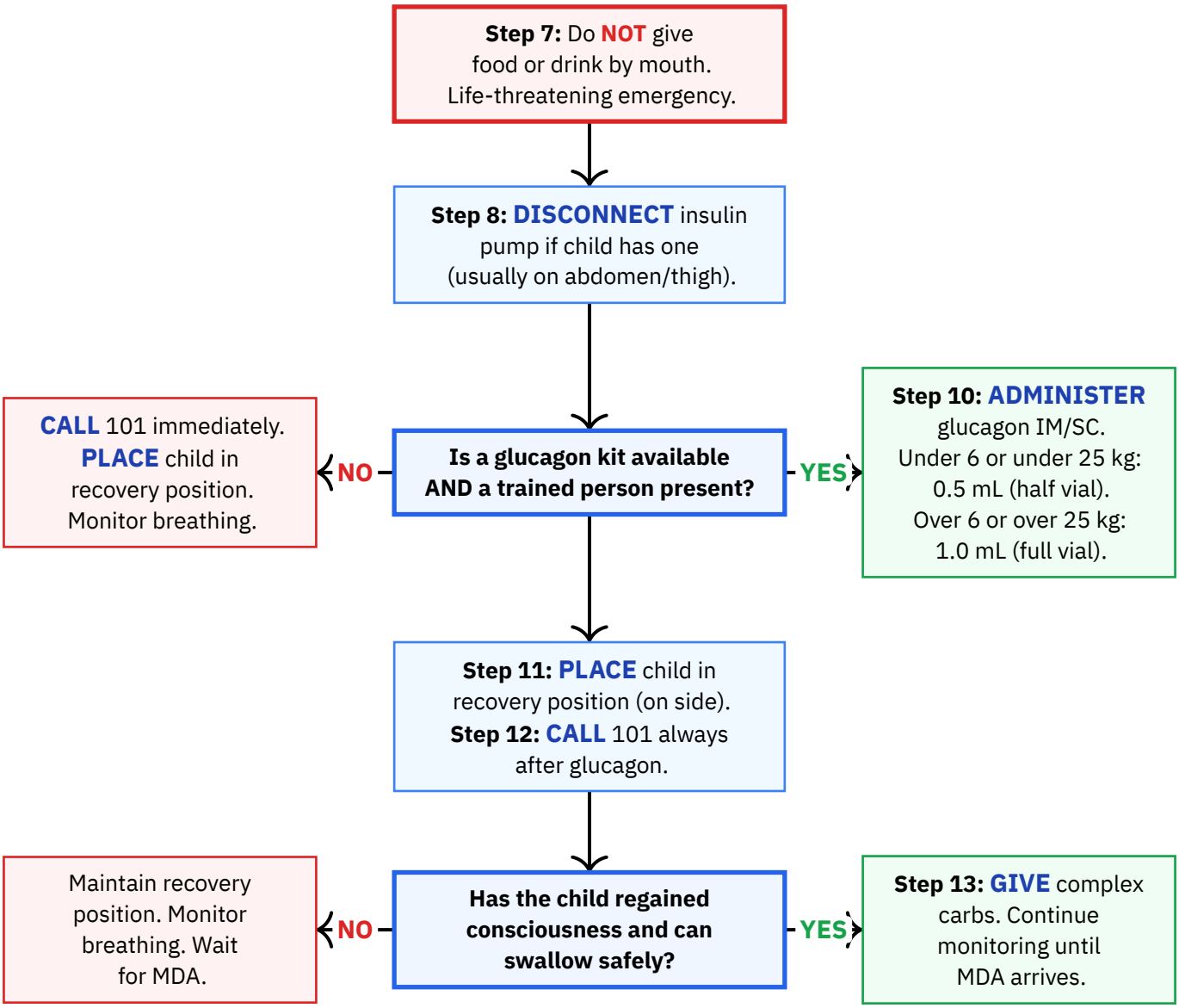
Child with known or suspected diabetes showing signs of hypoglycemia (paleness, sweating, trembling, rapid heartbeat, irritability, confusion, seizures) or hyperglycemia/DKA (excessive thirst, vomiting, rapid deep breathing, fruity breath, lethargy).

Key Principle: When in doubt whether hypoglycemia or hyperglycemia: **give sugar**. Sugar saves a hypoglycemic child's life. Sugar will not cause immediate harm to a hyperglycemic child.

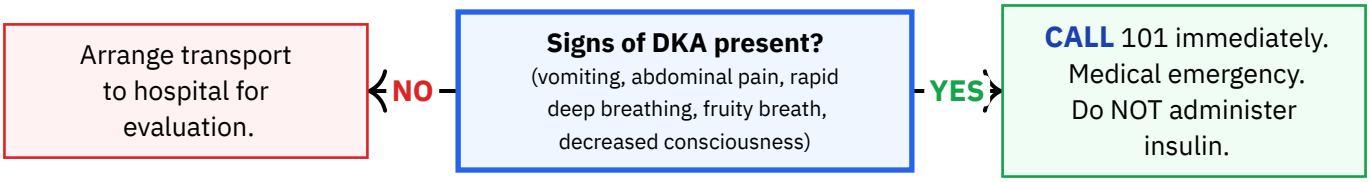
Assessment and Mild Hypoglycemia



Severe Hypoglycemia – Unconscious Child



DKA Assessment (Step 14)



Reference

Key Parameters – Rule of 15

Under 6: 10g sugar, target 80 mg/dL
Reassess interval: 15 minutes
Glucagon (under 6 / under 25 kg): 0.5 mL
Glucagon onset: 10–15 minutes

6 and over: 15g sugar, target 70 mg/dL
Call 101 if: below 54 mg/dL after 30 min
Glucagon (6+ / over 25 kg): 1.0 mL
Glucagon duration: 30 minutes

Step 15: When in Doubt – GIVE SUGAR

Giving sugar to a hypoglycemic child is **life-saving**.
Giving sugar to a hyperglycemic child will **not cause immediate harm**.
Withholding sugar from a hypoglycemic child **can be fatal**.

Only if conscious and can swallow safely. Never give anything by mouth to an unconscious child.

DO NOT:

X DO NOT administer insulin -- strictly a medical intervention, can be fatal if given inappropriately.
X DO NOT give food, drink, or oral medication to an unconscious or seizing child -- choking hazard.
X DO NOT use chocolate as initial sugar source -- fat/protein slows absorption.
X DO NOT administer glucagon unless trained and authorised.
X DO NOT delay calling 101 for severe hypoglycemia, seizures, or loss of consciousness.
X DO NOT attempt to manage hyperglycemia/DKA with first aid -- requires medical intervention.
X DO NOT use diet drinks or sugar-free products as a sugar source.

Equipment needed:

— Glucose gel syringe (15g glucose)
— Fast-acting sugar source (juice, glucose tablets, sugar cubes, honey)
— Complex carbohydrate snack (bread, crackers, fruit)
— Glucagon emergency kit (Hypo-Kit) if available and authorised to use
— Blood glucose meter (glucometer) if available

Source: Magen David Adom (MDA) – How to Treat a Diabetes Patient, corroborated by Shamir Medical Center paediatric endocrinology guidance
URL: https://www.mdais.org/101/diabetes
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: Unified from MDA and United Hatzalah sources. Child-specific dosing (Rule of 15 with 10g modification for under-6) sourced from MDA-aligned Israeli hospital guidance.

Drowning — Child

Israel — CHILD

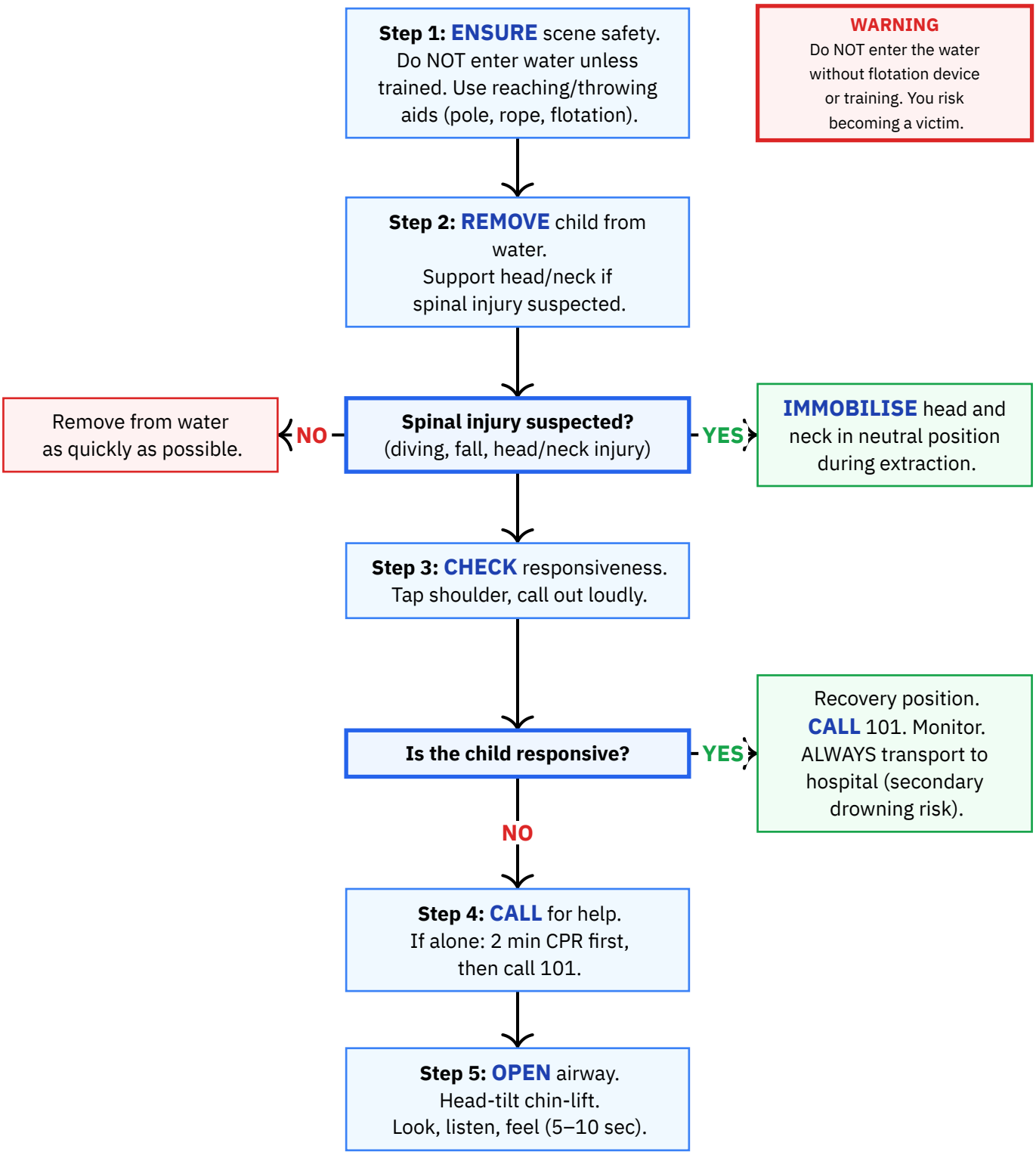
CALL 101 — RESCUE BREATHS ARE ESSENTIAL

MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

A child has been submerged or immersed in water and is unresponsive, not breathing, or breathing inadequately. Drowning is the 2nd leading cause of unintentional death in children under 18 in Israel; 83% of drownings in children under 10 occur in swimming pools.

Rescue and Assessment (Steps 1–5)



CPR and Recovery (Steps 6–10)

Step 6: GIVE 2 initial rescue breaths.
Pinch nose, seal mouth.
Watch for chest rise.

WARNING
Compression-only CPR is INSUFFICIENT for drowning. Rescue breaths are ESSENTIAL – drowning is a hypoxic emergency.

Does the chest rise?

- **NO** → Reposition head. Retry. If still no rise: proceed to compressions.
- **YES** → Effective. Proceed to compressions.

Step 7: BEGIN chest compressions.
Heel of one hand, centre of chest. Depth: 1/3 to 1/2 chest.
Rate: 100–120/min. Full recoil.

Step 8: CONTINUE CPR cycles.
Solo: 30:2. Two rescuers: 15:2.
Switch every 2 min if two rescuers. Minimise pauses.

Does the child show signs of life (breathing, coughing)?

- **NO** → **CONTINUE** CPR until MDA arrives, AED available, or physically unable.
- **YES** → **STOP** compressions. Recovery position (Step 9). Monitor until MDA arrives.

Step 9: Recovery position: on side, upper knee bent.
Maintain warmth – hypothermia risk high.

WARNING
ALWAYS transport to hospital, even if child appears to recover. Secondary drowning can develop hours later.

Step 10: Hospital transport is MANDATORY. Secondary drowning and electrolyte imbalances can occur hours later.

DO NOT:

X DO NOT attempt to remove water from the lungs -- impossible and dangerous.
X DO NOT perform abdominal thrusts (Heimlich) to expel water -- not effective, risks aspiration.
X DO NOT delay CPR to attempt water removal.
X DO NOT use compression-only CPR -- rescue breaths are essential for drowning.
X DO NOT assume the child is fine if they regain consciousness -- always transport to hospital.
X DO NOT enter the water unless you are a trained rescuer with flotation device.
X DO NOT use excessive ventilation force -- risk of gastric distention in children.

Equipment needed:

— Reaching aid (pole, rope, towel) or flotation device
— Firm, flat surface for CPR
— Blanket or covering (warmth, hypothermia prevention)

Source: Magen David Adom (MDA) — MDA 101 Public First Aid Guidance; MDA 2024 drowning statistics
URL: https://www.mdais.org/101/first-aid
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: Drowning is the 2nd leading cause of unintentional death in children under 18 in Israel (after traffic accidents). 83% of drownings in children under 10 occur in swimming pools.

Electrocution — Child

Israel — CHILD

DO NOT TOUCH CHILD WHILE CONNECTED TO SOURCE — CALL 101

MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Electric Co:** 103

When to apply:

Child has sustained electric shock from any source: household outlet (220V in Israel), electrical cord (oral contact/biting in toddlers), faulty appliance, exposed wiring, or high-voltage source. Signs: burns at contact points, loss of consciousness, muscle rigidity, difficulty breathing, cardiac arrest.

Scene Safety and Separation (Steps 1–3)

WARNING
Touching the child while connected causes chain electrocution. You will become a victim.

Step 1: STOP – do NOT touch the child while connected to electrical source.

↓

Step 2: DISCONNECT the source.

↓

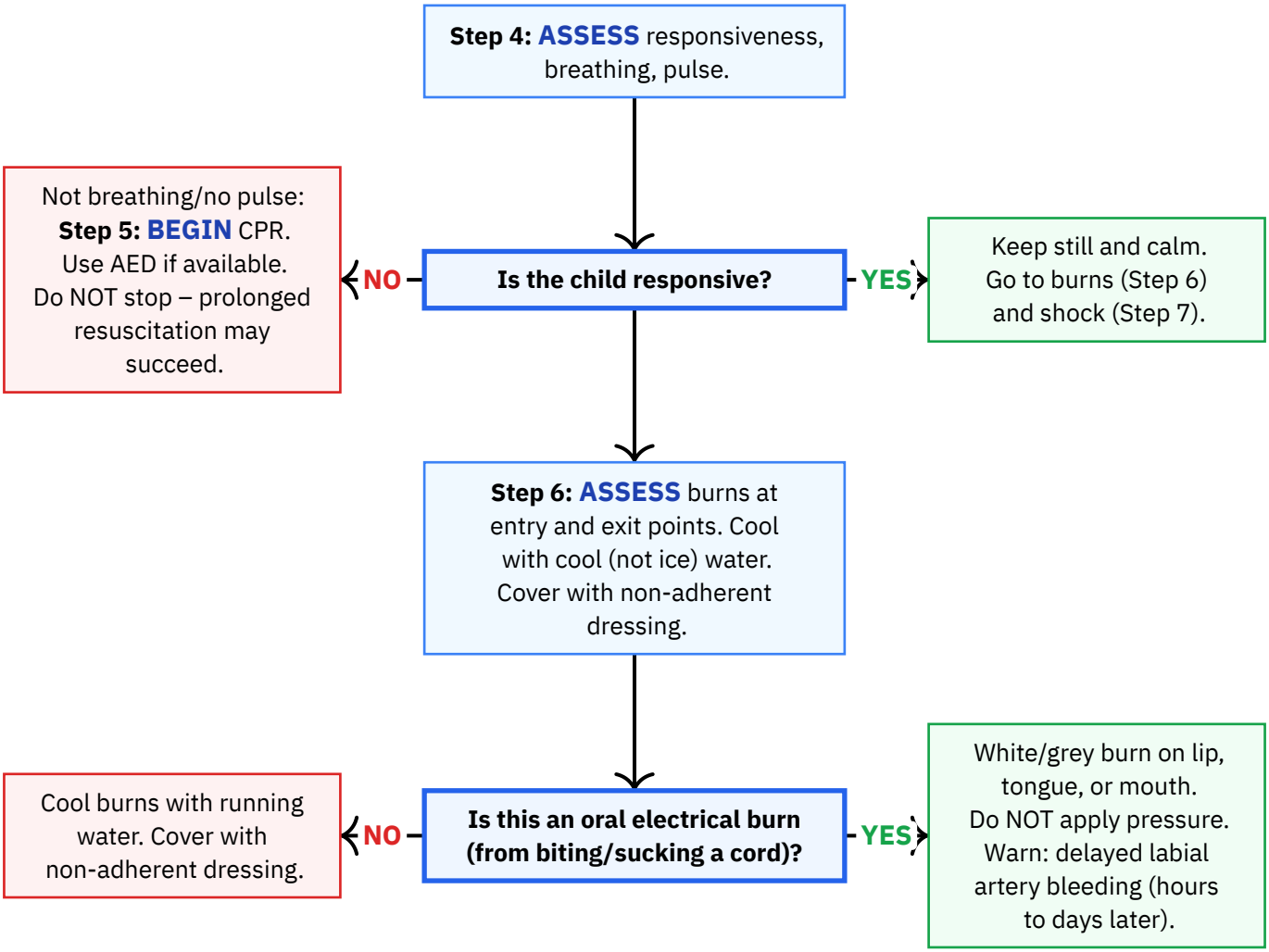
Is this a high-voltage source (power lines, transformer)?

- **NO** → Turn off circuit breaker or unplug appliance. If not possible: use dry non-conductive object (wooden broom, plastic chair).
- **YES** → Do NOT approach. **CALL** 101, 102, 103. Maintain safe distance. Wait for emergency services.

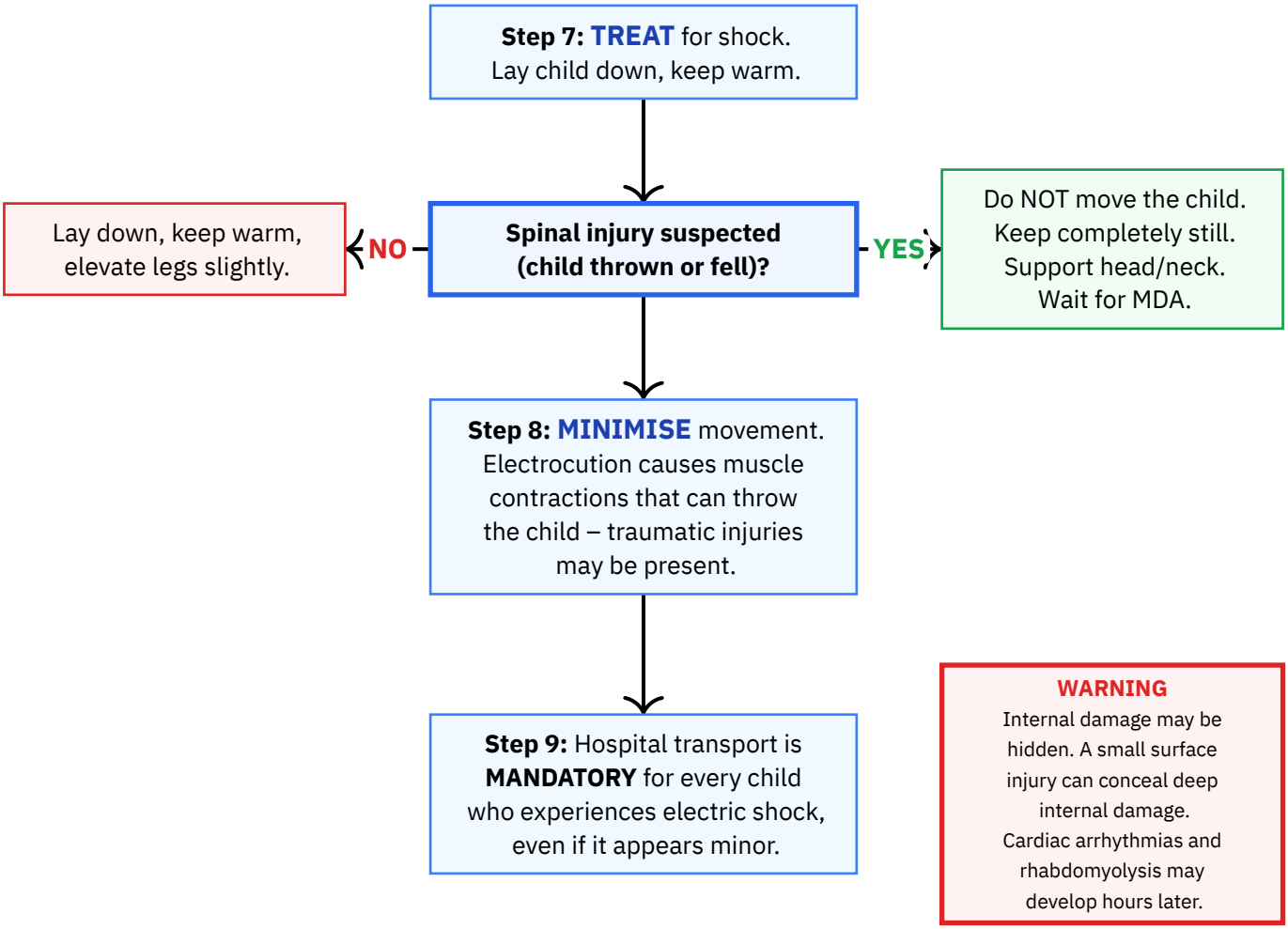
↓

Step 3: CALL 101 immediately. Provide: phone number, address, child's age, what happened, current condition.

Assessment and Treatment (Steps 4–6)



Shock and Transport (Steps 7–9)



DO NOT:

- X Do NOT touch the child while connected to the electrical source -- chain electrocution.
- X Do NOT use metal objects or wet/damp materials to separate from source.
- X Do NOT apply ice to electrical burns.
- X Do NOT apply creams, ointments, toothpaste, or home remedies to electrical burns.
- X Do NOT assume the child is fine because external injuries appear minor.
- X Do NOT delay calling 101 -- every electrocution requires hospital assessment.
- X Do NOT move the child unnecessarily -- spinal/traumatic injuries may be present.
- X Do NOT approach high-voltage sources -- call 101, 102, and 103.
- X Do NOT stop CPR based on apparent lack of vital signs -- prolonged resuscitation may succeed.
- X Do NOT give food or drink to a child in shock after electrocution.

Equipment needed:

— Non-conductive object (wooden broom handle, dry stick, plastic chair)
— AED / defibrillator (with paediatric pads if available)
— Cool running water (for burn cooling)
— Sterile non-adherent dressings (for burns)
— Blanket (for warmth and shock treatment)

Source: Magen David Adom (MDA) — MDA Electrocution First Aid Guidelines
URL: https://www.mdais.org/101/electrocution
Publication date: 2026-01-01 · **Imported:** 2026-03-15 · **Last verified:** 2026-03-15
Notes: Israeli household supply is 220V. Key child concerns: oral burns from cord-biting (under 6), delayed labial artery bleeding, lower body resistance increasing severity. Hospital evaluation mandatory for all cases.

Eye Injuries First Aid -- Child

Israel — CHILD

CALL 101 (MDA) — DO NOT RUB THE EYE

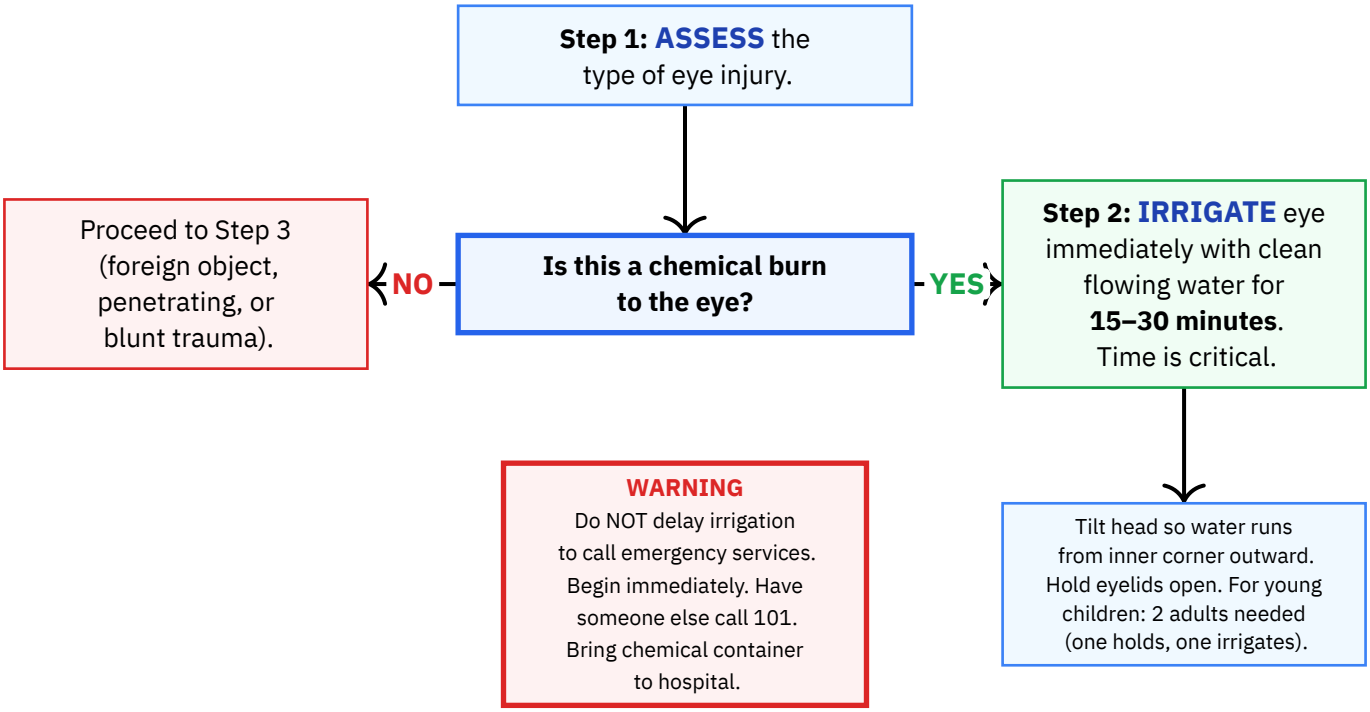
MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

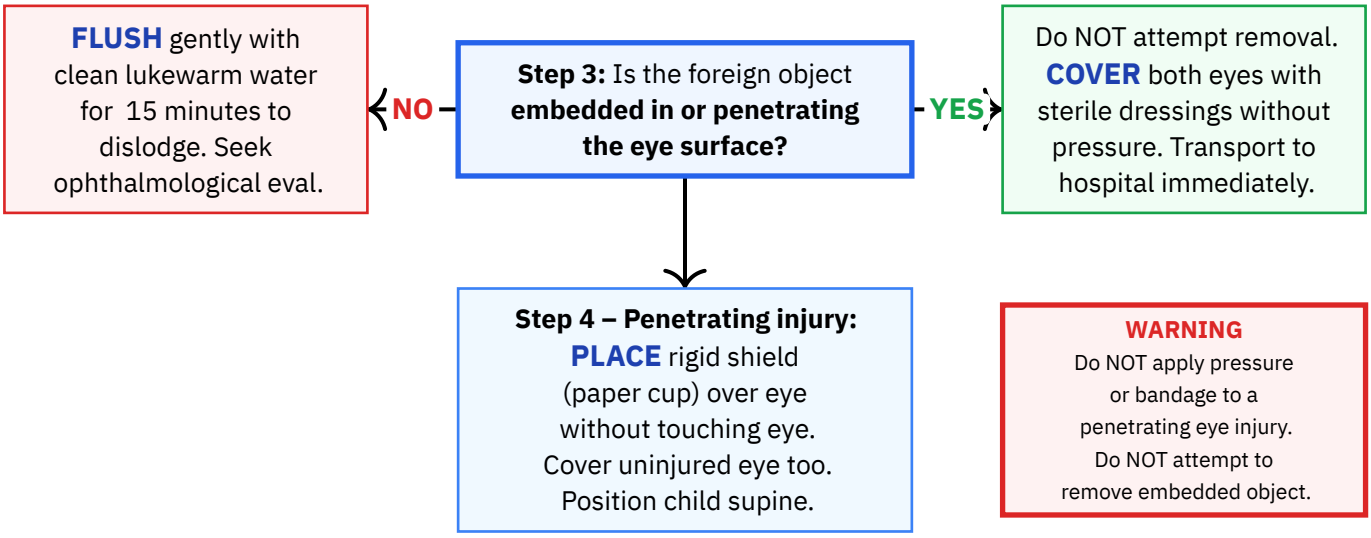
When a child has sustained any eye injury: chemical splash, foreign object, penetrating injury, blunt trauma, or thermal/UV burn. Common causes in Israeli children: playground injuries (sand, sticks), household chemicals (bleach, cleaners), toy-related injuries.

Summary: Chemical burns: irrigate immediately for 15–30 minutes. Foreign objects: flush if superficial, do NOT remove if embedded. Penetrating injuries: rigid shield, no pressure. Blunt trauma: cold compress gently.

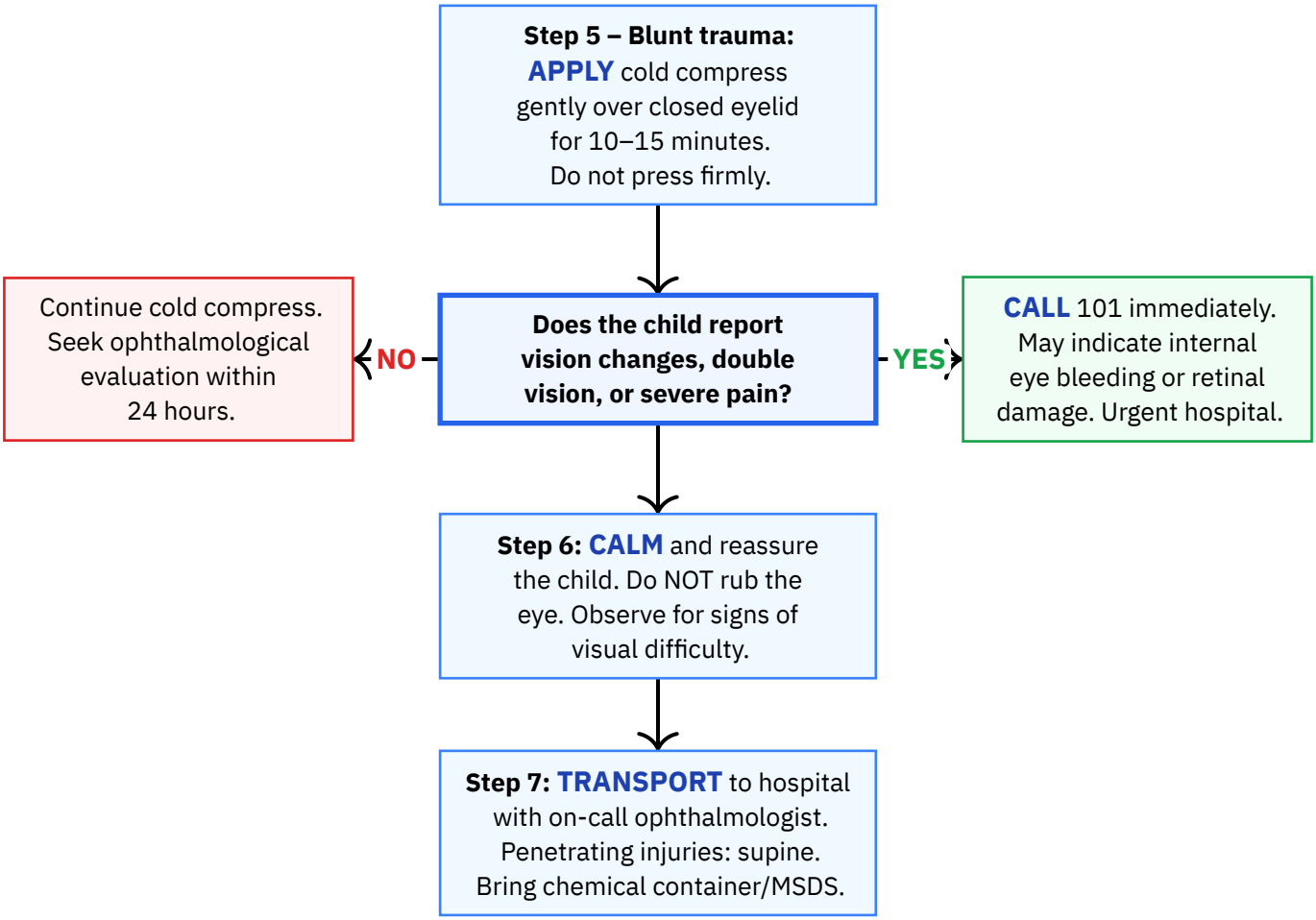
Assessment and Chemical Burns – Steps 1-2



Foreign Objects and Penetrating Injuries – Steps 3-4



Blunt Trauma and Care – Steps 5-7



DO NOT:

X Do NOT rub the injured eye -- worsens damage from foreign objects and chemical burns.
X Do NOT apply pressure or bandages to a penetrating eye injury.
X Do NOT attempt to remove an embedded object from the eye.
X Do NOT delay irrigation for chemical burns -- begin immediately.
X Do NOT use any eye drops or ointments without medical direction.
X Do NOT allow the child to rub or touch the injured eye.
X Do NOT apply antiseptic or alcohol to the eye.

Equipment needed:

— Clean water source (tap water or sterile water for irrigation)
— Rigid eye shield or paper cup
— Sterile gauze or clean cloth
— Cold compress or ice wrapped in cloth
— Phone (to call 101)

Source: Magen David Adom (MDA) – supplemented by Hatzalah Without Borders eye injury protocols (November 2021)
URL: https://www.mdais.org/101/first-aid
Publication date: 2026-01-01
Imported: 2026-03-16 · **Last verified:** 2026-03-16
Notes: Unified from MDA and United Hatzalah/Hatzalah Without Borders sources. Chemical burn irrigation duration 15–30 minutes per Hatzalah guidance.

Fractures and Sprains — Child

Israel — CHILD

CALL 101 FOR OPEN FRACTURES, SPINAL INJURY, OR SEVERE DEFORMITY

MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

Suspected fracture or sprain: severe pain worsening with movement, visible swelling/bruising, limb deformity, inability to move or bear weight, tenderness, audible snap at injury. In young children: sudden refusal to use arm/leg, inconsolable crying. Note: sprains in children may actually be growth plate fractures.

Assessment (Steps 1–4)

Step 1: **CALM** the child. Speak at eye level. Explain simply. Use distraction. Involve parent/caregiver.

↓

Step 2: **ASSESS** injury severity.

↓

Is bone visible through skin (open/compound fracture)?

- YES → Medical emergency. **CALL** 101 immediately. Go to Step 8 (open fracture management).

NO ↓

Step 3: **CALL** 101 if emergency criteria met (open fracture, spinal injury, severe deformity, loss of sensation/circulation).

↓

Suspected spinal or neck injury?

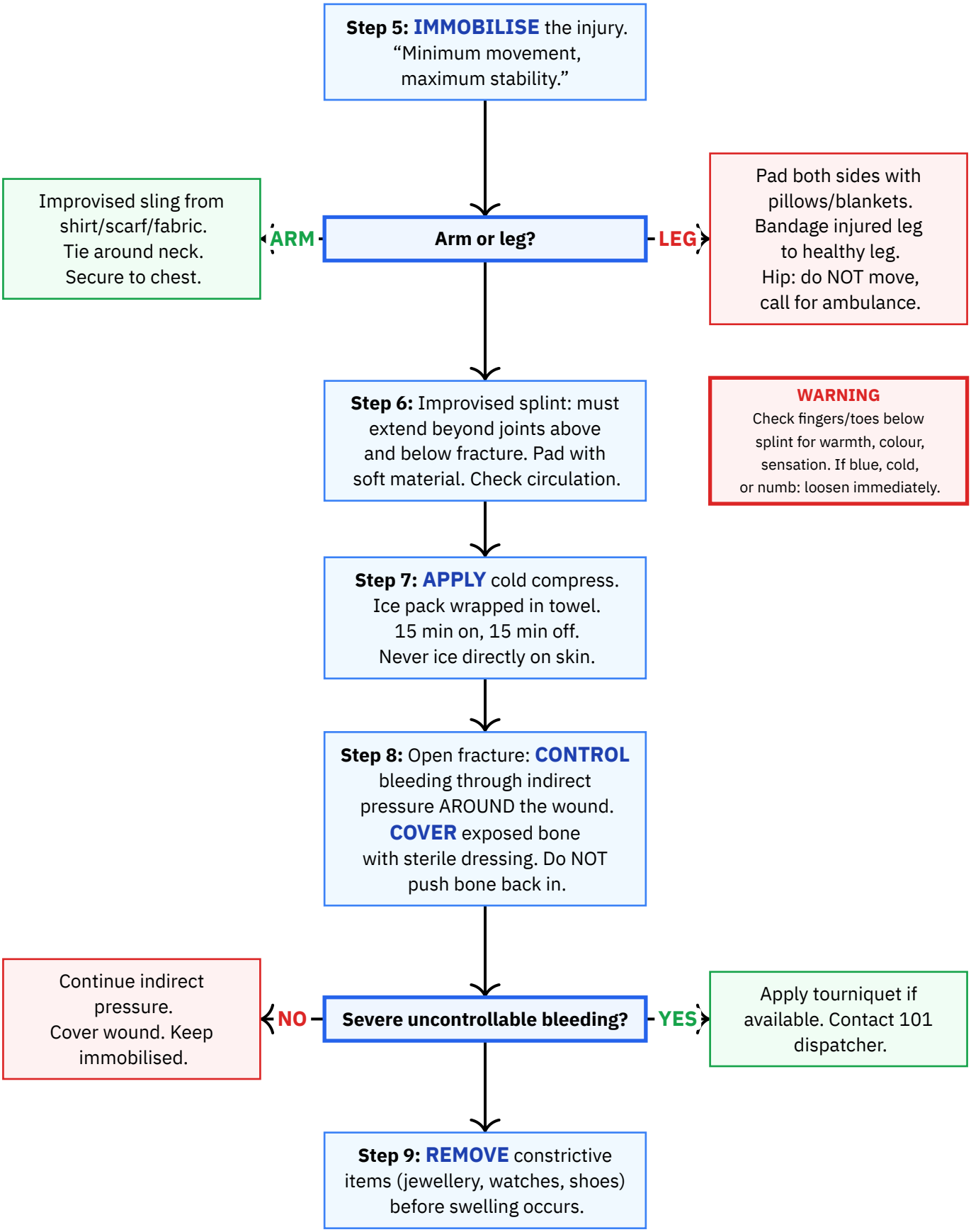
- YES → Do NOT move the child. **CALL** 101 immediately. Hold head steady. Wait for professionals.

NO ↓

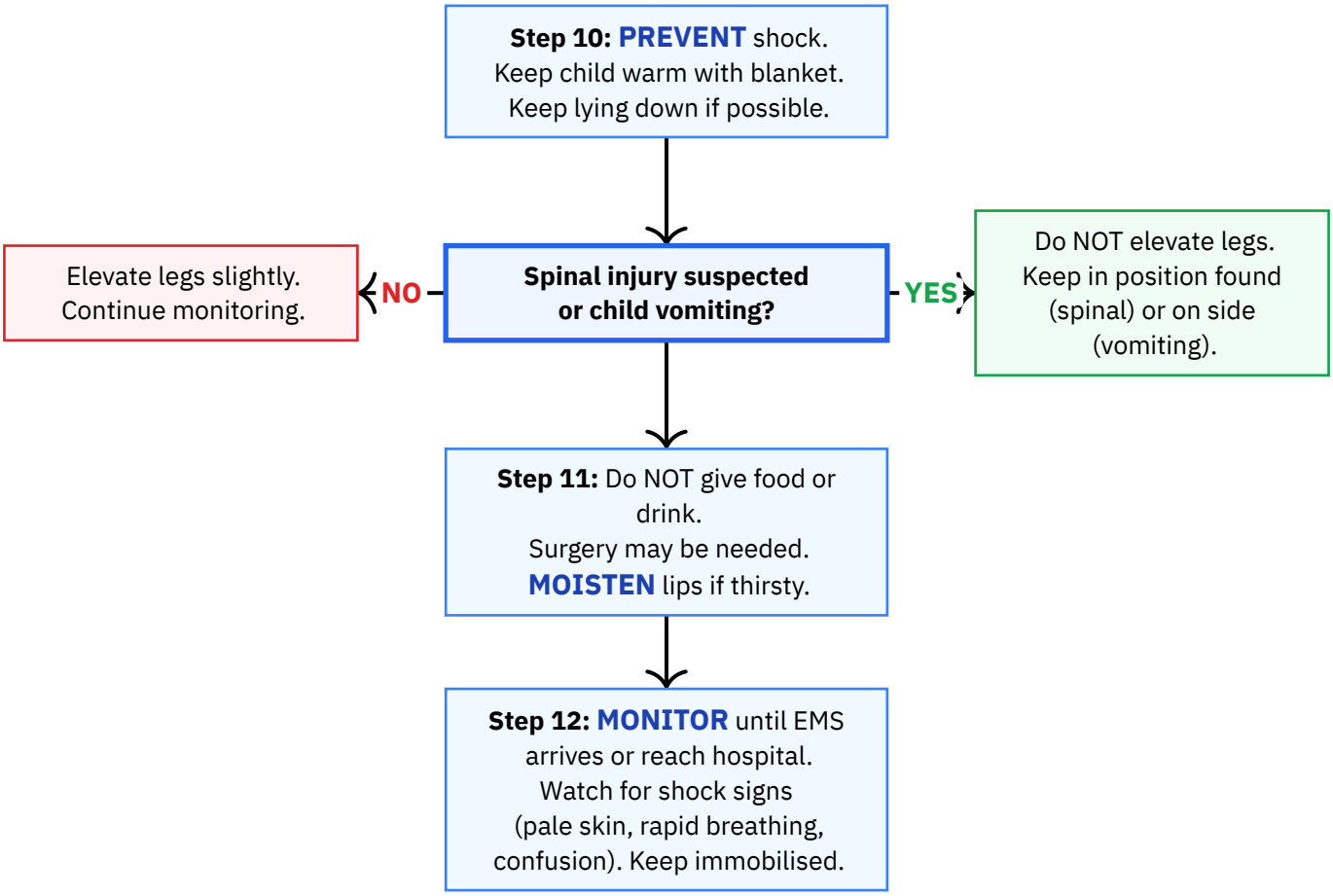
Step 4: Do NOT move or **REALIGN** the injury. Never straighten, pull, or push a bone back into place.

WARNING
Realigning can damage arteries, nerves, and growth plates – potentially affecting future bone development.

Immobilisation and Treatment (Steps 5–9)



Shock Prevention and Monitoring (Steps 10–12)



DO NOT:

- X Do NOT attempt to straighten, pull, or realign a broken bone -- damages arteries, nerves, growth plates.
- X Do NOT move the child unnecessarily, especially if spinal injury suspected.
- X Do NOT apply heat to the injury (no hot compresses, no warm water).
- X Do NOT apply ice directly to the child's skin -- always wrap in cloth.
- X Do NOT tie splints or bandages too tightly -- monitor circulation below.
- X Do NOT allow the child to bear weight on an injured leg.
- X Do NOT remove clothing by pulling over the injury -- cut it away.
- X Do NOT give aspirin to children (Reye's syndrome risk) -- use paracetamol or ibuprofen.
- X Do NOT push a protruding bone back into the wound.
- X Do NOT give the child food or drink -- surgery may be required.

Equipment needed:

— Improvised sling material (shirt, scarf, triangular bandage)
— Improvised splint (cardboard, newspapers, rulers, rigid material)
— Padding (clothing, towels, blankets) for splint and leg immobilisation
— Ice pack or cold compress (or frozen vegetables) + cloth for wrapping
— Sterile dressings or clean cloth (for open fractures)
— Bandages or fabric strips for securing splints
— Blanket for warmth and shock prevention
— Tourniquet (for severe uncontrollable bleeding, if available)

Source: Magen David Adom (MDA) — MDA 101 Public First Aid Guidance Series
URL: https://www.mdais.org/101/first-aid
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: MDA principle: “Minimum movement, maximum stability.” What appears to be a sprain in a child may be a growth plate fracture. All suspected sprains should be evaluated by a doctor.

Head / Spinal Injury First Aid -- Child

Israel — CHILD

CALL 101 (MDA) — DO NOT MOVE THE CHILD

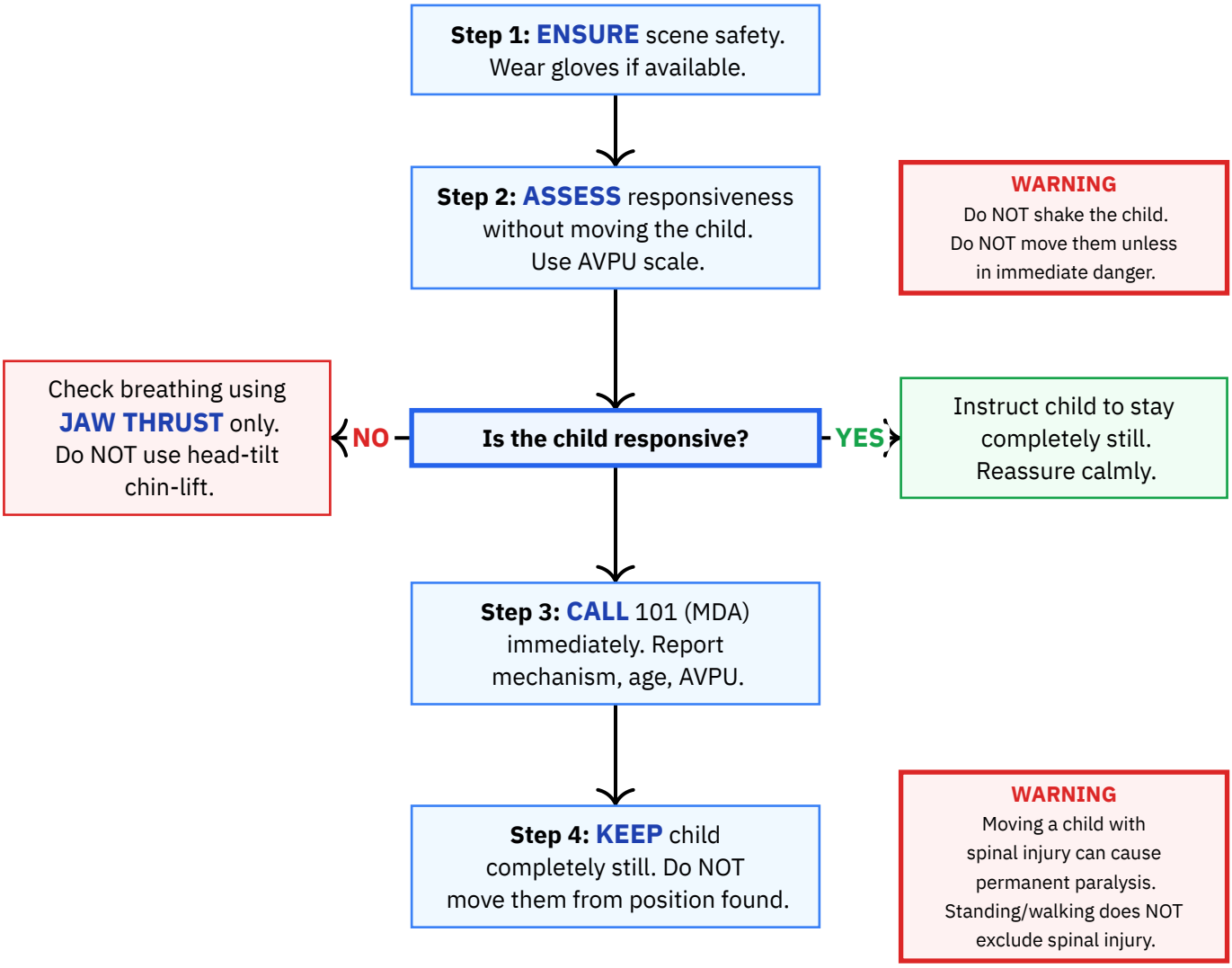
MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

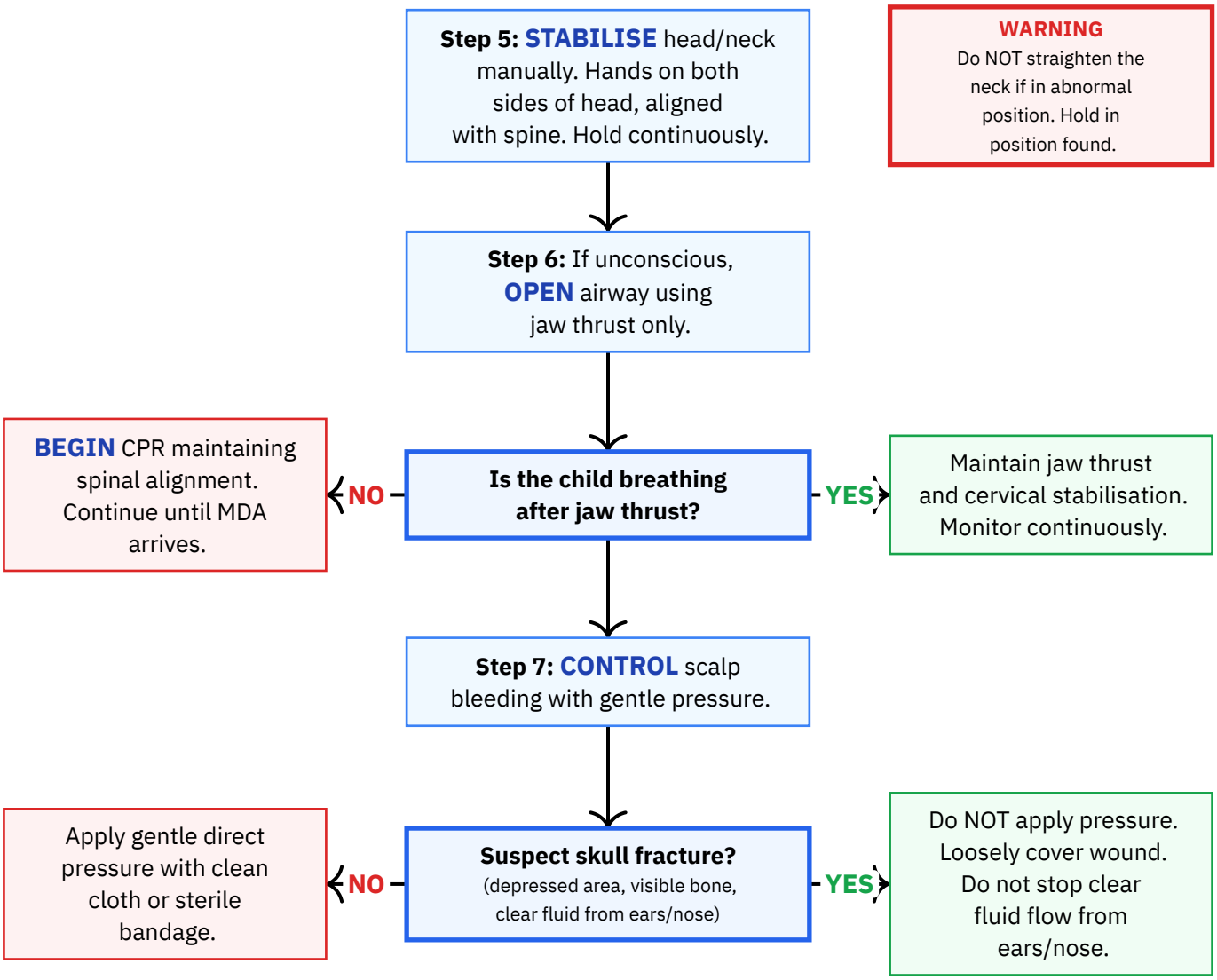
A child who has sustained or may have sustained a head or spinal injury from: falls from height (over 1 m or 2x child's height), vehicle accidents, bicycle/scooter crashes, playground falls, being struck by heavy objects, blast/shrapnel, any head impact in infants under 1 year.

Summary: Keep the child completely still. Call 101 immediately. Stabilise head and neck manually. Control scalp bleeding (avoid pressure on suspected fractures). Use jaw thrust only for airway. Monitor continuously until MDA arrives.

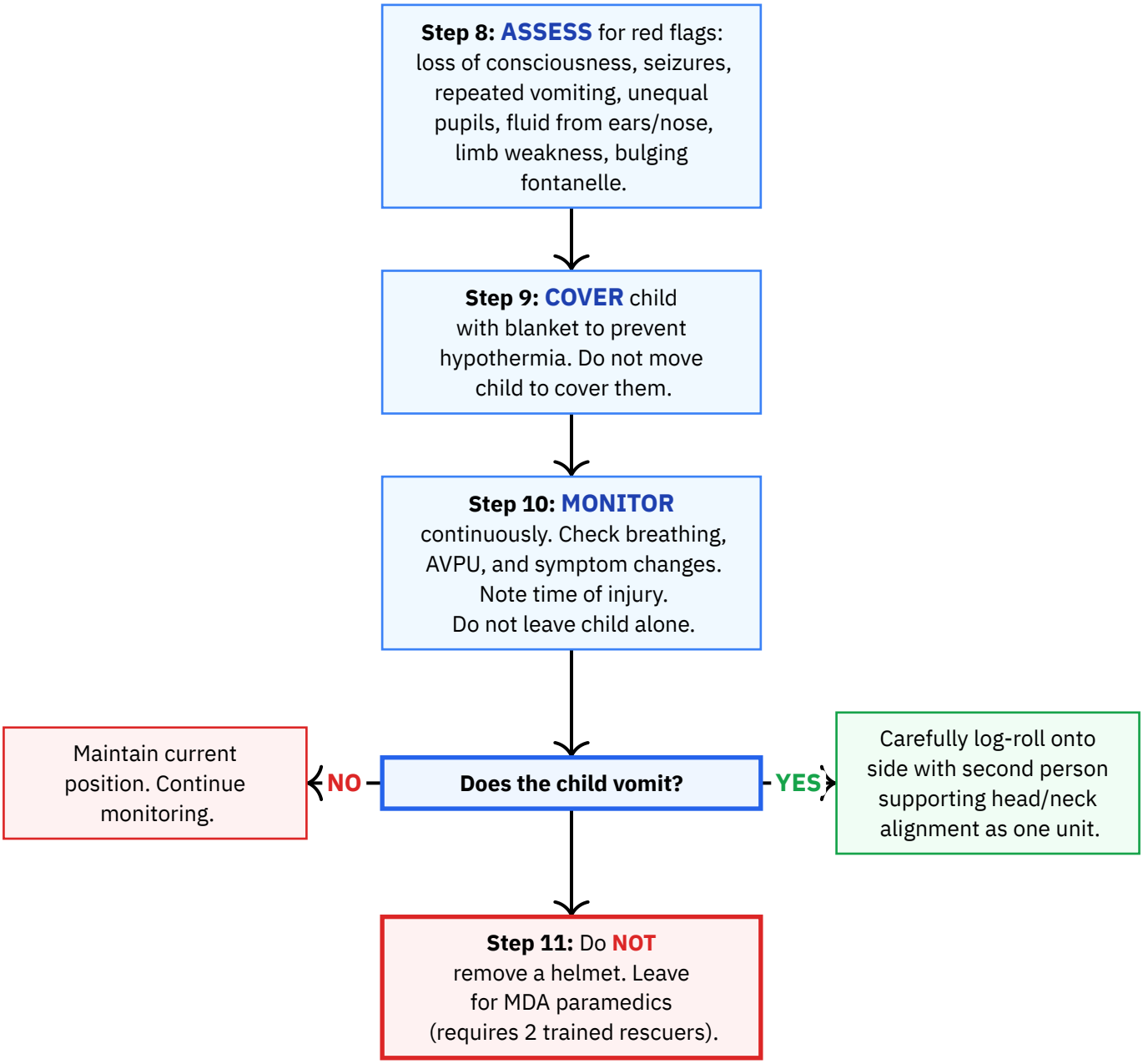
Initial Assessment – Steps 1-4



Stabilisation and Airway – Steps 5-7



Red Flags and Monitoring – Steps 8-11



DO NOT:

- X Do NOT move the child unless in immediate life-threatening danger.
- X Do NOT remove a helmet -- leave for MDA paramedics.
- X Do NOT allow the child to stand up or walk -- does NOT exclude spinal injury.
- X Do NOT give food or drink (may need surgery/anaesthesia).
- X Do NOT apply pressure to a depressed skull fracture or push back protruding matter.
- X Do NOT insert anything into a wound.
- X Do NOT attempt to straighten the neck if in abnormal position.
- X Do NOT use head-tilt chin-lift -- use jaw thrust only.
- X Do NOT shake the child to assess responsiveness.
- X Do NOT leave the child alone.
- X Do NOT stop clear fluid flow from ears or nose (may be cerebrospinal fluid).

Equipment needed:

— Disposable gloves
— Clean cloth or sterile bandages for wound control
— Blanket or coat for warmth

Source: Magen David Adom (MDA) – BLS Guide (2016), ALS Professional Manual (2024), Israeli Standard-of-Care Paediatric Head Injury Guidance
URL: https://www.mdais.org/101/first-aid
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: Unified from MDA and United Hatzalah sources. Both follow PHTLS-based trauma assessment (ABCDE) and selective immobilisation.

Heart Attack / Cardiac Emergency — Child

Israel — CHILD

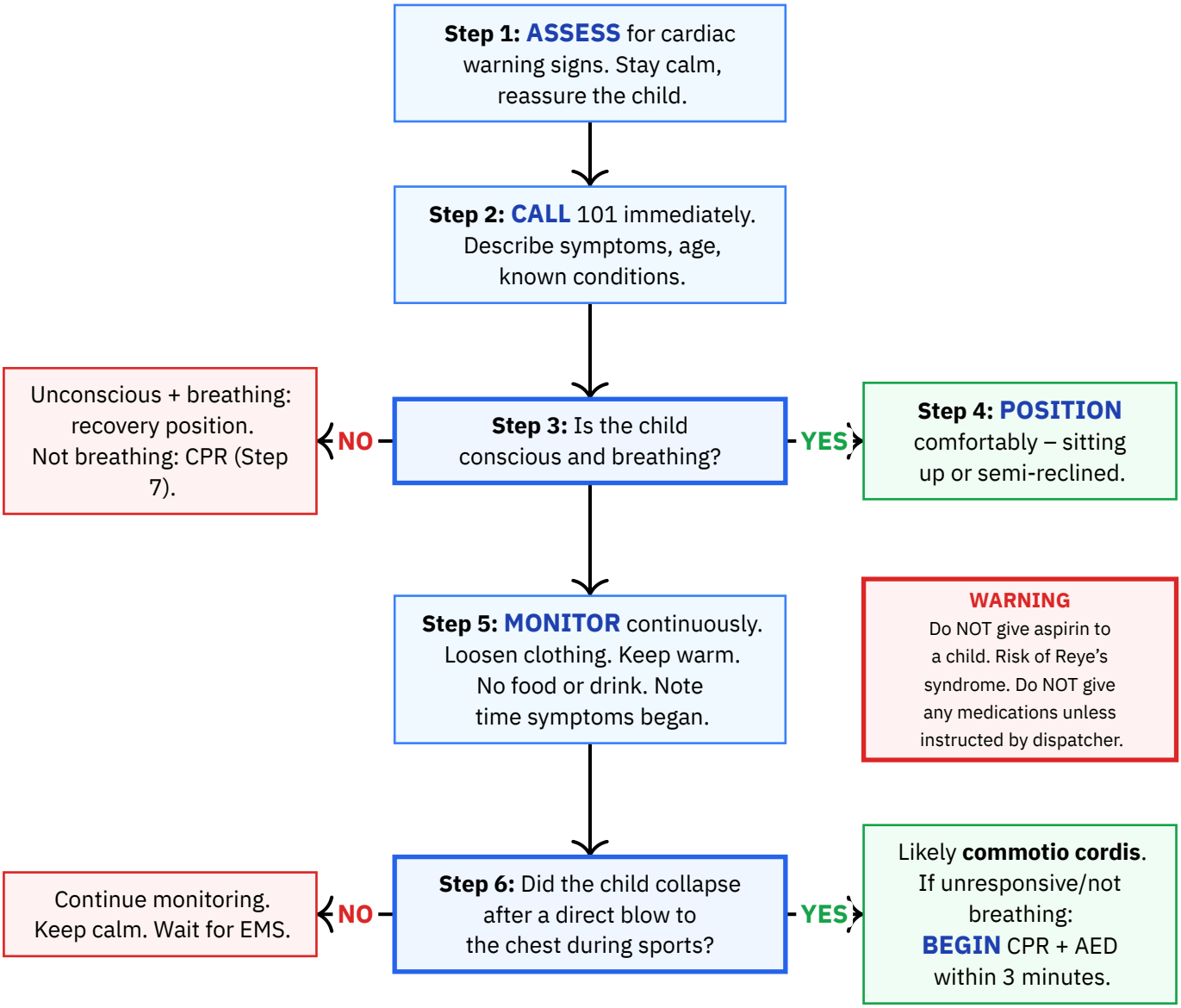
CALL 101 — DO NOT GIVE ASPIRIN TO CHILDREN

MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

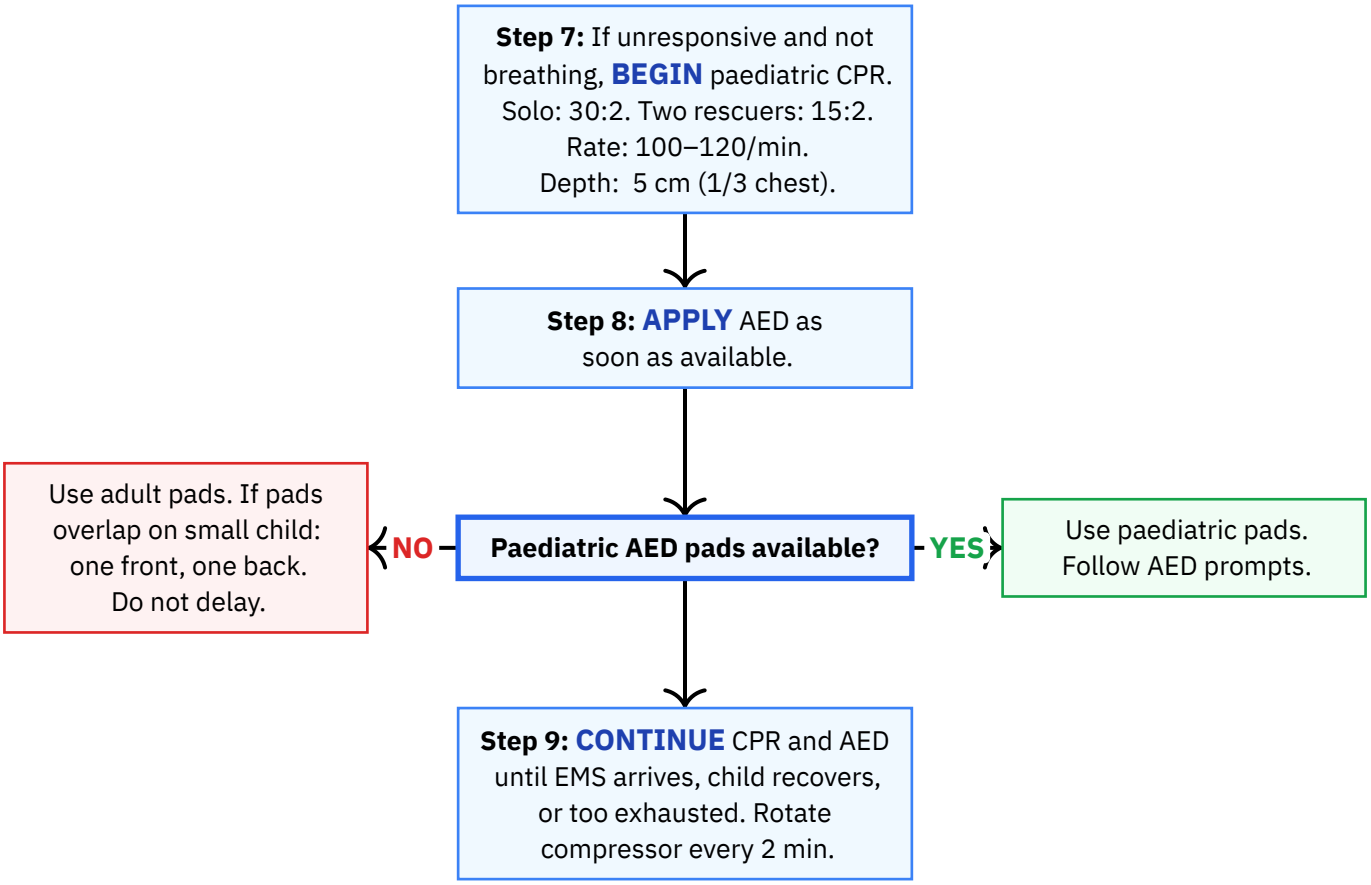
When to apply:

Child with persistent/severe chest pain (especially during exertion), syncope during activity, palpitations, unexplained breathlessness, sudden collapse during sports, cyanosis, chest pain with fever (possible myocarditis), or any cardiac symptoms in a child with known heart condition. True MI is exceedingly rare in children – cardiac causes include myocarditis, congenital defects, hypertrophic cardiomyopathy, commotio cordis.

Assessment and Response (Steps 1–6)



CPR and AED (Steps 7–9)



DO NOT:

X Do NOT give aspirin to a child -- risk of Reye's syndrome (unlike adult protocol).
X Do NOT give nitroglycerin or any cardiac medication unless prescribed and directed by a physician.
X Do NOT dismiss chest pain in a child with recent illness, known heart condition, or collapse during activity.
X Do NOT attempt to drive the child to hospital -- call 101 for ambulance with monitoring equipment.
X Do NOT leave the child unattended while waiting.
X Do NOT give food or drink (hospital procedures may be needed).
X Do NOT give any medications unless instructed by the 101 dispatcher or a physician.

Equipment needed:

— AED with paediatric pads or dose attenuator (if available)
— Phone to call 101 (MDA) or 1221 (United Hatzalah)
— MDA app (for AED locator and emergency calling)

Source: Magen David Adom (MDA) — MDA BLS Guide 2016; ALS Guide 2022; Training Syllabus 2026
URL: https://www.mdais.org/101/first-aid
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: True MI is exceedingly rare in children. Rare cardiac causes include myocarditis, congenital defects, hypertrophic cardiomyopathy, commotio cordis, Kawasaki disease, Long QT syndrome.

Heatstroke and Heat Exhaustion — Child

Israel — CHILD

CALL 101 (MDA) — HEAT STROKE IS LIFE-THREATENING

MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

Child shows signs of heat-related illness: intense thirst, headache, profuse sweating, pale clammy skin, nausea, vomiting, dizziness, floppiness or unusual sleepiness (young children), or severe signs such as hot dry red skin, confusion, seizures, or loss of consciousness. Children aged 0–4 are at highest risk.

Assessment and Cooling (Steps 1–5)

Step 1: ASSESS severity.

Hot dry red skin, temp above 40C, confusion, seizures, or unconscious?

- **NO** → Heat exhaustion. Move to shade, begin cooling. Call 101 if no improvement in 30 min.
- **YES** → HEAT STROKE – life-threatening. **CALL** 101 immediately. Begin aggressive cooling.

Step 2: CALL 101 or My MDA app.

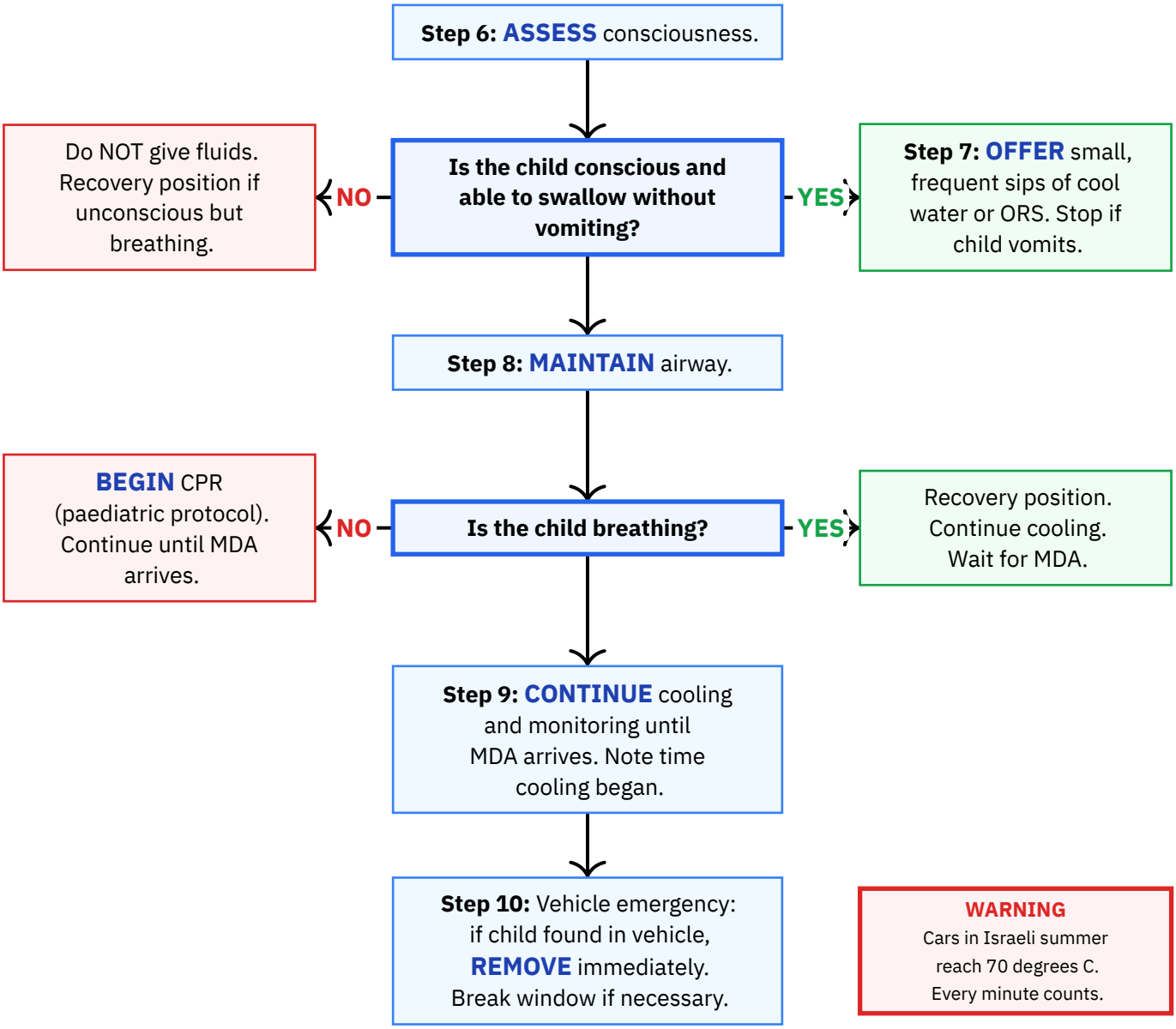
Step 3: MOVE child to cool environment immediately. Shade, A/C, or coolest location.

Step 4: REMOVE excess and tight clothing.

Step 5: BEGIN active cooling. Pour/spray tepid-cool water. Fan the child. Ice packs in armpits and groin.

WARNING
Do not use ice-cold water immersion for young children without medical supervision.

Hydration and Airway (Steps 6–10)



DO NOT:

X Do NOT give fever-reducing medications (Acamol, Nurofen, naproxen) -- they do not reduce heat stroke temperature and may cause harm.
X Do NOT use ice-cold water immersion for young children without medical supervision.
X Do NOT force fluids if the child is vomiting or has altered consciousness.
X Do NOT leave the child unattended at any point.
X Do NOT give fluids to an unconscious child.
X Do NOT delay calling 101 if heat stroke is suspected.
X Do NOT leave a child unattended in a vehicle, even for a moment.

Equipment needed:

— Cool water (for pouring or spraying)
— Fan or means of fanning
— Ice packs or cold wet cloths
— Oral rehydration solution (ORS) or cool drinking water
— Thermometer (if available)

Source: Magen David Adom (MDA) — ALS Manual 2024; public guidelines 2020–2024
URL: https://www.mdais.org/news/300820
Publication date: 2024-04-01 · **Imported:** 2026-03-15 · **Last verified:** 2026-03-15

Hypothermia First Aid -- Child

Israel — CHILD

CALL 101 (MDA) — HANDLE GENTLY

MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

When a child shows signs of hypothermia (core temperature below 35 C): shivering (may be absent in infants), cold pale skin, confusion, slurred speech, lethargy. Infants may show bright red cold skin, low energy, weak cry. Children are at higher risk due to immature thermoregulation and higher surface-area-to-mass ratio.

Key Principle: “No one is dead until they are warm and dead.” A severely hypothermic child may appear dead but can recover with proper rewarming. Handle with extreme gentleness – a cold heart can develop fatal arrhythmias from rough movement.

Personal reference only — not medical advice

Rewarming Protocol – Steps 1-5

Step 1: REMOVE child from cold environment. Handle very gently.

WARNING
Minimise all movement. A cold heart is extremely sensitive and can develop fatal arrhythmias.

↓

Step 2: REMOVE wet clothing. Cut or carefully remove. Replace with dry clothing.

↓

Step 3: DRY the child. **WRAP** in warm blankets. Position horizontally. Cover head, neck, chest, groin.

↓

Step 4: CHECK ABC (Airway, Breathing, Circulation). Take up to 60 seconds for pulse/breathing assessment.

↓

Is the child breathing and has a pulse?

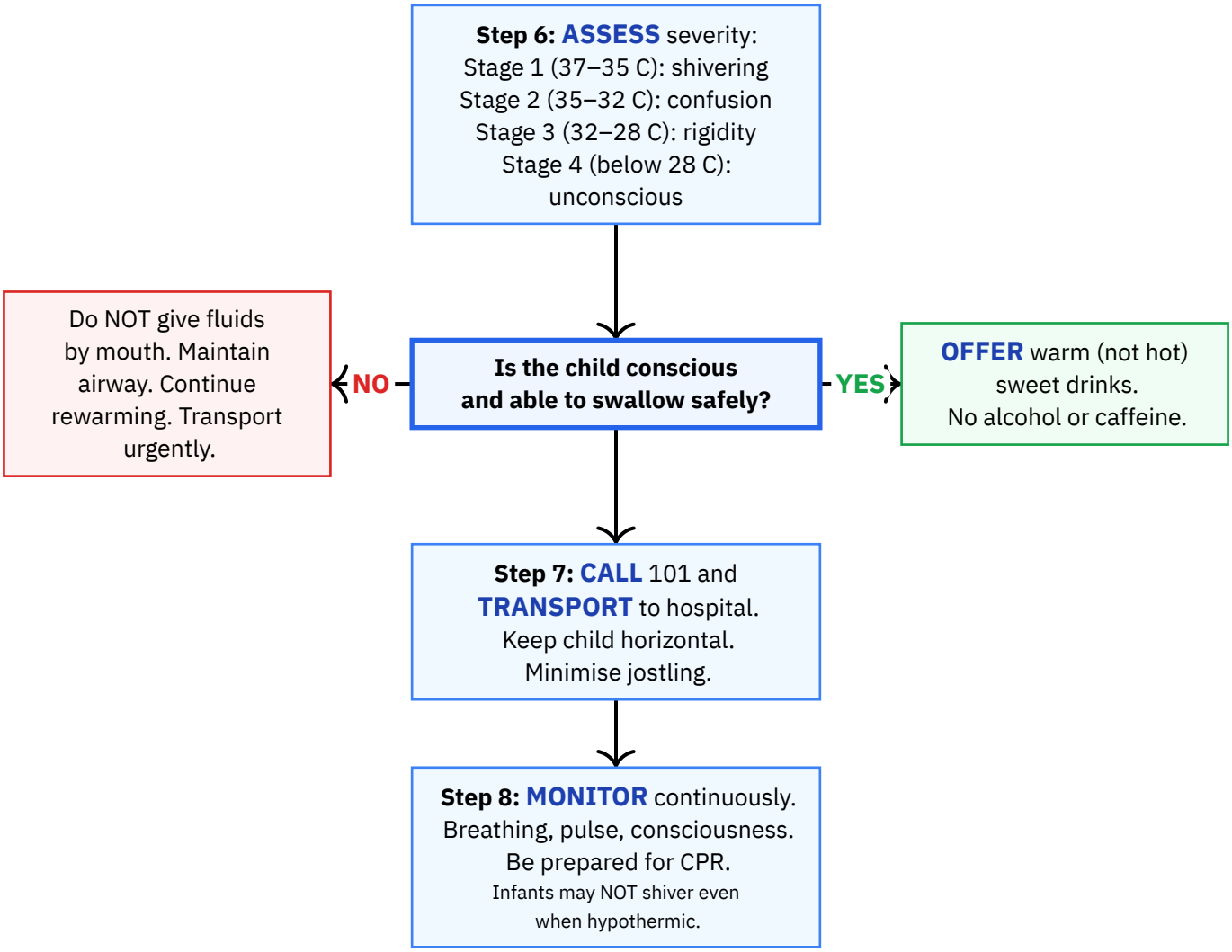
- **NO** → **BEGIN** CPR immediately. Continue CPR and rewarming until emergency services arrive.
- **YES** → Continue rewarming. Monitor continuously. Recovery position if unconscious but breathing.

↓

Step 5: APPLY warm blankets or wrapped heat packs to head, neck, chest, groin. Warm core first, not extremities.

WARNING
Do NOT apply direct heat to skin – risk of burns. Always wrap heat sources in cloth.

Severity Assessment and Transport – Steps 6-8



DO NOT:

X Do NOT give alcohol, caffeine, or allow smoking -- worsen hypothermia.
X Do NOT make excessive or rough movements -- risk of cardiac arrhythmias.
X Do NOT apply direct heat to skin without a cloth barrier -- risk of burns.
X Do NOT rewarm extremities before the core -- dangerous core temperature drop as cold blood returns to heart.
X Do NOT give fluids by mouth to unconscious or semi-conscious child -- aspiration risk.
X Do NOT assume a severely hypothermic child is dead -- 'no one is dead until they are warm and dead.'
X Do NOT use hot water immersion for rewarming in the field.
X Do NOT jostle or move the child unnecessarily during transport.

Equipment needed:

— Warm blankets or sleeping bags
— Dry clothing
— Heat packs or warm water bottles (wrapped in cloth)
— Phone (to call 101)

Source: Magen David Adom (MDA) – ALS Professional Manual (April 2024), supplemented by Ichud Hatzalah hypothermia guidelines
URL: https://www.mdais.org/101/first-aid
Publication date: 2024-04-01
Imported: 2026-03-16 · **Last verified:** 2026-03-16
Notes: Unified from MDA and United Hatzalah sources. Four-stage severity classification from Hatzalah incorporated.

Poisoning First Aid -- Child

Israel — CHILD

CALL 101 (MDA) — Poison Center: 04-7771900

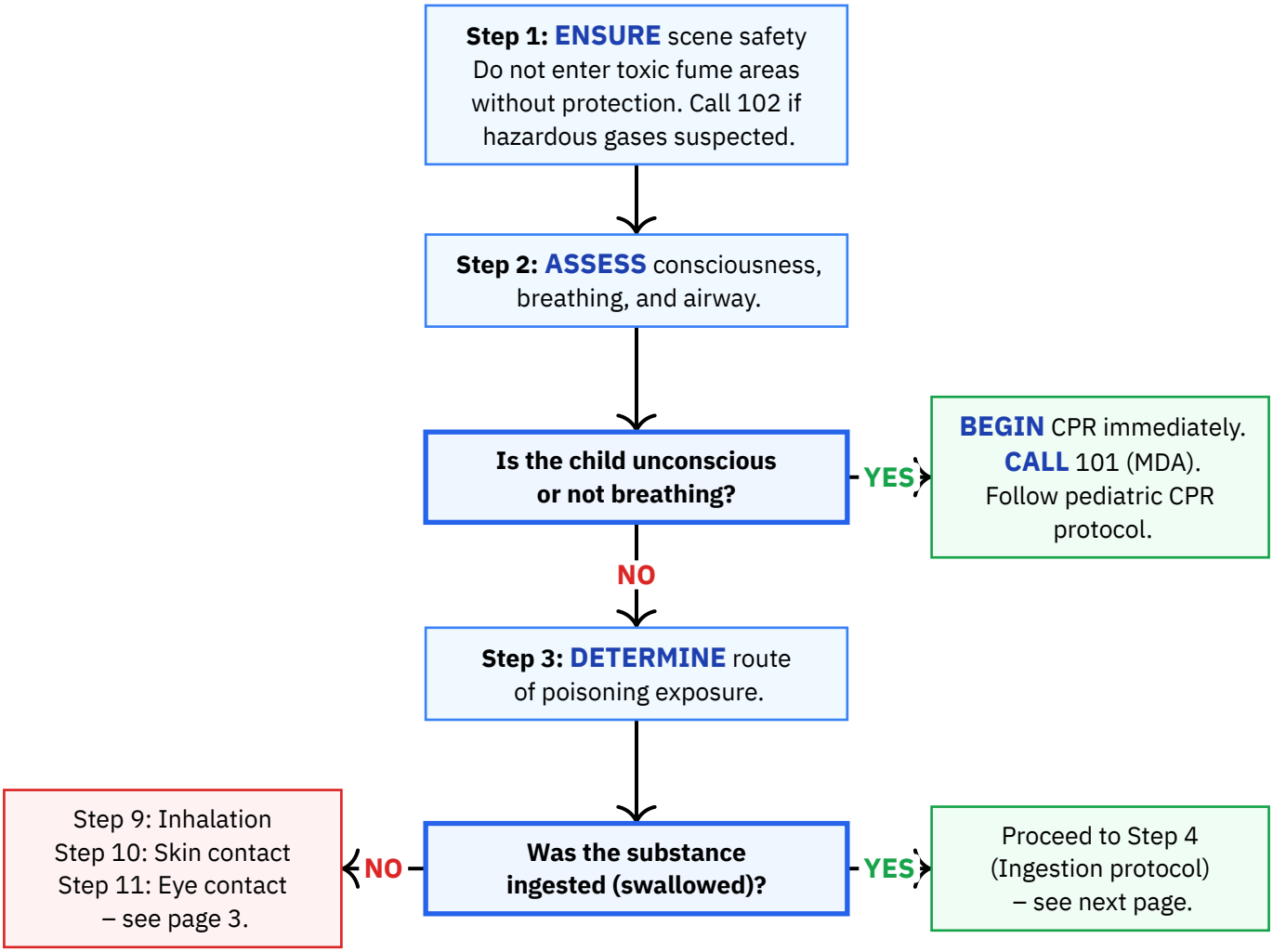
MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

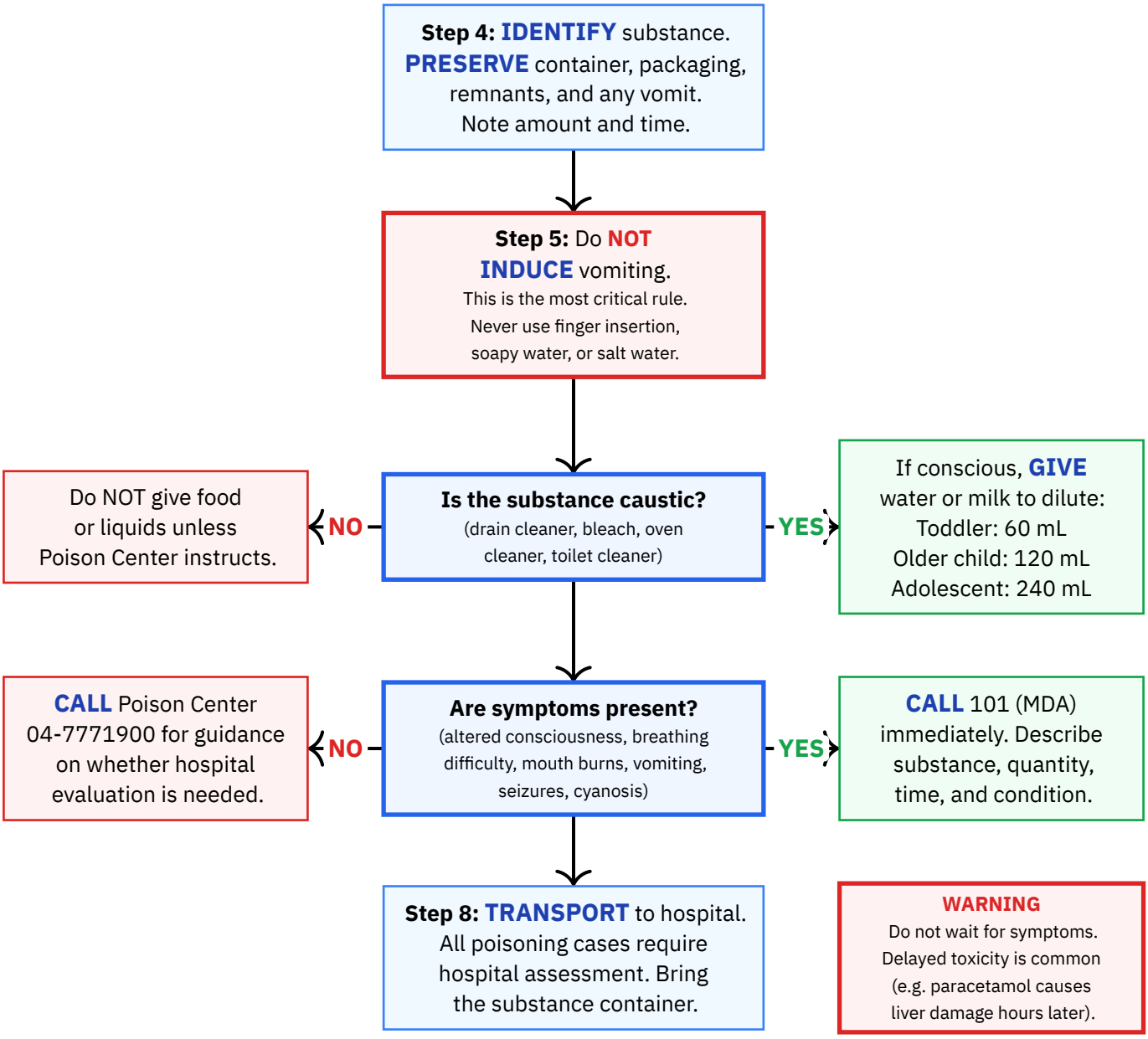
When a child is suspected of having ingested, inhaled, or come into contact with a toxic substance. Common substances: medications (49%), cleaning products (36%), pesticides, plants, alcohol. 90% of child poisoning occurs at home. Signs: altered consciousness, breathing difficulty, burns in/around mouth, vomiting, abdominal pain, drooling, unusual odour.

Summary: Never induce vomiting. Preserve the substance container. Call Poison Center (04-7771900) for non-symptomatic cases, 101 for symptomatic cases. Dilute caustic substances with water or milk if the child is conscious.

Assessment Phase



Ingestion Protocol



Non-Ingestion Exposure Routes

Step 9 – Inhalation:
MOVE child to fresh air.
If not breathing, begin CPR.
CALL 101 and 102 if hazardous gases suspected.

↓

Step 10 – Skin Contact:
REMOVE contaminated clothing.
FLUSH skin with running water for 15–20 minutes.
Do not apply creams.

↓

Step 11 – Eye Contact:
FLUSH eye(s) with running water for 15–20 minutes.
Gently lift eyelids.
Do not rub eyes.

WARNING
Never enter confined space with toxic fumes without protection. Never mix bleach with other cleaners.

Reference

DO NOT:

X Do NOT induce vomiting -- regardless of the substance. Causes additional harm (oesophageal burns, aspiration risk).
X Do NOT give activated charcoal at home unless instructed by Poison Center or physician.
X Do NOT give food to the poisoned child. Water/milk for dilution of caustic substances only.
X Do NOT attempt home remedies (raw eggs, olive oil, etc.).
X Do NOT wait for symptoms before calling for guidance -- delayed toxicity is common.
X Do NOT discard the container -- always bring it to the hospital.
X Do NOT mix cleaning products -- especially bleach with acids (releases toxic chlorine gas).
X Do NOT enter a toxic fume environment without proper protection.

Equipment needed:

— Cloth (to wipe substance from child's mouth if present)
— Water or milk (for dilution of caustic substances only, age-appropriate amounts)
— Running water source (for skin and eye decontamination, 15-20 minutes flushing)
— Container or bag (to preserve substance packaging and any vomit for medical teams)

Source: Magen David Adom (MDA) – MDA Poisoning First Aid Guidelines, supplemented by Rambam National Poison Information Center guidelines and Beterem child safety materials
URL: https://www.mdais.org/101/poisoningn
Publication date: 2026-01-01
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: Unified from MDA and United Hatzalah sources. 43% of poison center calls involve children under age 6. Pre-Passover cleaning season is a known spike period.

Recovery Position -- Child

Israel — CHILD

CALL 101 (MDA) FOR ANY UNCONSCIOUS CHILD

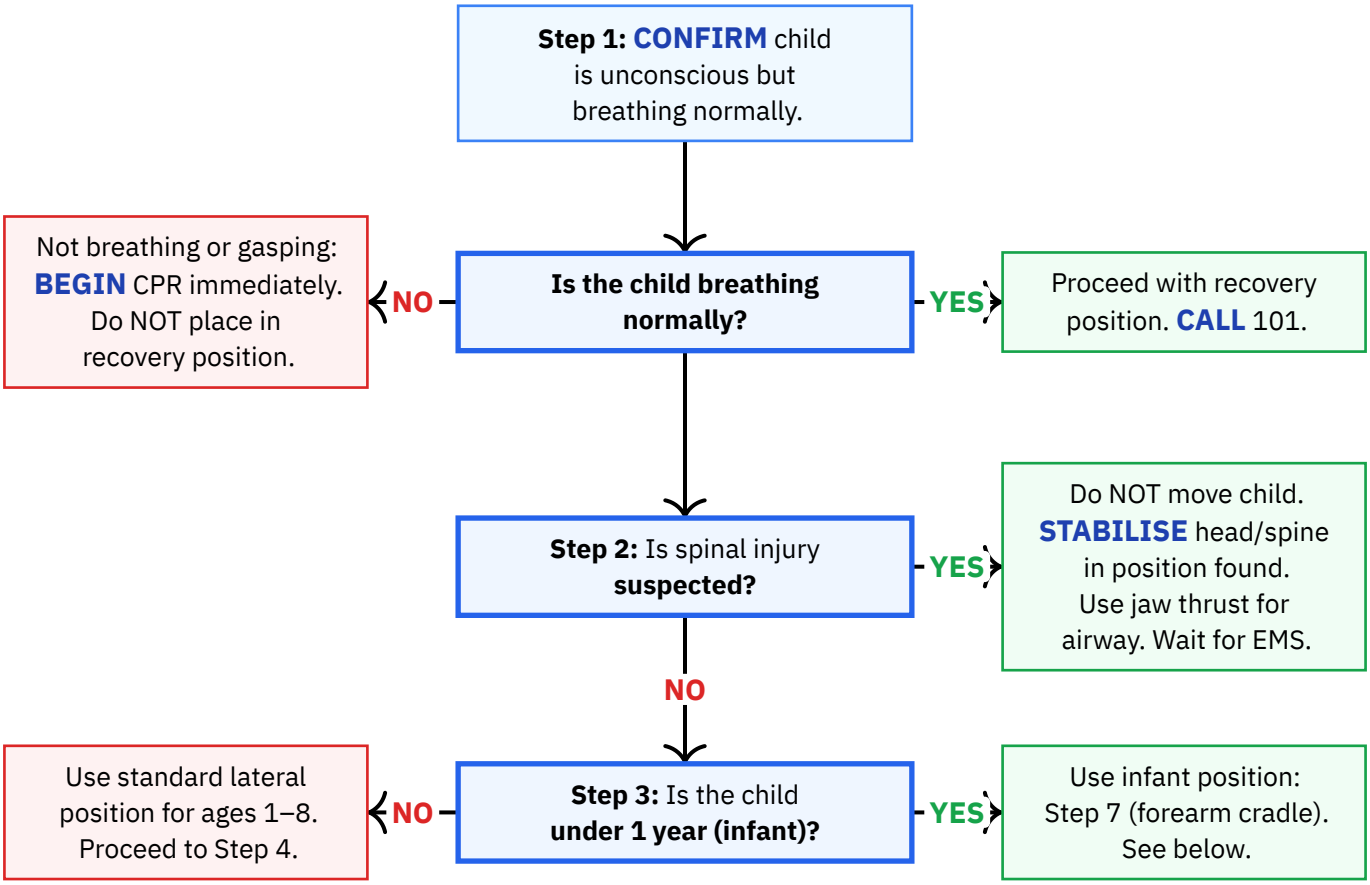
MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

When a child is unconscious but breathing normally and has no suspected spinal injury. Maintains an open airway, facilitates drainage of secretions, and reduces aspiration risk. Do NOT use if spinal injury is suspected.

Summary: Children aged 1–8: standard lateral recovery position (more gently than adults). Infants under 1: cradle/arm position (face-down on rescuer's forearm). Children have proportionally larger heads – position carefully to avoid neck flexion.

Assessment – Steps 1-3



Lateral Position (Ages 1-8) – Steps 4-6

Step 4: KNEEL beside child. Place near arm at right angle, elbow bent, palm facing up.

↓

Step 5: BRING far arm across chest. Hold back of hand against cheek. Bend far knee. **ROLL** child towards you gently.

↓

Step 6: ADJUST position. Upper leg: hip and knee at right angles. Tilt head back gently for open airway. Mouth angled downward for drainage.

WARNING
Children have larger heads proportionally. Ensure head position does not flex neck forward – this can obstruct the airway.

Infant Position and Monitoring – Steps 7-8

Step 7 – Infant (under 1): HOLD infant face-down on your forearm. Head supported by your hand, lower than body. Support jaw with fingers.

WARNING
Do NOT place infants in the standard lateral position. Use the forearm cradle only.

↓

Step 8: MONITOR breathing continuously (every minute). Cover with blanket. Be ready for CPR.

↓

Has the child stopped breathing?

- **NO** → Continue monitoring in recovery position. Maintain warmth and airway patency.
- **YES** → **ROLL** child onto back. **BEGIN** CPR immediately.

DO NOT:

X Do NOT use recovery position if child is not breathing -- begin CPR instead.
X Do NOT use recovery position if spinal injury is suspected -- stabilise the spine.
X Do NOT place infants under 1 year in standard lateral position -- use forearm cradle.
X Do NOT leave child unattended in recovery position -- monitor breathing continuously.
X Do NOT allow head to flex forward (especially in children with larger heads) -- obstructs airway.
X Do NOT forget to cover the child to prevent hypothermia.

Equipment needed:

— Blanket or coat (to prevent hypothermia)
— Phone (to call 101)

Source: Magen David Adom (MDA) – BLS Guide (2016), supplemented by Ichud Hatzalah pediatric treatment approach guidelines
URL: https://www.mdais.org/101/first-aid
Publication date: 2016-01-01
Imported: 2026-03-16 · **Last verified:** 2026-03-16
Notes: Children aged 1–8 use standard adult lateral recovery position with greater gentleness. Infants under 1 use cradle/arm position.

Scorpion Sting First Aid -- Child

Israel — CHILD

CALL 101 (MDA) FOR EVERY CHILD STING

MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

When a child has been stung or suspected stung by a scorpion. All scorpion stings in children require emergency evaluation – do not “wait and see.” The deathstalker scorpion (Leiurus quinquestriatus) can be deadly for young children. Black scorpions can also be dangerous.

Summary: Call 101 immediately. Keep the child calm and still. Cool the sting site with wrapped ice. Immobilise the limb below heart level. Monitor for systemic symptoms. Transport urgently to hospital.

Immediate Response

Step 1: MOVE child away from the scorpion. Do not attempt to catch it. Photograph if safe.

WARNING
Do NOT attempt to catch or handle the scorpion. Risk of additional stings.

Step 2: CALL 101 (MDA) immediately. Report age, weight, time of sting, location on body.

WARNING
Do NOT delay calling 101 for children. They deteriorate faster due to venom-to-body-weight ratio.

Step 3: KEEP the child calm and still. Have them lie down or sit still. Reassure with calm voice.

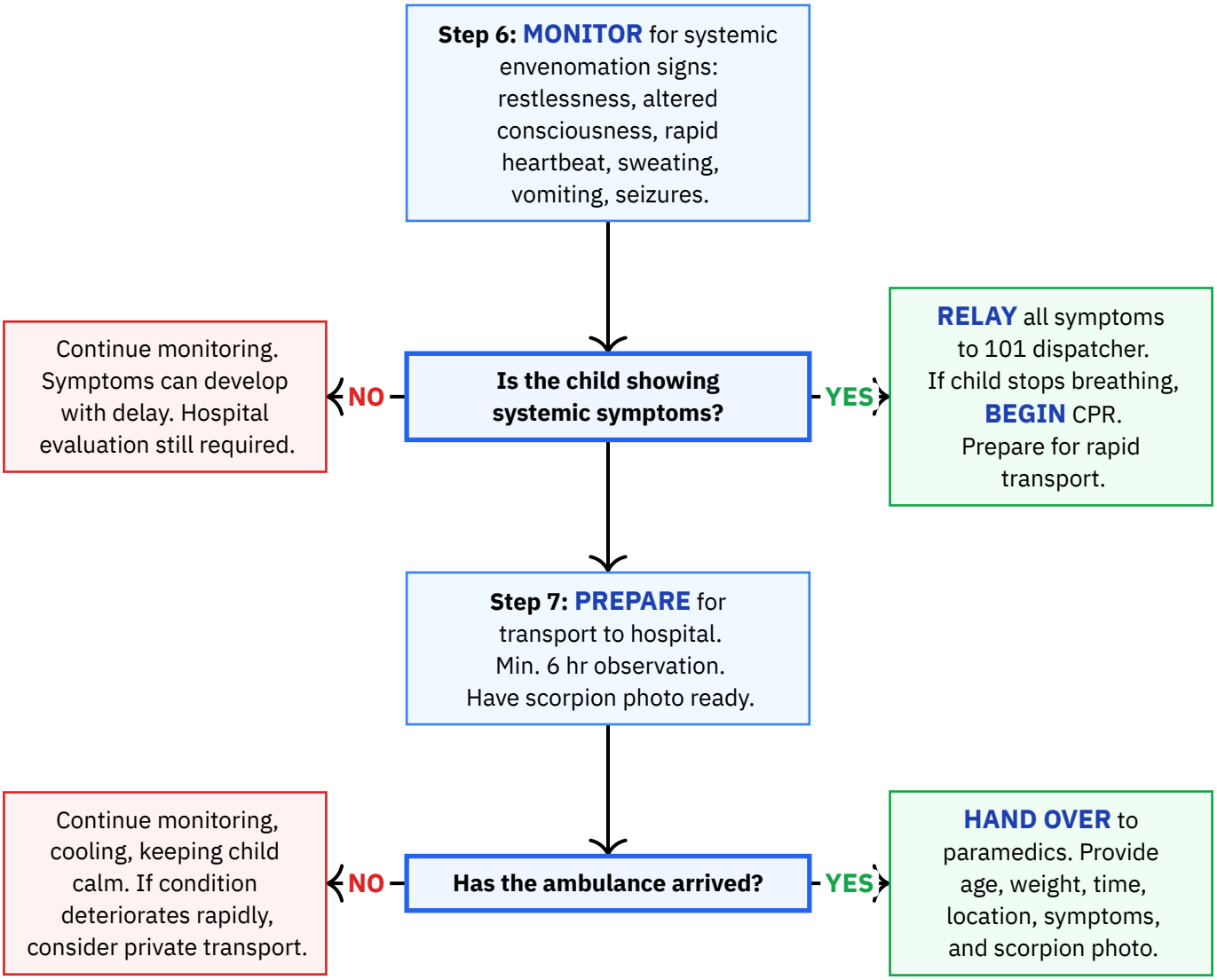
Is ice or cold compress available?

NO → Skip cooling. Focus on keeping child calm, still, and limb immobilised.

YES → **Step 4: APPLY** ice wrapped in cloth. 10 min on, 10 min off. Do NOT apply ice directly to skin.

Step 5: IMMOBILISE stung limb below heart level. Use splint if available.

Monitoring and Transport



Reference

DO NOT:

- X Do NOT apply a tourniquet -- concentrates venom and causes tissue damage.
- X Do NOT cut or incise the sting site -- causes additional injury, does not remove venom.
- X Do NOT attempt to suck out the venom -- ineffective and risks infection.
- X Do NOT apply alcohol or antiseptic to the wound.
- X Do NOT give the child medication without medical advice.
- X Do NOT underestimate a sting based on scorpion colour -- black scorpions can be lethal.
- X Do NOT delay calling 101 for children -- faster deterioration due to venom-to-body-weight ratio.
- X Do NOT apply ice directly to skin -- always wrap in cloth.
- X Do NOT allow the child to walk or move -- movement accelerates venom distribution.
- X Do NOT 'wait and see' with children -- all stings require emergency evaluation.

Equipment needed:

— Ice pack or cold compress (wrapped in cloth)
— Splint or rigid object for limb immobilisation
— Phone (to call 101 / 04-7771900 and photograph the scorpion)
— Cloth or towel (to wrap ice pack)
— Blanket or clothing (to keep child comfortable while lying still)

Source: Magen David Adom (MDA) – supplemented by Jerusalem Post articles, Israeli Ministry of Education first aid instructions, Midaat.org.il, and peer-reviewed Israeli pediatric clinical data (Paret et al. 1991)
URL: https://www.mdais.org/101/first-aid
Publication date: 2026-01-01
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: Unified from MDA and United Hatzalah sources. Deathstalker scorpion (Leiurus quinquestriatus) found throughout Israel. IV antivenom effective and safe in children with significant symptoms.

Seizures (Including Febrile Seizures) — Child

Israel — CHILD

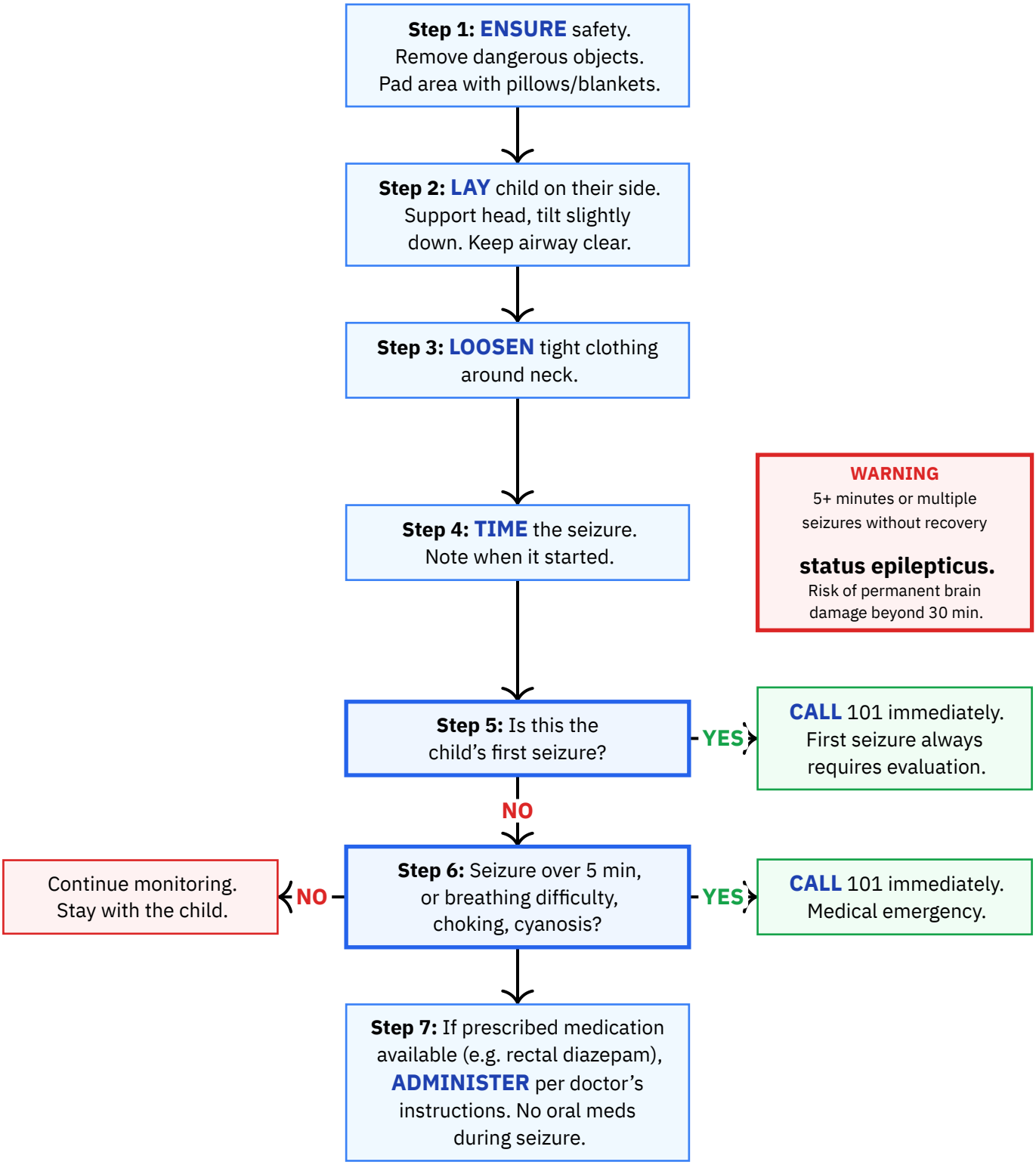
CALL 101 IF: FIRST SEIZURE, OVER 5 MIN, OR NO RECOVERY

MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

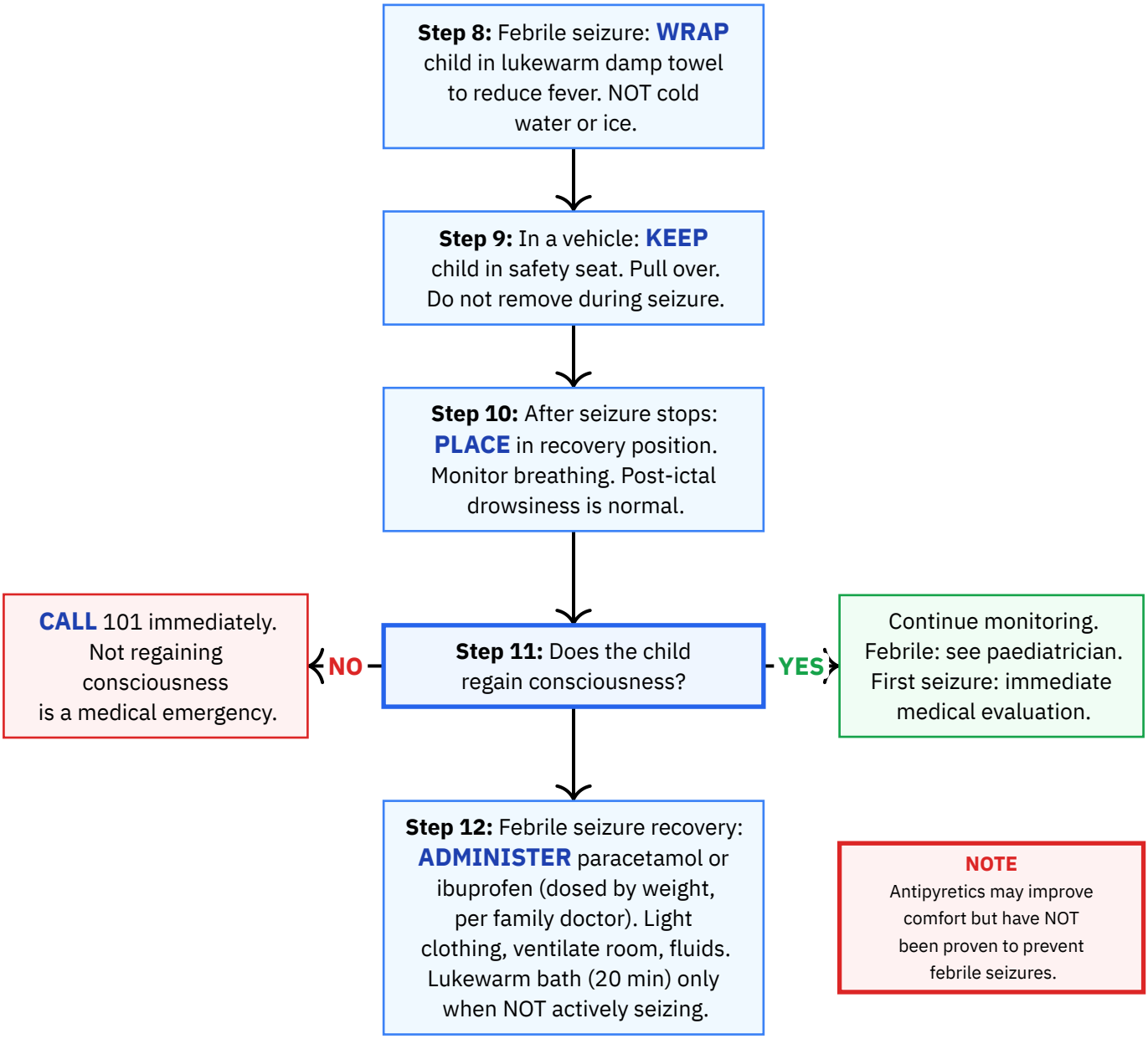
When to apply:

Child experiencing a seizure: rhythmic jerking or stiffening, unresponsiveness, eyes rolling, loss of consciousness. Febrile seizures: ages 6 months to 5–6 years, in the context of fever. A seizure lasting 5+ minutes is a medical emergency (status epilepticus).

During the Seizure (Steps 1–7)



After the Seizure (Steps 8–12)



DO NOT:

- X DO NOT force the child's mouth open -- the child will not swallow their tongue.
- X DO NOT insert anything into the mouth -- can injure teeth or jaw.
- X DO NOT attempt CPR during the seizure -- this can only cause harm.
- X DO NOT put the child in a bath or shower during the seizure.
- X DO NOT splash water on the child during the seizure.
- X DO NOT try to wake the child or restrain/hold them down.
- X DO NOT give medications by mouth during the seizure.
- X DO NOT attempt rapid cooling with cold water or ice.
- X DO NOT insert an airway device during an active seizure.
- X DO NOT attempt forced airway opening during an active seizure.

Equipment needed:

— Lukewarm damp towel (for febrile seizures)
— Pillows or blankets (to pad surroundings)
— Prescribed seizure-stopping medication if available (e.g. rectal diazepam)
— Fever-reducing medication for post-seizure (paracetamol or ibuprofen, per doctor)

Source: Magen David Adom (MDA) — MDA 101 Public First Aid Guidance Series
URL: https://www.mdais.org/101/seizures
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: Febrile seizures affect children aged 6 months to 5–6 years, peak at 18–22 months, prevalence 3–4%.

Severe Bleeding (Hemorrhage Control) — Child

Israel — CHILD

CALL 101 (MDA) IMMEDIATELY

MDA: 101 **Police:** 100 **Fire:** 102 **Hatzalah:** 1221 **Poison:** 04-7771900

When to apply:

Severe or uncontrolled external bleeding in a child (1–12 years). Children have significantly smaller blood volumes (a 3-year-old has only 1,200 mL total) – what appears moderate in an adult may be life-threatening in a child.

Initial Response (Steps 1–5)

Step 1: ENSURE scene safety.
Wear gloves if available.
Involve parent/caregiver.

↓

Step 2: CALL 101 immediately.
State child's age and weight.
Children deteriorate faster.

↓

Step 3: CALM the child.
Speak softly, involve parent.
Shield from sight of blood.

↓

Step 4: CHECK for embedded objects.

↓

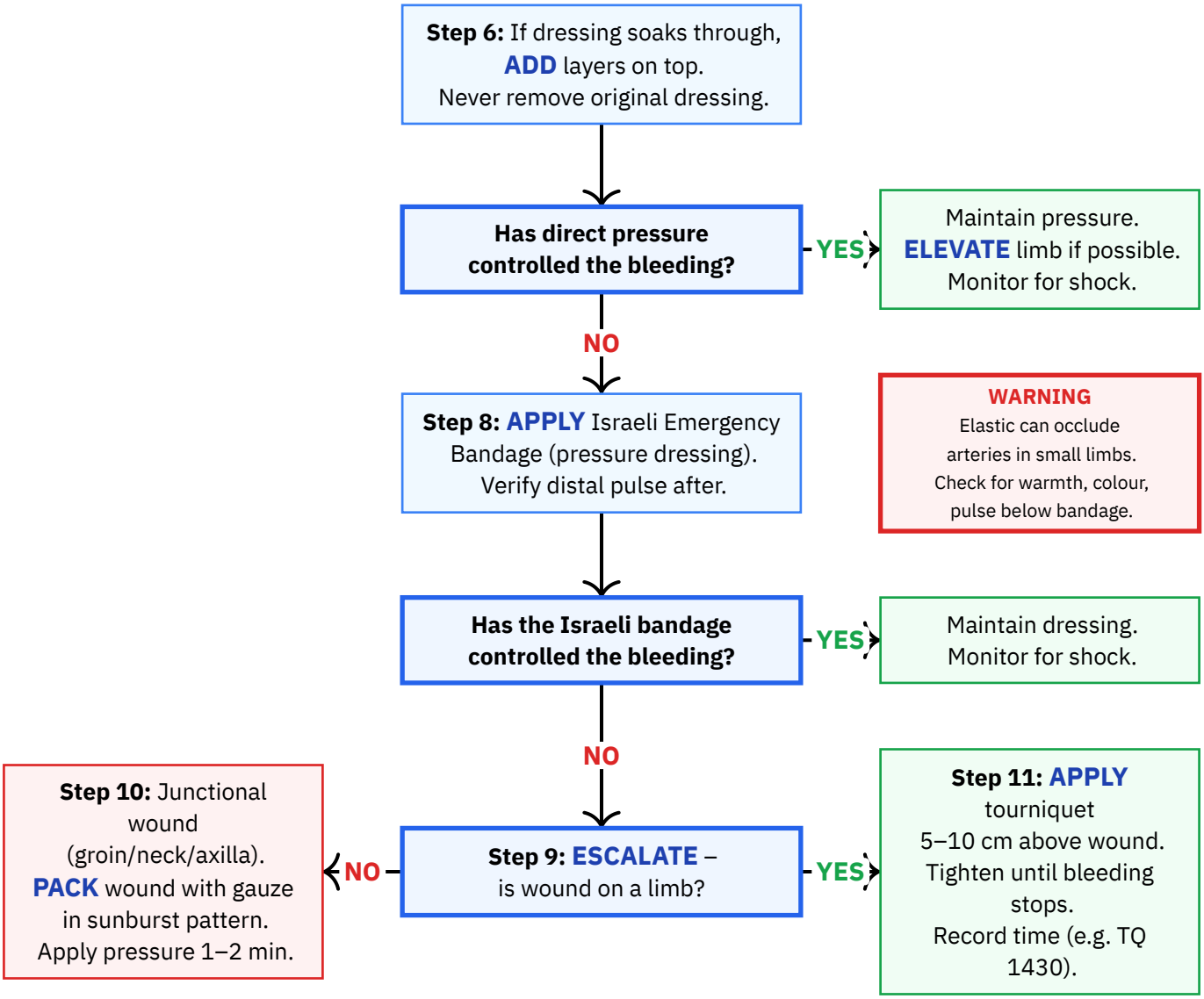
Is there an embedded or impaled object in the wound?

- **YES** → Do NOT remove it.
STABILISE with padding.
Apply pressure AROUND it, not on it.
Skip to shock monitoring.

- **NO** ↓

Step 5: APPLY strong direct pressure to the wound with gloved hand or gauze.
Maintain continuous pressure.

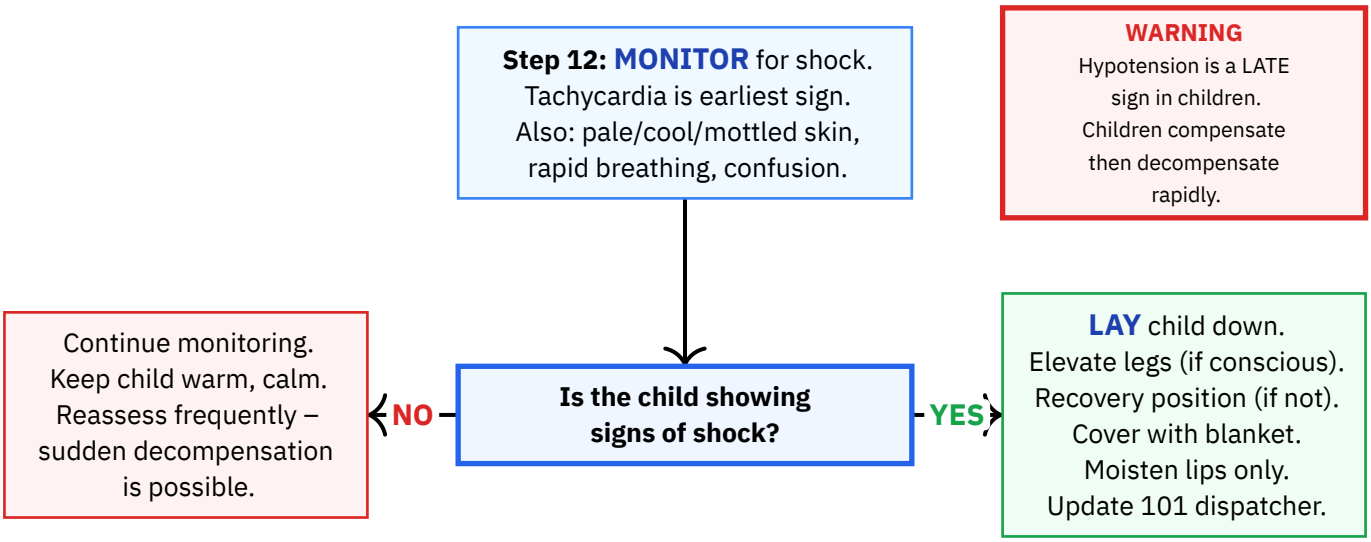
Escalation (Steps 6–11)



Tourniquet by Age

Age 2+, limb circumference 13 cm+: CAT Gen 7 tourniquet.
Age 6+, limb circumference 16 cm+: Reliable upper and lower extremity.
Under age 2: No evidence for manufactured tourniquets. Use sustained direct pressure.

Shock Monitoring (Step 12)



DO NOT:

X Do not remove an embedded or impaled object from the wound.
X Do not lift or remove a blood-soaked dressing -- add layers on top.
X Do not remove or loosen a tourniquet once applied unless instructed by a paramedic or physician.
X Do not loosen a tourniquet because the child is screaming -- pain is expected.
X Do not give food or drink to a child with severe bleeding or suspected shock.
X Do not use wound packing for penetrating stab wounds.
X Do not enlarge wound openings to facilitate wound packing.
X Do not assume tachycardia is caused only by anxiety -- suspect significant blood loss.
X Do not wait for hypotension to suspect shock -- it is a late, pre-arrest sign.
X Do not delay calling 101 for severe or uncontrolled bleeding.
X Do not apply a tourniquet directly over a joint.
X Do not attempt a manufactured tourniquet on a child under 2 years -- use direct pressure.

Equipment needed:

— Disposable gloves
— Sterile gauze or clean cloth
— Israeli Emergency Bandage (pressure dressing)
— Tourniquet (CAT Gen 7 -- for children aged 2+ with limb circumference 13 cm+)
— Wound packing material (hemostatic gauze preferred)
— Blanket or coat (for shock management and warmth)

Source: Magen David Adom (MDA) — MDA 101 First Aid Public Guidance (Bleeding Control)
URL: https://www.mdais.org/101/first-aid
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: Unified from MDA and United Hatzalah. Children have 80–90 mL/kg blood volume but much smaller total volume. Compensate longer, then decompensate rapidly.

Shock (Helem) — Child

Israel — CHILD

CALL 101 (MDA) IMMEDIATELY

MDA: 101 **Police:** 100 **Fire:** 102 **Hatzalah:** 1221 **Poison:** 04-7771900

When to apply:

Child showing signs of inadequate tissue perfusion: tachycardia (earliest sign), pale/cool skin, prolonged capillary refill (over 2 seconds), cold clammy extremities, rapid shallow breathing, mottled skin, altered mental status. Children have much smaller total blood volumes (newborn 240 mL, one-year-old 800 mL).

Initial Response (Steps 1–5)

WARNING
Children can appear deceptively stable during compensated shock, then deteriorate suddenly.

Step 1: CALL 101 immediately. State child's age. Shock is a time-critical emergency.

↓

Step 2: ASSESS ABC. Check airway, breathing, circulation.

↓

Is the child breathing?

- NO → **BEGIN** CPR immediately.
- YES → Continue to Step 3.

↓

Step 3: CONTROL any visible bleeding. Apply direct pressure.

↓

Step 4: POSITION the child.

↓

Is the child conscious?

- NO → **Step 5:** Spinal injury suspected? Yes: do NOT move. No: recovery position.
- YES → Lay flat on back. **ELEVATE** legs above head level.

Ongoing Care (Steps 6–10)

Step 6: KEEP child warm. Cover with blanket. Insulate from cold ground. Even in warm weather, shock causes hypothermia.

↓

Step 7: LOOSEN restrictive clothing. Belts, collars, tight waistbands.

↓

Step 8: CALM and reassure the child. Involve parent or caregiver.

↓

Step 9: MOISTEN lips with water only. Do NOT give food or drink.

↓

Step 10: MONITOR continuously until EMS arrives. Track breathing, pulse, skin colour, consciousness.

↓

Is the child's condition worsening?

- **NO** → Continue monitoring, warmth, positioning, and reassurance.
- **YES** → **UPDATE** 101 dispatcher. Maintain legs elevated / airway protected. Re-check bleeding. Prepare for CPR.

WARNING
Bradycardia (slow heart rate) is a PRE-ARREST sign in children. If bradycardia develops, be ready for CPR.

DO NOT:

X Do not sit the child upright -- reduces blood flow to the brain.
X Do not give the child anything to eat or drink -- moisten lips only.
X Do not leave the child unattended -- continuous monitoring essential.
X Do not delay calling 101 -- early IV fluids are critical for survival.
X Do not assume normal blood pressure means no shock -- children compensate then suddenly decompensate.
X Do not attribute tachycardia solely to anxiety or pain -- suspect shock.
X Do not move a child with suspected spinal injury.
X Do not remove blood-soaked dressings -- add layers on top.

Equipment needed:

— Blanket or coat (for warmth and insulation from ground)
— Sterile gauze or clean cloth (for bleeding control)
— Disposable gloves
— Pillow or folded blanket (for leg elevation)

Source: Magen David Adom (MDA) — BLS Guide 2016; MDA 101 First Aid Public Guidance
URL: https://www.mdais.org/101/first-aid
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: Tachycardia is the earliest and most reliable sign. Hypotension is a late, pre-arrest sign. Children compensate longer, then decompensate rapidly.

Smoke / Tear Gas Inhalation First Aid -- Child

Israel — CHILD

CALL 101 (MDA) + 102 (FIRE)

MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

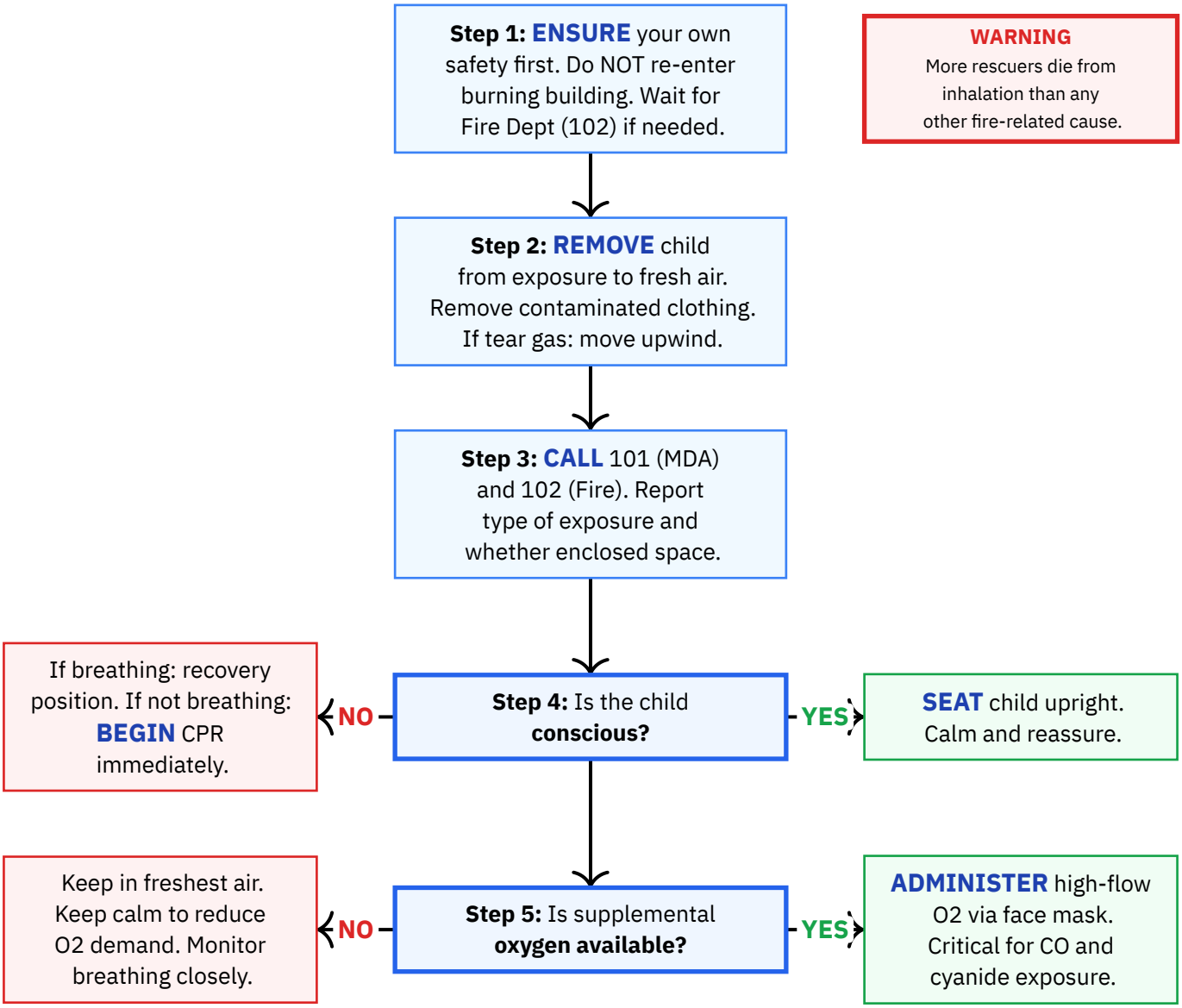
When to apply:

When a child has inhaled smoke, tear gas, toxic fumes, or combustion byproducts. Common Israeli contexts: Lag B'Omer bonfire smoke, apartment fires (winter space heaters), tear gas near residential areas, cleaning product fumes. Children more vulnerable: smaller airways, higher respiratory rate, proximity to ground-level gases.

Summary: Do NOT re-enter hazardous area. Remove child to fresh air. Call 101 and 102. Seat upright. Administer oxygen if available. Watch for stridor (airway swelling). Symptoms can be delayed by hours – all exposed children need hospital evaluation.

Personal reference only — not medical advice

Rescue and Assessment – Steps 1-5



Airway Assessment and Tear Gas – Steps 6-9

Step 6: Does the child **have stridor or severe breathing difficulty?**

- NO → Continue monitoring. Delayed onset of respiratory distress can develop hours later.
- YES → Urgent. Keep calm and upright. Do NOT examine throat. Ensure 101 called. Be prepared for CPR.

WARNING
Stridor after inhalation = danger sign. Airway may swell further and close. Do NOT examine throat.

↓

Step 7: **MONITOR** for CO (pink lips, headache, confusion) and cyanide (seizures, altered consciousness) signs. Note materials that were burning.

↓

Step 8 – Tear gas:
IRRIGATE eyes with clean water for 15 min.
WASH skin with soap.
Remove contaminated clothing.
Move upwind.

WARNING
Do NOT rub eyes after tear gas – worsens irritation.

↓

Step 9: **COVER** with blanket.
MONITOR breathing continuously until EMS.
Be prepared for CPR.
All exposed children need hospital evaluation.

WARNING
Symptoms can be delayed by hours.
A well-appearing child may deteriorate later.
Hospital evaluation required for all.

DO NOT:

- X Do NOT re-enter a burning building or hazardous area -- wait for Fire Department.
- X Do NOT ignore light smoke exposure in a child -- assess even with mild exposure.
- X Do NOT assume a well-appearing child is uninjured -- respiratory deterioration can be delayed by hours.
- X Do NOT rub eyes after tear gas exposure -- worsens irritation.
- X Do NOT give food or drink to a child with stridor or significant breathing difficulty.
- X Do NOT examine the throat of a child with stridor -- can worsen airway swelling.
- X Do NOT leave the child unattended -- monitor breathing continuously.
- X Do NOT position a child with breathing difficulty lying flat -- keep upright.

Equipment needed:

— Supplemental oxygen and face mask (if available)
— Clean water (for eye irrigation after tear gas)
— Blanket (to prevent hypothermia)
— Cool mist humidifier (if available, for stridor management)
— Phone (to call 101 and 102)

Source: Magen David Adom (MDA) – supplemented by Ichud Hatzalah inhalation poisoning and respiratory distress guidelines
URL: https://www.mdais.org/101/first-aid
Publication date: 2026-01-01
Imported: 2026-03-16 · **Last verified:** 2026-03-16
Notes: Common Israeli contexts: Lag B'Omer bonfires, winter apartment fires, tear gas near residential areas. Children vulnerable due to smaller airways and higher respiratory rate.

Snakebite — Child

Israel — CHILD

CALL 101 — TREAT EVERY BITE AS VENOMOUS

MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

Child bitten or suspected bitten by a snake. Treat every snakebite as venomous until proven otherwise. Primary venomous species in Israel: Palestine viper (Daboia palaestinae). Common bite locations: 63% lower limbs, 33% upper limbs. Children are at higher risk due to lower body weight.

Response Steps (1–8)

Step 1: ENSURE scene safety. Move child and bystanders away from snake (2+ metres). Photo snake from safe distance.

WARNING
Do NOT approach, chase, or attempt to catch the snake. Risk of additional bites.

Step 2: CALL 101 immediately. Report child's age, weight, bite location, time. Do NOT wait for symptoms.

Step 3: KEEP child calm and still. Have them lie down. Prevent all movement. Movement accelerates venom spread through lymphatic system.

Step 4: POSITION bitten limb below heart level. Immobilise as for a fracture.

Splint available?

- **NO** → Position limb against body. Secure with clothing if possible.
- **YES** → Apply splint. Keep limb below heart level.

Step 5: REMOVE constrictive items (rings, bracelets, watches, anklets, tight clothing).

Step 6: CLEAN bite gently with water and soap only.

Step 7: MONITOR for systemic signs: swelling, tachycardia, vomiting, difficulty breathing, altered consciousness.

Has the child lost consciousness and stopped breathing?

- **NO** → Continue monitoring. Keep calm, still, lying down. Wait for EMS.
- **YES** → **BEGIN** CPR immediately. Continue until EMS arrives.

Reference

Step 8: Hospital Transport

All snakebite victims require urgent medical evaluation, even without symptoms. Children are at higher risk due to proportionally more venom per kilogram. Antivenom (monovalent equine anti-V. palaestinae) is stocked at Israeli hospitals. Have snake photo ready for medical staff. Israeli paediatric data shows 0% mortality with appropriate care.

DO NOT:

X Do NOT apply a tourniquet -- concentrates venom and causes tissue damage.
X Do NOT cut the bite site -- causes additional injury, does not remove venom.
X Do NOT attempt to suck out the venom -- ineffective and risks infection.
X Do NOT apply ice or cold compresses -- can cause tissue damage.
X Do NOT apply alcohol, antiseptic, or any other substance to the wound.
X Do NOT give the child food or drink -- risk of vomiting, may need anaesthesia.
X Do NOT give hot liquids or alcohol.
X Do NOT chase, catch, kill, or handle the snake.
X Do NOT use a pressure immobilisation bandage -- not recommended for viper bites.
X Do NOT allow the child to walk or move -- movement accelerates venom spread.
X Do NOT wait for symptoms before seeking emergency care.

Equipment needed:

— Splint or rigid object for limb immobilisation
— Water and soap (for wound cleaning)
— Phone (to call 101 and photograph the snake)
— Blanket or clothing (to keep child warm while lying still)

Source: Magen David Adom (MDA) — MDA Snakebite First Aid Guidelines
URL: https://www.mdais.org/101/snake-bite
Publication date: 2026-01-01 · **Imported:** 2026-03-15 · **Last verified:** 2026-03-15
Notes: Primary venomous species: Palestine viper (Daboia palaestinae). Venom travels through lymphatic system, activated by movement. 0% mortality with appropriate care.

Spider Bites First Aid -- Child

Israel — CHILD

CALL 101 (MDA) FOR SYSTEMIC SYMPTOMS

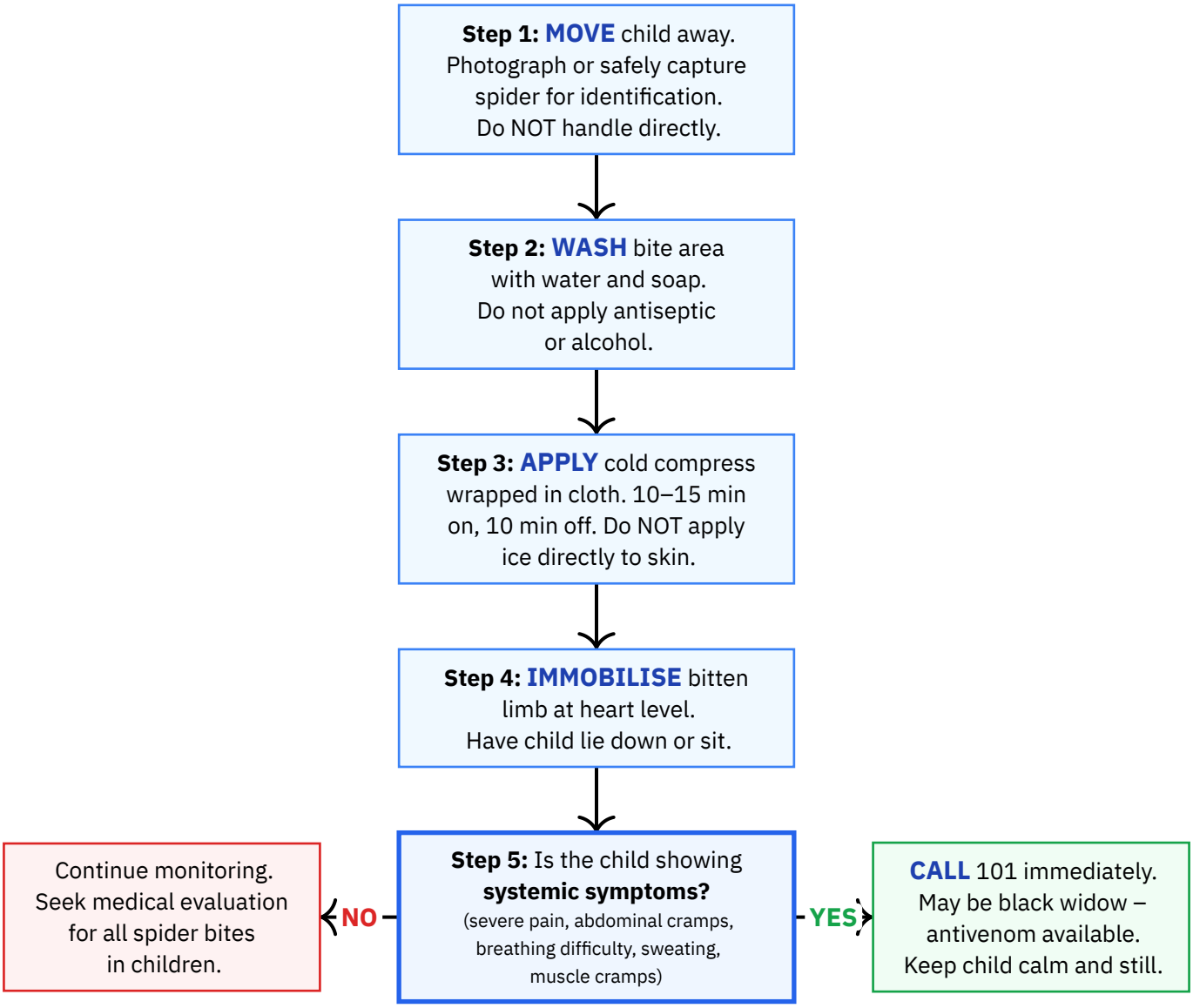
MDA: 101 **Police:** 100 **Fire:** 102 **Hatzalah:** 1221 **Poison:** 04-7771900

When to apply:

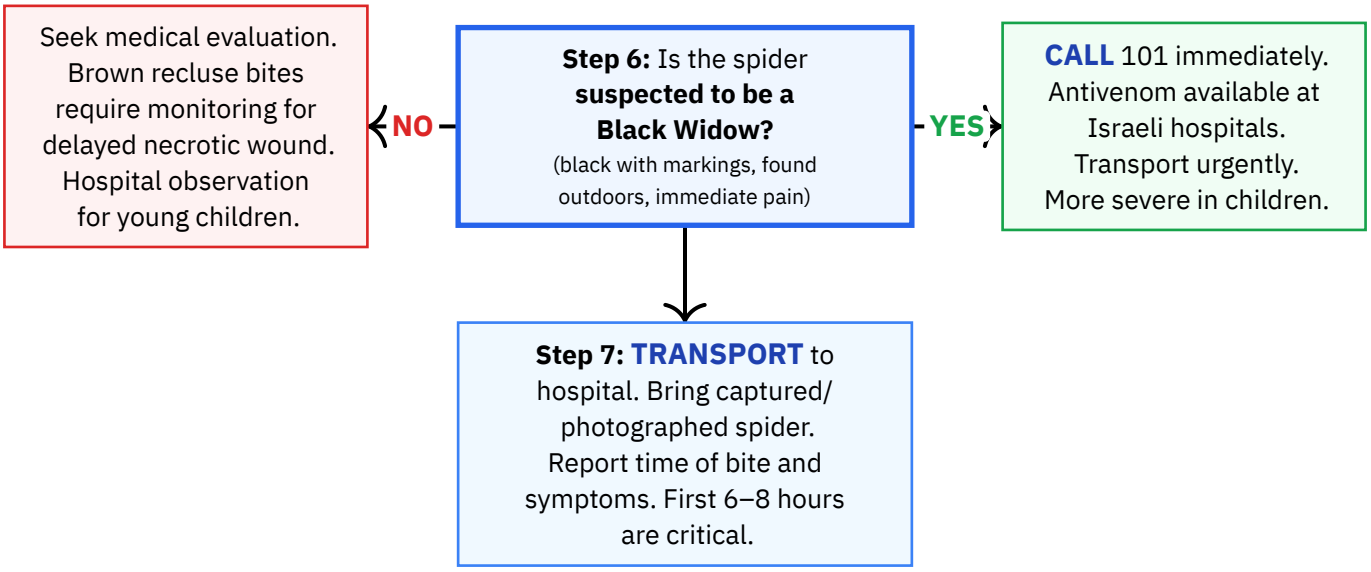
When a child has been bitten or suspected bitten by a spider. Two venomous species in Israel: Brown Recluse (wardrobes, shoes, stored clothing) and Black Widow (gardens, stone walls). Children more vulnerable due to greater venom-to-body-weight ratio. Treat all spider bites in children with medical evaluation.

Summary: Move child away from spider. Photograph spider if safe. Wash bite with soap and water. Apply cold compress. Immobilise bitten limb. Monitor for systemic symptoms. First 6–8 hours are critical. All spider bites in children need medical evaluation.

First Aid and Monitoring – Steps 1-5



Species Identification and Transport – Steps 6-7



Israeli Venomous Spiders

Brown Recluse (shshan chum)
Small, brown, long legs
Lives indoors: wardrobes, shoes
Bite barely felt initially
Effects appear hours later
Causes necrotic ulcer

Black Widow (almana shchora)
Black with markings
Found outdoors: gardens, walls
Immediate pain and burning
Rapid systemic symptoms
Antivenom available in hospitals

DO NOT:

X Do NOT apply a tourniquet -- worsens envenomation by concentrating venom.
X Do NOT cut the bite site -- additional injury, does not remove venom.
X Do NOT attempt to suck out the venom -- ineffective, risks infection.
X Do NOT apply antiseptic, alcohol, or any substance other than soap and water.
X Do NOT bandage tightly over the bite site.
X Do NOT delay medical evaluation for any spider bite in a child.
X Do NOT ignore a painless bite -- brown recluse bites are often painless initially.
X Do NOT give the child medications without medical direction.

Equipment needed:

— Water and soap (for wound cleaning)
— Ice or cold compress wrapped in cloth
— Container or glass (to safely capture the spider)
— Phone (to call 101 and photograph the spider)

Source: Magen David Adom (MDA) – supplemented by Ichud Hatzalah venomous animal guidelines, Ichilov Hospital spider bite guidance, and DrKids.co.il
URL: https://www.mdais.org/101/first-aid
Publication date: 2026-01-01
Imported: 2026-03-16 · **Last verified:** 2026-03-16
Notes: Two venomous species: Brown Recluse and Black Widow. First 6–8 hours critical for recovery. Antivenom for black widow available in Israeli hospitals.

Stroke (Cerebrovascular Event) — Child

Israel — CHILD

CALL 101 — TIME IS BRAIN

MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

Child showing: facial drooping, arm weakness/drift, slurred speech, sudden severe headache with vomiting, new-onset seizure (especially one-sided), sudden altered mental status, sudden loss of balance, sudden vision loss. Children may present atypically: seizures at onset, gradual symptoms, irritability, feeding difficulty, lethargy in young children.

FAST Assessment

Face: Ask child to smile – check for drooping/asymmetry
Arms: Ask child to raise both arms – check if one drifts down
Speech: Ask child to repeat a phrase – check for slurring
Time: Note exact time symptoms began

Note: FAST is not validated for the paediatric population and may miss atypical presentations.

Response Steps (1–7)

WARNING
In children, stroke may present differently. Seizures at onset, gradual symptoms, irritability in young children.

Step 1: APPLY FAST assessment. Also check for atypical signs: seizures, headache with vomiting, sudden behavioural changes.

↓

Step 2: CALL 101 immediately. State: suspect child stroke.

↓

Step 3: NOTE exact time symptoms began. Critical for treatment decisions. Alteplase: within 4.5 hrs. Clot removal: within 6–24 hrs.

↓

Step 4: Is the child conscious?

- **NO** → **Step 5:** Breathing? Yes: recovery position. No: **BEGIN** CPR.
- **YES** → Lay down with head and shoulders slightly elevated. Keep calm/still.

↓

Step 6: Is the child having a seizure?

- **NO** → Continue monitoring.
- **YES** → Clear area. Do NOT restrain. Do NOT insert anything in mouth. After: recovery position. Note duration/type.

↓

Step 7: MONITOR continuously until EMS arrives. Watch for changes in consciousness, breathing, symptoms. Be ready for CPR.

DO NOT:

X Do NOT delay calling 101 to wait and see if symptoms resolve.
X Do NOT give any food or drink -- stroke impairs swallowing, risk of aspiration.
X Do NOT give any medications, especially not aspirin -- if haemorrhagic, aspirin will worsen it.
X Do NOT attempt to drive to hospital -- MDA ambulances begin treatment en route.
X Do NOT assume a child cannot have a stroke -- children can have strokes at any age.
X Do NOT restrain the child during a seizure.
X Do NOT put anything in the child's mouth during a seizure.
X Do NOT rely solely on FAST -- adult screening tools are not validated for children.

Source: Magen David Adom (MDA) — MDA Stroke Recognition Protocol (FAST); AHA 2024/2026
URL: https://www.mdais.org/101/stroke
Publication date: 2024-11-18 · **Imported:** 2026-03-15 · **Last verified:** 2026-03-15
Notes: Treatment windows: alteplase 4.5 hrs (ages 28 days–18 yrs), mechanical clot removal 6–24 hrs (age 6+).

Tourniquet Application -- Child

Israel — CHILD

CALL 101 (MDA) — STOP THE BLEEDING

MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

When a child has uncontrollable life-threatening limb haemorrhage: blast/shrapnel injuries, amputations, severe lacerations with arterial bleeding, or when direct pressure is insufficient. Children have less total blood volume and develop haemorrhagic shock faster.

Summary: CAT tourniquet effective for most children aged 2+. SWAT-T/RATS for very small limbs. Place 5–7 cm above wound, tighten until bleeding stops, record time, never remove. Transport urgently.

Application – Steps 1-5

Step 1: Is bleeding from a **limb and cannot be controlled with direct pressure?**

- NO → Use direct pressure with sterile gauze. Torso/neck: pack wound and apply max pressure.
- YES → **APPLY** tourniquet immediately. Proceed to Step 2.

Step 2: Is child's limb **large enough for CAT?** (13 cm circumference, age 2+)

- NO → Use SWAT-T or RATS. If unavailable: max direct pressure or improvised tourniquet.
- YES → Use CAT tourniquet.

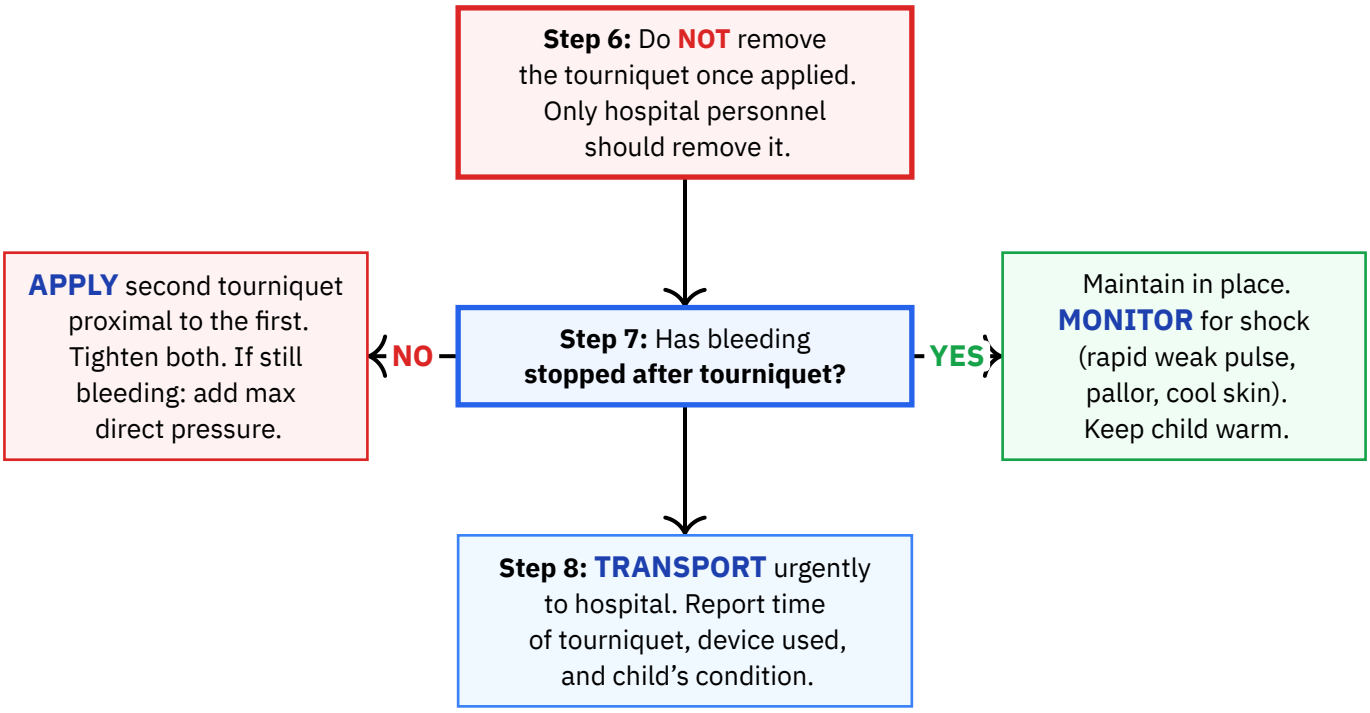
Step 3: **PLACE** tourniquet 5–7 cm above wound. Over single bone (upper arm/thigh), not over joint.

Step 4: **TIGHTEN** until bleeding stops. Turn windlass, secure in clip. Check distal bleeding has stopped.

Step 5: **RECORD** time of application. Write on tourniquet, skin, or tape. Use 24-hour format.

WARNING
Very painful. Warn child. Pain is expected. A loose tourniquet is WORSE than none (causes venous bleeding without arterial occlusion).

Post-Application – Steps 6-8



DO NOT:

X Do NOT remove a tourniquet once applied -- only hospital personnel should remove it.
X Do NOT apply a tourniquet for venomous animal bites (snakes, scorpions, spiders) -- worsens outcomes.
X Do NOT apply over a joint (elbow, knee) -- place on upper arm or upper thigh.
X Do NOT apply too loosely -- worse than no tourniquet (venous without arterial occlusion).
X Do NOT use standard CAT on very small infant limbs (under ~13 cm circumference).
X Do NOT delay tourniquet to attempt prolonged direct pressure on massive haemorrhage.
X Do NOT forget to record the time of application.

Equipment needed:

— CAT (Combat Application Tourniquet) Gen 7 -- for children aged ~2+ with sufficient limb size
— SWAT-T tourniquet -- for all pediatric sizes including very small limbs
— RATS tourniquet -- alternative for all pediatric sizes
— Improvised tourniquet materials (triangular bandage + rigid windlass) if no manufactured device
— Marker or pen (to record time of application)
— Sterile gauze (for wound packing if needed)
— Phone (to call 101)

Source: Magen David Adom (MDA) – supplemented by Ichud Hatzalah tourniquet guidance, Israeli First Aid resources, and NAEMSP/TCCC pediatric tourniquet evidence
URL: https://www.mdais.org/101/first-aid
Publication date: 2026-01-01
Imported: 2026-03-16 · **Last verified:** 2026-03-16
Notes: CAT Gen 7 effective for limb circumference 13–16 cm and above (most children aged 2+). SWAT-T and RATS effective across all pediatric sizes.